

Contents

Part I Fundamentals of Parent-Child Interaction Therapy

1 Overview of Parent-Child Interaction Therapy	3
What Is PCIT?	4
Typical Course of Treatment	5
Theoretical and Historical Underpinnings	6
Key Features of PCIT	7
Using the Second Edition	14
References	15
2 Research on PCIT	17
<i>Stephanie Wagner</i>	
Early PCIT Research	17
Recent Research Initiatives	17
Suggestions for Future Research	23
References	26
3 Intake Assessment and Therapy Orientation Session	31
Who Should Attend the Intake Session?	32
Flexible Battery Approach	33
Semi-structured Intake Interview	34
Eyberg Child Behavior Inventory (ECBI)	35
Sutter-Eyberg Student Behavior Inventory – Revised (SESBI – R)	36
Dyadic Parent-Child Interaction Coding System – III (DPICS – III)	36
Joining with and Motivating Parents	40
Sharing Test Results with Parents	43
Explaining “Specialized Parenting”	43
Introducing PCIT to Parents and Children	44
Expectations for Attendance	45
Reducing Barriers to Treatment	46
Explaining the First Homework Assignment	47
Summary	47
References	48

4 Teaching Child-Directed Interaction	49
Overview of Teaching Session	49
Review Homework	51
Presenting the Goals of Child-Directed Interaction	51
Explaining the Five Minutes of Homework Each Day	51
“Selling” CDI to Skeptical Parents	53
Explaining the Overriding Rule of Letting the Child Lead	55
Teaching the “Avoid” Skills of Child-Directed Interaction	56
Teaching the “Do” Skills of Child-Directed Interaction	59
Using Strategic Attention	66
Using Selective Ignoring	67
Handling Disruptive Behaviors That Cannot Be Ignored	71
Modeling Skills in Combination	71
Role-Plays of Child-Directed Interaction	71
Appropriate Toys for Child-Directed Interaction	72
Dealing with One-Parent and Two-Parent Families	73
Incorporating Siblings	74
Adjusting CDI to the Child’s Developmental Level	74
Problem-Solving with Parents on Logistical Issues	74
Assigning Child-Directed Interaction Homework	75
References	76
5 Coaching Child-Directed Interaction	77
Overview of a Typical Coaching Session	78
Setting Up for the Coaching Session	79
Check-In and Homework Review	80
Parental Non-compliance with CDI Homework	81
Observing and Recording Child-Directed Interaction Skills	85
Coaching the “Do” and “Avoid” Skills: Tips for Therapists	86
End of Session Debriefing and Homework Assignment	99
Progression of CDI Coaching Sessions	99
What if a Caregiver Does Not Reach CDI Mastery?	100
References	101
6 Teaching Parent-Directed Interaction	103
Rationale for Why Young Children Should Comply with Parental Commands	103
Structuring the PDI Teaching Session	104
Importance of Consistency, Predictability, and Follow Through	104
Importance of Memorizing PDI Diagrams (e.g., Using Exact Words)	106
Rationale for Use of “Compliance Exercises”	106
Giving Effective Instructions	107
Practicing How to Give Effective Instructions	112
Determining Compliance	113

Praising Compliance	115
Rationale for Disciplining Children with Time-Out in a Chair	116
The Time-Out Warning	118
Logistical Issues Associated with Time-Out	119
Getting the Child to Time-Out	119
What If the Child Agrees to Comply on the Way to Time-Out?	121
What if the Child Takes a Toy to Time-Out?	122
What if the Child Puts Himself in Time-Out?	122
Length of Time-Out	122
Common Misbehaviors in Time-Out that Should Be Ignored	123
Time-Out Does Not End Until the Original Instruction Is Obeyed	124
Use of a Second Instruction to Over-Teach Compliance	125
Use of Play Therapy to Decrease the Child's Anger Level	126
Three Time-Out Behaviors That Cannot Be Ignored	126
Using the Time-Out Room as the Back-Up for Time-Out Chair Behaviors that Cannot Be Ignored	127
Role-Playing and Wrap-Up	131
References	132
7 Coaching Parent-Directed Interaction	133
Overview of a Typical PDI Coaching Session	133
Preparing for PDI Coaching Sessions	134
Check-In and Homework Review	135
Observing and Recording Parent-Directed Interaction Skills	136
Criteria for Mastery of Parent-Directed Interaction Skills	137
General Guidelines for Coaching PDI	138
Debriefing Parents Following the Session	148
Homework	151
References	152
8 Progressing Through the Parent-Directed Interaction Sessions	153
First PDI Coaching Session	157
Second PDI Coaching Session	165
Third PDI Coaching Session	166
Fourth PDI Coaching Session	169
Fifth PDI Coaching Session	172
Sixth PDI Coaching Session	175
Seventh PDI Coaching Session and Beyond	176
Graduation Session	178
Conclusion	181
Reference	182

Part II Adaptations of Parent-Child Interaction Therapy

9	Younger Children	185
	“Meaningful Differences in the Everyday Experience of Young American Children”	186
	Using Child-Directed Interaction with Infants	191
	Adapting Child-Directed Interaction and Parent-Directed Interaction for Toddlers	192
	Case Illustration	195
	Parent-Child Interaction Therapy and Prevention	198
	References	199
10	Older Children	201
	Developmental Issues	201
	University of Oklahoma Health Sciences Center Adaptations for Older Children	202
	Concerns Raised by the SAMHSA Study Regarding PCIT with Older Children	202
	Adapting CDI to Older Children	204
	Adapting PDI to Older Children	208
	Command Training (CT) Module	209
	Time-out with Incentive Chart (TIC) Module	214
	Time-Out with Suspension of Privilege (TSP) Module	219
	Alternatives to the TSP Module for Children Who Are Extremely Aggressive and Defiant	221
	Conclusion	222
	References	223
11	Siblings	225
	Including Siblings with Disruptive Behavior Problems	226
	Including Siblings Without Behavior Problems	226
	Coaching Older Siblings as Babysitters	227
	Direct Coaching of Children to Decrease Sibling Conflict	227
	Direct Coaching of Children to Improve Social Skills (e.g., Asperger’s)	228
	Using the Cooperation Game	229
	Conclusion	235
12	Autism Spectrum Disorders	237
	<i>Joshua Masse</i> Theoretical Similarities of PCIT and Empirically Supported Treatments for ASD	239
	Child-Directed Interaction	240
	Working with Stereotyped, Repetitive Behavior During CDI	241
	Parent-Directed Interaction	242
	Time-Out Concerns	244
	Clinical Limitations	245

Adapting PCIT: Communication and Social Skills Component	247
Case Study	250
Conclusion	252
References	252
13 Child Physical Abuse	255
<i>Lisa M. Ware and Amy D. Herschell</i>	
Understanding the Population	256
Typical Referral Concerns	257
Adaptations and the Unique Treatment Needs of Families with a History of CPA	261
General Therapeutic Issues	262
Assessment	265
Treatment	270
Incorporating Additional Coaching Targets	270
Tailoring and Adaptations Specific to PDI	272
Case Illustration	274
Conclusion	281
References	282
14 Anxiety Disorders	285
Separation Anxiety Disorder	285
CARD Protocol for Adapting PCIT for Treatment of SAD	287
Treatment Outcome Research	290
Generalized Anxiety	291
Enhancing PCIT for Young Children with Generalized Anxiety	291
The Case of Alexander H	293
References	299
15 Attention Deficit Hyperactivity Disorder	301
Conducting PCIT with Children Diagnosed with ADHD	302
References	317
16 Extremely Aggressive and Explosive Children	319
The Case of Mario	319
CDI Pitfalls with Explosive Children	320
CDI Adaptations for Explosive Children	321
PDI Adaptations	325
Conclusion	327
Reference	327
17 Marital Conflict	329
<i>Ashley B. Tempel and Matthew E. Goldfine</i>	
Encouraging Both Parents to Attend Treatment	330
Assessment Techniques	330
Integrating Marital Therapy Within PCIT	333
Termination of Treatment	339
References	340

18	Parents with Major Life Stressors	341
	<i>Kimberly P. Foley</i>	
	Strategies for Working with Multi-stressed Families in PCIT	343
	Divorce	344
	Substance Abuse	348
	Intimate Partner Violence	351
	Low-Income Families	357
	Parting Words	360
	References	361
19	Ethnic Minority Children and Families	363
	<i>Karla Anhalt and Joaquin Borrego</i>	
	Parent Training, PCIT, and Application to Ethnic Minorities	363
	PCIT with Hispanics	364
	PCIT with Native Americans	368
	PCIT with African-Americans	371
	Examining Social Validity	373
	Conclusion and Final Recommendations for Therapists	374
	References	375
20	Staff-Child Interaction Therapy	377
	<i>Gus Diamond</i>	
	Baseline	380
	SCIT Video on Child-Directed Play	381
	Didactic Child-Directed Interaction Session with Trainer	381
	Coding and Coaching of Child-Directed Interaction	381
	SCIT Video on Staff-Directed Interaction	382
	Didactic Staff-Directed Interaction Session with Trainer	382
	Coding and Coaching of Staff-Directed Interaction	382
	Coaching Sessions on the Milieu (Two-Way Radio with Earpiece for Staff)	382
	Post-intervention Findings	384
	Conclusion	384
	References	384
21	Teacher-Child Interaction Therapy for Preschool Classrooms . .	385
	<i>Jennifer Tiano</i>	
	Research Underpinnings	385
	Establishing Rapport with the Teacher	385
	Format of Training	386
	Coaching TCIT	386
	Child-Directed Interaction in TCIT	386
	TCIT Mastery Criteria	388
	Teacher-Directed Interaction Phase	388
	Follow-Up Consultation	390
	References	390

22 School Consultation	393
Assessment	394
Establishing Rapport and Credibility with the Teacher	395
Conclusion	411
References	411
23 Home-Based PCIT: From the Lab to the Living Room	413
<i>Joshua Masse</i>	
Clinical Advantages	413
Clinical “Tips”	415
Concluding Remarks	419
References	419
24 PCIT Around the World	421
<i>Åse Bjørseth, Anne Kristine Wormdal, and Yi-Chuen Chen</i>	
Adapting PCIT for New Countries	422
International Issues with PCIT	424
Conclusion	426
References	427
25 Training Issues	429
PCIT National Advisory Board	429
Minimum Professional Qualifications to Conduct PCIT	430
Becoming a PCIT Therapist: Minimum Training Requirements	431
Becoming a PCIT In-agency Trainer: Minimum Training Requirements	434
Becoming a PCIT Master Trainer: Minimum Training Requirements	434
Costs of Starting a PCIT Program	434
References	437
Appendices	439
Subject Index	469

Part I
Fundamentals of Parent-Child Interaction
Therapy

Chapter 1

Overview of Parent-Child Interaction Therapy

A 3-year-old boy, “Christopher,” ran recklessly around the playroom overturning chairs and tables, stopping just long enough to poke a Lincoln Log in his mother’s face and yell, “Stupid bitch! I’m gonna kill you! Pottyhead.” She had come for help after awakening from a nap to find her young son hovering over her with a kitchen knife. Christopher’s mother had a history of being abused by her step-father and was now in a violent relationship with her spouse. Christopher had witnessed many confrontations between his parents including incidents in which his father choked his mother, a vase was thrown and shattered on a wall, and both of his parents shouted obscenities at each other. Christopher was exhibiting serious aggression both at home and toward other children at daycare. His mother felt little self-worth and no sense of control over her life, and she interpreted her son’s behavior problems as further evidence of her personal inadequacy.

“Mr. Cheng” was a chemical engineer while his wife worked as a receptionist in a dental practice. They presented with a 5-year-old girl who was bossy, non-compliant, sassy, and disrespectful at home. Mrs. Cheng tearfully reported, “I just don’t get what we’re doing wrong. Her teacher says she’s fine at school, and she behaves for my mom.” The parents admitted that they have a tendency to spoil “Sela,” giving in to her demands for toys and candy. They reported that when they do say “no” to Sela, she argues, stomps her feet, slams doors, and even spits at them. Sela’s behavior had begun to interfere with their social life. The parents stopped bringing her to their friends’ homes because they were so embarrassed by the scenes she made. Additionally, Mrs. Cheng stopped volunteering in her class at Chinese School on the weekend because the teacher expressed concern that Sela only threw tantrums when her mother was present.

By age 6, “Antonio” had been involved with both the fire department and the police for vandalism. He was lucky to escape injury when he took a cigarette lighter

Throughout this book, we present many case examples. All of these case examples are fictional and represent composites of numerous clients we have worked with over our careers. We did not use the real names of any actual clients. Case examples also were carefully chosen such that no identifying information was provided (e.g., actual last names, addresses, specific or unusual medical history). We thank our clients for providing us with a wealth of experience that hopefully helps PCIT come to life for the readers of this book.

from his home and set a neighbor's discarded Christmas tree on fire. On another occasion, he was brought home in a police car after throwing rocks at a passing minivan. The driver stopped and called police when Antonio refused to give his name or say where he lived. When the police questioned him, he initially insisted that he was from Mars and had no home on earth. Antonio's mother presented as a young, overwhelmed mother working two jobs to support her four children. She often had to leave the younger children in the care of her 13-year-old latch key son. The mother's biggest concern was the fear that Antonio would turn out like his father, who was serving time in prison for theft. She finally sought help after a "really bad week" in which Antonio was caught stealing some money from his teacher's desk and drowned his sister's hamster in the toilet.

These families are representative of the types of clients referred for parent training (please see footnote below describing client confidentiality for this book). A common sentiment expressed by these parents is, "I love my child, but I just don't like him very much." Referred parents often feel incompetent in their parenting role and acknowledge that anger, despair, and depression interfere with their ability to provide nurturance to their young children. The treatment approach described in this volume, Parent-Child Interaction Therapy (PCIT), was designed specifically for families such as these with young children who are experiencing acting-out behavior problems. We view PCIT as a basic framework for treating many of the disruptive behaviors that arise in children aged 3–6 years.

What Is PCIT?

PCIT is an evidence-based behavioral parent training program developed by Dr. Sheila Eyberg that involves working with parents and their young children (ages 3–6). In this approach, the therapist coaches the parent during real-time interactions with the child. Most often, the therapist coaches from behind a one-way mirror, communicating with the parent through a hearing aide type device called a bug-in-the-ear. Parents are coached in two sets of skills. In Child-Directed Interaction (CDI), parents learn to use traditional play therapy skills to enhance the parent-child relationship. The skills of CDI include praising, reflecting, imitating, describing, being enthusiastic, and providing contingent attention. In Parent-Directed Interaction (PDI), parents learn skills for improving child compliance and decreasing disruptive behaviors. PDI skills include giving good commands, praising compliance, using time-out in a chair for non-compliance, and establishing standing house rules. PCIT is a short-term intervention that usually requires approximately 12 one-hour weekly sessions. Although short-term, PCIT is not time-limited. Progression through the treatment program is based on skill mastery (assessed via a standardized coding system) so that treatment length varies across families. PCIT is concluded when the parent masters both the CDI and PDI skills and the child's behavior improves to within normal limits.

Typical Course of Treatment

PCIT always begins with a pre-treatment assessment session in which interview information is gathered on history and presenting problems, questionnaires are given, and the therapist observes (and may videotape) a sample of how the parents and child relate to one another. Feedback regarding assessment results and treatment planning is provided to families either at the end of the pre-treatment assessment session or in a separate “therapy orientation session.” The typical course of treatment is described to the family, emphasizing the ways in which PCIT can address the specific concerns that brought them in for treatment and any additional areas of concern that may have emerged from the evaluation.

Treatment begins with CDI. A teaching session is conducted in which the therapist meets only with the parents and teaches them the CDI basics using didactic presentation, discussion, live modeling, and role-playing. Parents are active participants. They are encouraged to generate numerous questions and process how the principles discussed fit with their own parenting philosophies. After the teaching session, the parents and child are seen together for several sessions of CDI skills coaching. The specific number of sessions needed depends on how rapidly parents are able to acquire the skills and the nature of the child’s presenting problems, but we find that most families are ready to move on to the discipline stage after approximately four coaching sessions. Pre-determined skill criteria are used to assist the therapist and family in deciding when it is time to move ahead to the discipline component of PCIT.

After mastering the CDI criteria, the caregivers again meet alone with the therapist for a session to learn the basics of the discipline stage of treatment. This PDI teaching session consists of didactic information, discussion, and role-playing. In the sessions that follow, the parents are again seen together with the child for several sessions of direct skills coaching. These sessions begin in the clinic setting and may be extended to community settings (e.g., the grocery store) to enhance cross-setting generalization. Treatment is concluded with a “graduation session” when all of the presenting problems have been resolved or substantially improved. Upon graduation, parents receive a certificate and children receive a prize (e.g., blue ribbon) to acknowledge their successes. We find that most families meet their treatment goals after approximately six discipline sessions.

A post-treatment evaluation session is held in which the measures that were administered before therapy are repeated. At the end of this session, feedback is provided to the family in which pre- to post-treatment improvements are reviewed. This session helps parents to solidify their recognition of improvements that have been occurring gradually over several weeks of treatment. For most families, the full course of treatment can be conducted in approximately twelve sessions (see Table 1.1), and is consistent with the short-term treatment philosophy adhered to by many health maintenance organizations. A booster session is usually scheduled at 3 months but may occur earlier if needed. Additional booster sessions may be scheduled to enhance maintenance of parenting skills and address problems that arise as children face new developmental challenges.

Table 1.1 Steps of parent-child interaction therapy

Step 1:	Pre-treatment assessment of child and family functioning (1–2 sessions)
Step 2:	Teaching Child-Directed Interaction skills (1 session)
Step 3:	Coaching Child-Directed Interaction skills (3–4 sessions)
Step 4:	Teaching Parent-Directed Interaction skills (1 session)
Step 5:	Coaching Parent-Directed Interaction skills (4–6 sessions)
Step 6:	Post-treatment assessment of child and family functioning (1–2 sessions)
Step 7:	Boosters (as needed)

Theoretical and Historical Underpinnings

When Dr. Sheila Eyberg developed PCIT, she drew from her broad background in operant theory, traditional child psychotherapy, and early child development (Eyberg, 1988). The theoretical basis of PCIT lies in Diana Baumrind's work (1966, 1967) regarding parenting styles. In the following excerpt, Sheila Eyberg (n.d.) described the influence of Baumrind's work on the development of PCIT.

Baumrind demonstrated the importance of parents meeting young children's dual needs for nurturance and for limits, which she described as authoritative parenting. Her research showed that to promote optimal child outcomes, we must focus on promoting optimal parenting styles and parent-child interactions. PCIT draws on both attachment and social learning theories to achieve authoritative parenting. Attachment theory asserts that sensitive and responsive parenting provides the foundation for the child's sense of knowing that he or she will be responded to when necessary. Thus, young children whose parents show greater warmth, responsiveness, and sensitivity to the child's behaviors are more likely to develop a secure sense of their relationships and more effective emotional and behavioral regulation.

Eyberg emphasized that the two aspects of Baumrind's authoritative parenting, nurturance and limit-setting, parallel the two stages of Parent-Child Interaction Therapy (Child-Directed Interaction and Parent-Directed Interaction). Whereas Eyberg drew heavily from attachment theory in conceptualizing the effects of the child-directed phase of the program, she refers to social learning theory, particularly Patterson's coercion theory (1982), when conceptualizing the parent-directed (limit-setting) aspect of PCIT. According to this theory, disruptive child behavior is developed and maintained through parental reinforcement such as providing negative attention and allowing children to escape demands. In the second phase of PCIT, parents are taught to set limits and provide consistent consequences while avoiding escalating, coercive interactions with the child.

Historically speaking, Dr. Eyberg (2004) was heavily influenced by the work of her colleague, Dr. Constance Hanf, who had developed a two-stage operant model for modifying the problematic behavior of multiply handicapped young children (Hanf, 1969). In the first stage of treatment, mothers were taught the technique of differential reinforcement. In other words, they were taught to give their attention to their child's positive behaviors and to ignore negative behaviors. In the second stage, parents were taught to give clear directions, consistently reward compliance with praise, and provide a time-out consequence for non-compliance. Perhaps the most

appealing aspect of Hanf's approach was that she worked with the parent and child together, doing coaching parenting skills on-the-spot. Eyberg is one of several researchers who have developed and evaluated aspects of Hanf's original model (e.g., Barkley, 1987; Forehand & McMahon, 1981; McMahon & Forehand, 2003; Webster-Stratton, Reid, & Hammond, 2004).

Although Hanf's approach produced clear and rapid behavioral change, Eyberg recognized that traditional play therapy also had much to offer these families, with its emphasis on developing a warm and safe therapeutic relationship. She found that just as parents could be taught the operant skill of differential attention, they could be taught the traditional play therapy skills of following the child's lead, providing undivided attention, describing play activities, reflecting and expanding upon child verbalizations, and imitation. PCIT was "born" in 1974 when Sheila Eyberg wrote her first grant examining the effectiveness of the model (Eyberg, 2004).

Eyberg's integration of operant methods and traditional play therapy techniques took place within a solid developmental framework. That is, PCIT is conducted in the context of dyadic play situations largely because "play is the primary medium through which children develop problem-solving skills and work through developmental problems" (Eyberg, 1988, p. 35). A developmental perspective is essential to providing effective PCIT. Many of the problems that arise in parent-child dyads are related to developmental struggles for autonomy or inappropriate developmental expectations held by parents.

Key Features of PCIT

Working with the Parent and Child Together. We believe it is critical that any therapeutic work done with a preschooler directly involves the child's caregivers. Parents have enormous influence over their young children's behavioral and emotional development, and some parenting practices may cause or exacerbate their young children's problems. According to Eyberg (1988, p.35),

...many of the behavioral problems young children present are established through their earliest interactions with their parents. Even in those cases where the child's problems seem to originate because of biological characteristics, such as difficult temperament, or neurological defects suspected in autistic, hyperactive, or developmentally impaired youngsters, many of the problem behaviors seem to be intensified by the interaction patterns between parent and child. (p. 35)

Just as parents may negatively influence their children's behavior and the parent-child relationship, they have enormous power to influence their preschoolers in a positive way. In early childhood, parents are the center of the child's world, providing nurturance, sustenance, safety, and learning opportunities. Preschool-age children are not cognitively sophisticated enough to reason independently, and the influence of peers is minimal. At no other time in childhood or adolescence are parents in a position to influence their children in such a dramatic and pervasive way as they are during the preschool years. In later childhood and adolescence, the influence of parents is overshadowed by the substantial influence of peers, teachers,

romantic partners, and developmental needs for autonomy. The therapist's power to influence a young child in a one-to-one therapeutic relationship pales in comparison to the power of parents to produce change through their interactions with their children.

Sometimes the parents we work with express disappointment that they cannot simply drop off their child for an hour of "magical" therapy with us each week and expect their problems to be resolved. We explain to these parents that what we can do in 1 h of individual child therapy once a week is a mere drop in the bucket in comparison to what we can accomplish if the parent becomes a "therapist" for their child at home, every day of the week. We also help some parents to recognize the economic reality that individual therapy is a very long, expensive proposition, whereas most problems of early childhood can be effectively treated in a relatively short time period using PCIT.

Direct Coaching of Parent-Child Interactions. We believe that the feature of PCIT that makes it so effective is the use of direct coaching of parent-child interactions. In indirect approaches, skills are taught individually to parents, are practiced at home, and then the parents report back any problems they had at the following therapy session. Direct coaching of dyads presents several advantages over the indirect method.

The first advantage of direct coaching is that parental errors can be corrected promptly, before they become well ingrained through a week of home practice. Second, every child presents his or her own unique challenges and the creative clinician can use the direct coaching method to make quick modifications as problems arise, modeling good problem-solving skills for parents. Third, many parents lack the confidence to use the new skills without the initial encouragement and support offered by the therapist-coach. Fourth, because the therapist is able to shape parenting skills by rewarding successive approximations, direct coaching results in faster learning. And fifth, parents are not always accurate reporters of their own or their young children's behavior. Relying on parent report of the skills they use and the child's response can result in inaccurate perceptions of treatment progress.

When explaining the importance of direct coaching to parents, we sometimes use a tennis analogy. We often tell parents:

Learning new skills is challenging. For example, let's consider tennis. Suppose that you want to improve your tennis skills. Would it make sense to meet once a week with your tennis pro in his office to discuss your tennis game? No, it won't work. The tennis pro needs to watch your strokes and give you immediate feedback on how to hit a better backhand or forehand, and you need to over-practice a particular stroke through drills so that it comes naturally when you're in a real game. It's the same with parenting. It will not do you much good for us to just talk in my office about parenting because I need to see you and your child together in action. Then I can coach you in the use of new skills. When your child throws a Mr. Potato Head at you, whines, spits, disobeys, and tells you to "shut up," I will coach you in exactly what to say and do. You will have the opportunity to over-practice special skills here in the clinic until they become habits. Then, when you are in real-life parenting situations outside of my office, these skills will come naturally.

Direct coaching is both the heart and the art of PCIT. Nearly any beginning therapist can quickly learn the mechanics of conducting PCIT and will be able to

teach the core skills outlined in Chapter 4 and Chapter 6. However, the challenging and creative aspect of this therapy is recognizing the subtle qualities of parent-child interactions that characterize dysfunctional or more adaptive parent-child relationships, and then translating those observations into clinically sensitive and effective coaching strategies. We find that the more we do PCIT, the more we learn about families and the more we add to our coaching repertoires. In Chapter 5 and Chapter 7, we present a vision for how a skilled PCIT clinician might go beyond coaching the standard set of PCIT skills to coaching parents in a range of complex interactions (e.g., recognizing and working at their child's level of development, making use of body language and voice qualities, encouraging child autonomy within developmental norms, supporting self-acceptance and problem-solving efforts, de-escalating tantrums).

Using Data to Guide Treatment. Although PCIT is a manualized program with clear objectives for each session, it is not a cookbook approach. (Bahl, Spaulding, & McNeil, 1999; Greco, Sorrell, & McNeil, 2001; McNeil, Filcheck, Greco, Ware, & Bernard, 2001). The therapy is individualized for each child and family based on data that are gathered each session. Parent-child interactions are coded at the beginning of each session to determine a family's progress toward the pre-established mastery criteria. This information guides the emphasis of coaching in that session. For example, one of the PDI mastery criteria is that 75% of the parent's commands must be "effective" (e.g., direct, positively stated). If during the coding a parent is found to have used a large percentage of indirect commands, the therapist will focus coaching on increasing the use of direct commands. Parents must master the CDI criteria before progressing to the PDI phase of treatment. And, the parents must meet mastery criteria for the PDI skills for therapy to be terminated. Thus, although PCIT is most often a short-term intervention, it is *not* time-limited. The number of sessions may vary widely among families, but each family receives their optimal number of sessions.

Sensitivity to Developmental Concerns. Many young children engage in non-compliant, aggressive, and highly active behavior during the course of normal development. In most children, behavior problems peak at about age 3 and decline during the remaining preschool years. The nature of behavior problems displayed by typical preschoolers is clearly related to the particular developmental hurdles facing the child (Forehand & Wiersen, 1993). For example, at ages of 2–3, young children begin seeking independence and autonomy. As they work on these developmental tasks, they are likely to be non-compliant with parents and to have temper tantrums when they do not get their way. Doing things oneself, without parental assistance, is very important to a 3-year-old. This developmental need can precipitate confrontations between the parent and child. Parents who resist allowing the child to pour his or her own juice or who cannot allow their child to wear a favorite summer outfit during the cold of winter may experience the strength of such autonomy needs firsthand.

Rates of child compliance in normally developing children vary as a function of how they are measured, but most preschoolers obey between 50 and 75% of their parents' requests (Schroeder & Gordon, 1991). As preschoolers develop greater

verbal competence, the nature of their non-compliance changes from simple direct defiance to more complex negotiation to avoid or defer compliance (Kinzyski, Kochanska, Radke-Yarrow, & Ginius-Brown, 1987). Excuses for not obeying a parent's request often mirror the excuses children have heard from parents, including: "I can't, I have a headache," "I don't have enough time," "I'm too busy right now," or "I'll do it after I'm done playing."

At ages 4–5, young children are expected to begin learning to play cooperatively with other children and normative behavior problems include aggression, difficulty sharing, and difficulty taking turns. Aggression toward other children declines as preschoolers learn more sophisticated means of solving problems through verbal negotiation and begin to learn to regulate their own emotions. These critical preschool experiences in learning how to interact effectively with peers set the stage for social adjustment in kindergarten and early elementary school.

Throughout PCIT, we educate parents about appropriate developmental expectations. At times we notice that parents seem to have unrealistic expectations for what their child should be able to do (e.g., preschoolers cleaning their own rooms independently). At other times, we find that parents do too much for their young children, hampering the development of age-appropriate self-help skills (e.g., dressing a 4-year-old). A PCIT coach must have a keen sense of each child's developmental capabilities. For example, when coaching PDI, it is important for the therapist to suggest only commands for behaviors that the child is capable of performing. It would be inappropriate to tell a 3-year-old to draw a dragon or to use commands with vocabulary that is above the child's level. By educating parents to use language that their child can comprehend, we teach them to set their children up for success. It is important for parents to understand that all young children act out at times. This helps parents to be more tolerant of common behavior issues that occur during the preschool years (e.g., whining, temper tantrums, occasional biting, bed wetting).

Intervening Early. A common belief is that most problems that occur during the preschool years will be outgrown as the child passes through a difficult developmental phase. This is generally true for children whose behavior and developmental problems fall within normal limits of individual variability. However, there is mounting evidence that serious problems persist, placing these children at risk for adjustment problems in elementary school and beyond (e.g., Campbell & Ewing, 1990; McGee, Partridge, Williams, & Silva, 1991). Although a wide variety of oppositional and aggressive behaviors occur during the course of normal development, very few preschoolers display these behaviors at extreme levels. When behavior problems occur across caregivers and with extreme frequency or intensity, they are unlikely to be transient and, when they occur, are indicative of serious and persistent conduct problems (Farrington, 1995; Lahey et al., 1995; Loeber, 1990). There are some serious conduct problems that are rarely exhibited by preschoolers (e.g., persistent stealing outside the home, using a weapon such as a knife) and these are clearly indicative of a conduct problem that requires intervention (Campbell, 1990, 2002). Other indicators that problems are likely to persist across the preschool years and into elementary school include significant conflict in the

parent-child relationship, ongoing family stress and disruption, and multiple problems of great intensity.

For children with serious conduct problems, intervention during the preschool years is critical. Untreated problems displayed by preschoolers tend to get worse over time, interfering with their development of self-help skills, socialization, and early academic skills. Also, therapy during the preschool years may be more effective than treatments initiated after age 7. There are several possible explanations for this. First, problem behaviors in preschoolers tend to be less well-ingrained than in older children who have longer learning histories. Second, intervening through the parents is much more potent with young children as they do not have many of the competing external influences (e.g., peers, school) experienced by older children. Third, young children have fewer cognitive resources for questioning and challenging behavioral interventions. Compared to older children, preschoolers are more accepting of new behavioral expectations, and less skeptical when parents suddenly begin providing large amounts of positive attention. Finally, very young children with significant conduct problems still exhibit affection toward their parents as well as cooperative behaviors which can be shaped to occur more frequently. After several years of behavior problems without effective intervention, older children display fewer of these positive qualities upon which to build. A strength of PCIT is that it is an early intervention model targeting high-risk children during the critical preschool years.

Targeting a Range of Behavior Problems. PCIT is an early intervention approach that may be applied to a broad range of behavior problems in young children (see Table 1.2). Generally speaking, PCIT is appropriate for young children demonstrating (1) externalizing problems such as non-compliance, defiance, verbal and physical aggression; (2) pre-conduct-disordered behaviors such as cruelty to animals, stealing, lying, and fire-setting; (3) inattention and over-activity; (4) internalizing problems such as sad affect, low self-esteem, and perfectionism; and (5) parent-child relationship problems in the context of divorce and adoption.

Table 1.2 Some presenting problems that may be addressed using PCIT

Non-compliance
Verbal and physical aggression
Over-activity
Cruelty to animals
Stealing
Lying
Fire-setting
Destructive behavior
Perfectionism
Low self-esteem
Sad mood
Bonding in blended families
Post-divorce adjustment
Whining

Specialized Space and Equipment. Standard PCIT is conducted with a particular type of space and equipment. The therapist typically coaches from an observation room through a one-way mirror using a hearing aid device while the family interacts in an adjoining playroom. The playroom should be large enough for at least four people (i.e., therapist, mom, dad, child) to sit comfortably. The room needs to be carefully childproofed, to minimize the potential for danger and the need for limit-setting. The room should not include furniture that can tip over, working sinks, sand tables, artwork, plants, venetian blind cords, computers, uncovered electrical outlets, floor lamps, swivel chairs, or toys that are inappropriate for PCIT (e.g., toy weapons, balls, bobo dolls). The ideal playroom is sparsely furnished, including only a table and chairs, has covered light switches, and contains constructional and creative toys. The playroom is typically linked to an observation room with a one-way mirror (using unbreakable glass), although some facilities without one-way mirrors use video monitoring in which the therapist coaches from another room while viewing a real-time video feed. A third room, ideally adjoining the first two, is used as a time-out back-up room. The perfect time-out back-up room is small (e.g., 5' × 5'), can be accessed by both the observation room and the playroom, has two small unbreakable windows in each door (so that both the parent and the therapist can see the child), is lighted and ventilated, contains no furniture or objects of any sort, and does not have electrical outlets or light switches. It is helpful for the playroom, observation room, and time-out back-up room to be constructed with extra sound proofing/insulation out of consideration for other nearby offices. Please see Fig. 1.1 for a sample schematic for how to configure a PCIT clinic.

The playroom must be equipped with a sound system that allows the therapist in the observation room to hear both the parent and child through a speaker. To coach unobtrusively, a behind-the-ear (BTE) receiver and microphone device is used. The best technology available is a small wireless hearing aid worn by the parent. The therapist talks to the parent from the observation room through a microphone and transmitter device. This wireless technology is available through SSL Industries, P.O. Box 3113, Diamond Springs, CA 95619, 530-644-0233, <http://www.sslinc.net/pcit.php>. The approximate purchase price of two BTE receivers (hearing aids) and a wireless transmitter is US \$2400. For more information about the total cost of setting up a PCIT program, see Goldfine, Wagner, Branstetter, and McNeil (2008).

We recognize that many community agencies do not have the resources to begin a PCIT program using the ideal set-up described above. Until resources can be garnered, there are less expensive ways to conduct the therapy. In place of a sophisticated speaker system, we have been successful using a baby monitor to provide sound from the playroom to an observation room. Also, when no observation window or video monitoring is available, it is possible to coach in the playroom with the family (see Chapter 23 for a description of in-room coaching). Walkie talkies connected by a wire to ear buds, wireless telephone head sets, and cell phones with blue tooth technology can be used in lieu of the more expensive but less cumbersome BTE and transmitter devices. Finally, if the ideal time-out back-up room described above cannot be constructed, it is possible to use a different room as a time-out area. This might include a conference room, office, or testing room that the

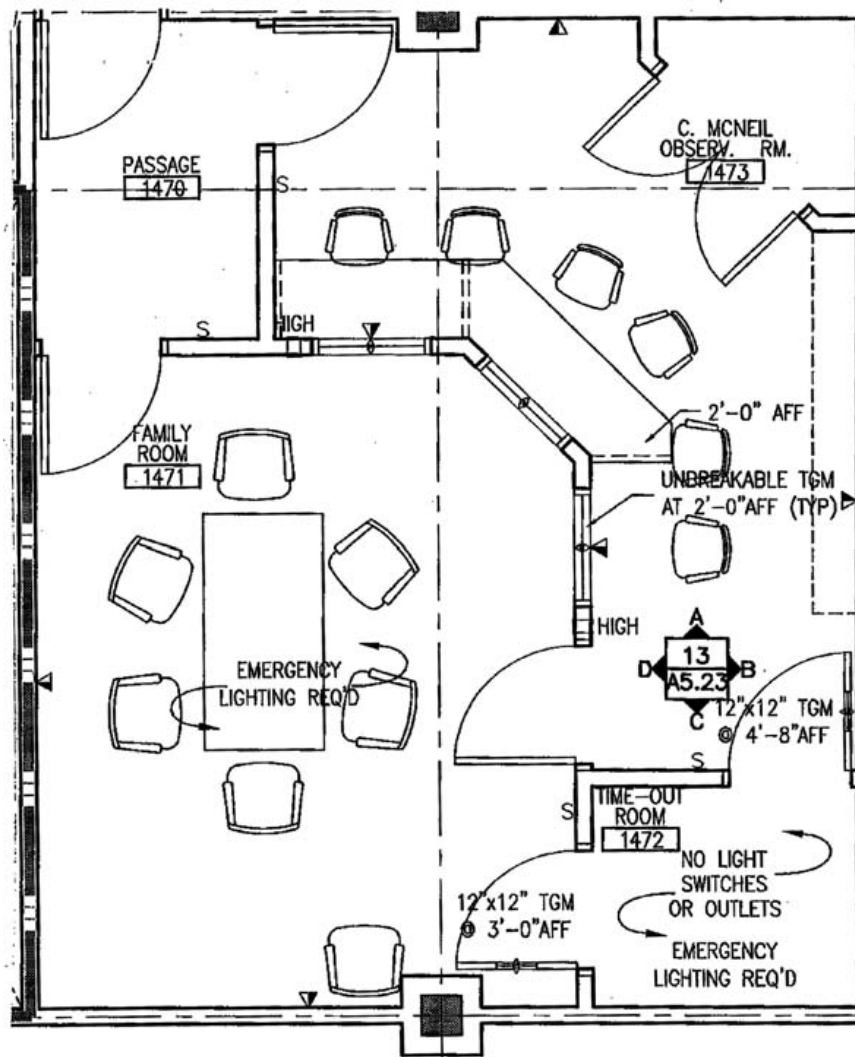


Fig. 1.1 Schematic of a PCIT clinic

therapist childproofs before the session. When seclusion is not permitted or accepted (e.g., in some agencies serving children with histories of abuse and neglect), a back-up room can be constructed using a half-door or Dutch door, allowing the child to be contained without being isolated.

Targeting Patterns of Interaction Rather than Discrete Behaviors. Unlike many behavioral parent training approaches that target discrete behaviors, PCIT focuses on changing broader patterns of interaction. A traditional behavior analytic approach might target one operationalized behavior at a time such as whining. The therapist would conduct a functional assessment to identify and change antecedents, consequences, and setting events that maintain the problem behavior. Treatment plans often involve reinforcing alternative behaviors using tangible reinforcers in a sticker chart type system. These systems work, but can be impractical for addressing the full range of behavior problems in a typical PCIT referral. We have never had a

child referred for PCIT who displays only one or two disruptive behaviors (e.g., a “spitting referral”). More typically, referred children are rated outside normal limits on twenty or more discrete problem behaviors. Imagine having to formulate twenty separate treatment plans for a host of behavior problems including spitting, hitting, whining, arguing, throwing tantrums, cussing, talking back, defying rules, screaming, breaking things, stealing, lying, writing on the walls, putting a slice of cheese in the DVD player, etc. In PCIT, we recognize that these behaviors serve two general functions for our clients. First, children engage in many of these behaviors to access attention and stimulation (e.g., whining to get a reaction from mother). Second, children engage in other misbehaviors to escape parent demands (e.g., arguing to avoid cleaning up). And many times, a particular behavior can simultaneously serve both the functions of getting attention and escaping a demand (e.g., temper tantrum when asked to go to bed). The two phases of PCIT address the two primary functions of misbehavior. In CDI, parents learn to use contingent attention, increasing pro-social behavior with attention and decreasing inappropriate behavior with ignoring. In PDI, we teach parents to provide consistent consequences for non-compliance such that children are not allowed to escape demands. Rather than attempting to modify a series of individual behavior problems, PCIT targets two broader patterns of dysfunctional behavior: negative attention-seeking and non-compliance. By reducing negative attention-seeking and non-compliance, PCIT more efficiently addresses the large number of behavior problems present in preschoolers with externalizing disorders.

Positive, Non-judgmental Philosophy. As PCIT therapists we use the same behavioral principles with parents that we teach parents to use with their children. Just as parents are taught to give their children specific labeled praise, we look for many opportunities in each session to use labeled praise with parents. As a general philosophy, we assume that parents are doing the best they know how and we convey respect for their efforts. We are careful to avoid judging and blaming parents. Instead, we accept them as they are and praise them for what they are doing correctly (e.g., caring enough to get help). Rather than communicating that we believe the parent to be deficient in their skills, we present PCIT as a method of training them to become expert behavior therapists. We avoid referring to our therapy as “parent training” because it implies that it is a remedial treatment for people who are failing as parents. Just like we teach parents to avoid criticism and give positively stated commands to their young children, we provide constructive feedback to parents, telling them what “to do,” instead of what “not to do.” This positive approach causes parents to feel good about coming to treatment and motivates them to work cooperatively with the therapist (see Chapter 3).

Using the Second Edition

This book is divided into two parts. Part one presents the fundamentals of PCIT as developed and practiced by Sheila Eyberg at the University of Florida. Dr. Eyberg (1999) has written a treatment manual that is composed of

session-by-session integrity checklists and other session materials. We strongly recommend that therapists wishing to conduct PCIT obtain and use Dr. Eyberg's manual. At present, it is available to be downloaded from Dr. Eyberg's website (www.pcit.org). Eyberg's treatment manual will be updated regularly. Therapists are expected to obtain and use the most recent version of the treatment manual. When discrepancies exist between the Eyberg treatment manual and this text, therapists should follow Dr. Eyberg's treatment integrity checklists. We recommend that therapists bring the treatment integrity checklists to each PCIT session, using them as an outline to insure adherence to all aspects of the protocol. Part One of our *Second Edition* provides a rich clinical description of how to implement the procedures outlined in Dr. Eyberg's integrity checklists. Part Two of our text, presents exciting, cutting-edge developments in the application of PCIT to special populations and settings. This section of the book includes advancements and exploratory work that has been conducted since the original text was published in 1995. In the last 14 years, the scope of PCIT has widened dramatically. PCIT was initially developed as a clinic-based program for disruptive 3-to 6-year-olds. Since the publication of our first PCIT book, PCIT is now being conducted in schools, group homes, shelters, private homes, and in mobile units. It is no longer conducted only with preschoolers, but has been used as a prevention model for infants and toddlers as well as an intervention for older elementary school-age children. Disruptive behaviors are not the only presenting problems now targeted by PCIT. It is now considered one of only two evidence-based interventions for physically abusive parents and has been disseminated widely by the National Child Traumatic Stress Network as an intervention for children with a history of trauma. Exciting work has been done demonstrating the potential for PCIT as an intervention for children with separation problems and other anxiety issues. And, PCIT is now being conducted with children who have ADHD, as well as those with developmental disorders, such as Mental Retardation and high-functioning Autism. Whereas we previously knew little about the cultural sensitivity of PCIT, recent trials have suggested that it is an acceptable intervention for U.S. children of Hispanic, Native American, and African-American backgrounds. The international community has shown interest in PCIT with programs now established in Europe, Asia, and Australia. It is our hope that this *Second Edition* will assist clinicians to conduct the standard, evidence-based PCIT with integrity, as well as inspire them to explore ways to make PCIT work with the variety of complex clinical scenarios therapists confront in their practices.

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Chapter 2

Research on PCIT

Stephanie Wagner

Early PCIT Research

Establishing an evidence-based treatment often involves single-subject studies, program evaluations, and randomized controlled trials. However, after research demonstrates the efficacy of a particular treatment through these methods, there are many additional areas to investigate before the intervention is disseminated and widely employed. PCIT, like other evidence-based interventions, has been developed in a similar way, beginning with early research demonstrating changes in disruptive behavior at post-treatment in comparison to waitlist children (e.g., McNeil, Capage, Bahl, & Blanc, 1999; Schuhmann, Foote, Eyberg, Boggs, & Algina, 1998). In addition to demonstrating efficacy, this research suggested additional benefits such as generalization to other settings (e.g., school) (McNeil, Eyberg, Eisenstadt, Newcomb, & Funderburk, 1991) and other individuals (e.g., untreated siblings) (Brestan, Eyberg, Boggs, & Algina, 1997). Furthermore, many of these beneficial results observed immediately after treatment were found to be maintained 1–6 years following PCIT (Boggs et al., 2004; Eyberg et al., 2001; Hood & Eyberg, 2003).

Herschell, Calzada, Eyberg, and McNeil (2002) reviewed the research on PCIT and offered suggestions for furthering the literature. Research was grouped into different categories including effectiveness, diagnostic variables, cultural variables, therapist variables, delivery of treatment, maintenance, and dissemination. The current review utilizes a similar organization to highlight progress in different lines of recent PCIT clinical research and also offers suggestions for future research endeavors. The goal of this chapter is to provide an update on PCIT research; and therefore, does not include a comprehensive review of the early PCIT literature.

Recent Research Initiatives

Diagnostic Groups. Since the review conducted by Herschell and colleagues (2002), examining the effects of PCIT with new diagnostic groups has received considerable

empirical focus. At the time of the review, Herschell and colleagues emphasized that the theoretical principles underlying PCIT apply to many different childhood disorders; however, limited literature existed on PCIT with diagnostic groups other than disruptive behavior disorders. Further work has been conducted on the use of PCIT with internalizing populations. In particular, Choate, Pincus, Eyberg, and Barlow (2005) examined standard PCIT in three children who met criteria for Separation Anxiety Disorder (SAD) and found that these children did not meet diagnostic criteria for SAD following treatment. Other research suggests that PCIT benefits both internalizing and externalizing symptoms in children with co-morbid SAD and Oppositional Defiant Disorder (ODD) (Chase & Eyberg, 2008).

The principles underlying PCIT also conceptually apply to families of children with developmental disabilities such as mental retardation and autism (Masse, McNeil, Wagner, & Chorney, 2008; McDiarmid & Bagner, 2005). Therefore, researchers have tested this intervention with children who are developmentally and cognitively delayed. Bagner and Eyberg (2007) randomly assigned children with mental retardation to PCIT or a waitlist control group and detected improvements in child disruptive behavior and parenting skills in the PCIT group. Furthermore, a recently completed study by Josh Masse and Cheryl McNeil at West Virginia University demonstrated the efficacy of PCIT for improving compliance in children with autism (Masse, McNeil, & Wagner, 2009).

Children with chronic illness or medical problems represent another population that has received limited focus in the PCIT literature. Bagner, Fernandez, and Eyberg (2004) reported positive child and parent behavioral change in a case study of a young child with cancer and ODD. PCIT was tailored slightly in this case so that the parents received help providing positive reinforcement for adaptive medical behaviors. Further research is needed to better understand the effectiveness of PCIT in pediatric settings for children with various medical conditions.

Cultural Groups. Additional research has specifically looked at PCIT outcomes with different cultural groups to help understand how to maximize treatment efficacy while maintaining cultural sensitivity (Butler & Eyberg, 2006). Borrego, Anhalt, Terao, Vargas, and Urquiza (2006) translated PCIT materials into Spanish and documented a successful case involving a Mexican family. Similar positive findings were found in a study translating, implementing, and evaluating PCIT in Puerto Rico (Matos, Torres, Santiago, Jurado, & Rodriguez, 2006) and in a modified version of PCIT for Mexican-American families termed Guiding Active Children (GANA Program; McCabe, Yeh, Garland, Lau, & Chavez, 2005). Based on these preliminary findings, it appears that many treatment components are acceptable to a range of parents and that this intervention may be efficacious with different Hispanic cultural groups.

Another culture that has been qualitatively and quantitatively studied in PCIT research is Chinese families in Hong Kong (Tsang, Leung, Chan, & Choi, 2007). Study findings indicated that PCIT led to significant decreases in child disruptive behavior and increases in effective parenting skills in Chinese families of children with behavior problems. Generally, these families were accepting treatment, although some skills such as praise may be less congruent with parenting practices in Hong Kong.

The research and utilization of PCIT in Hispanic and Chinese cultures suggest that it is a promising intervention for families located in many different regions and with various cultural backgrounds. However, it is likely that PCIT will need to be tailored slightly for widespread use with these cultures (See Chapter 19 and 24). For example, both Hispanic and Chinese cultures may benefit from systematic efforts by PCIT therapists to involve extended family members in treatment (i.e., Matos et al., 2006; Tsang et al., 2007).

Other Parent Characteristics. Early PCIT research often examined parent and child behavior changes after treating the *mother*–child dyad, which is consistent with methods commonly employed in the behavioral parent training literature (Tiano & McNeil, 2005). However, there is increased recognition of the need to include other individuals, particularly fathers, in research and clinical practice (Tiano & McNeil). PCIT researchers have attempted to extend research findings to other parental characteristics by investigating not only fathers, but physically abusive parents, parents who have experienced intimate partner violence (IPV), and foster parents.

An examination of family composition in several PCIT treatment outcome studies found different effects in families with involved fathers, uninvolved fathers, and absent fathers (Bagner & Eyberg, 2003). Although these researchers did not find many group differences at post-treatment, children in families with an involved father exhibited greater maintenance of treatment gains compared to other families. This study highlights the importance of engaging fathers in PCIT and conducting further work on treatment outcome and maintenance with involved fathers.

Recently, many PCIT researchers have been interested in utilizing this intervention with abusive parents. Since Urquiza and McNeil (1996) discussed how the principles underlying PCIT and the format of the intervention could apply to maltreating parents, empirical research has confirmed the usefulness of treatment in this population. For instance, Chaffin et al. (2004) compared standard PCIT, enhanced PCIT, and standard parenting program in groups of physically abusive parents. Results demonstrated that there were fewer drop-outs in the PCIT groups and that standard PCIT significantly reduced the reoccurrence of physical abuse in comparison to the other groups (Chaffin et al.). Also, Timmer, Urquiza, Zebell, and McGrath (2005) compared PCIT outcomes in families with and without maltreating caregivers, finding that PCIT decreased child behavior problems, parenting stress, and risk for abuse in families in both groups.

A characteristic of many abusive families is the presence of IPV. Children exposed to IPV often have internalizing and externalizing behaviors as well as problems in the bond with their parent (See Chapter 13). Therefore, theoretically, PCIT will likely be an efficacious intervention for children exposed to IPV (see Borrego, Gutow, Reicher, & Barker, 2008). To date, controlled research has not been used to directly address PCIT's effects with this population, however, a published case study demonstrated success following PCIT (Pearl, 2008) and research is underway at West Virginia University comparing PCIT versus treatment as usual (Foley & McNeil, 2009).

In addition to examining the effects of PCIT on maltreating caregivers and children exposed to IPV, research has also focused on maltreated children in foster care (McNeil, Herschell, Gurwitch, & Clemens-Mowrer, 2005). Several researchers have published successful cases of PCIT with children placed in foster care (Fricker, Ruggiero, & Smith, 2003; Timmer, Urquiza, Herschell, et al., 2006). Also, Timmer, Urquiza, and Zebell (2006) conducted a more rigorous test of PCIT with foster families by comparing the effects of PCIT in foster parents and non-abusive biological parents with findings suggesting that the treatment is equally effective for the two groups.

Therapist Characteristics and Behavior. Compared to other areas of treatment development and investigation, less PCIT literature exists on therapist behaviors that enhance or hinder outcome. However, specific research on the interaction between the therapist and the client or therapy-process variables is imperative. Research has established that therapy-process variables are important for successful completion and outcome following intervention for child psychopathology (e.g., Kazdin, Holland, & Crowley, 1997; Shirk & Karver, 2003).

Harwood and Eyberg (2004) examined the relation between therapist behavior early in PCIT and treatment completion, which was defined as meeting the PCIT mastery criteria. These researchers found that therapist verbalizations (e.g., therapist support, questioning, and facilitation) during the assessment and first CDI coaching session predicted treatment drop-out. These findings are consistent with other research showing the importance of using skills to engage clients (Herschell, Capage, Bahl, & McNeil, 2008); however, more research on therapist–client interaction in PCIT is warranted to better understand the effects of therapeutic alliance on adherence and outcome.

Intervention Format. Considerable advances have been made in the area of the PCIT format including research on age range, group treatment, number of sessions, location of intervention (i.e., home-based services and classroom), and intensity of intervention. Although these innovations are still in the preliminary stages of design, implementation, and research, they may be promising adaptations that promote widespread use in a variety of contexts, while at the same time preserving many core components of treatment.

Parent-Child Attunement Therapy (PCAT) employs many behavioral techniques utilized in PCIT but was specifically designed to address needs and concerns of younger children (ages 12–30 months) (Dombrowski, Timmer, Blacker, & Urquiza, 2005). Differences between PCAT and PCIT include: (1) PCAT places a greater emphasis on physical praise (hugs) and enthusiasm, (2) PCAT does not include a phase comparable to PDI, and (3) PCAT is typically conducted in shorter sessions. Dombrowski and colleagues reported successful treatment with a mother and her 23-month-old child. Just as PCAT has been developed for younger children, there also have been attempts to extend PCIT's age range to older children (Chaffin et al., 2004). See Chapter 9 and 10 for information regarding the adaptation of PCIT for children outside of the 2–7 age range.

Other research has examined more subtle changes to traditional PCIT. In particular, Nixon, Sweeney, Erickson, and Touyz (2003) developed an abbreviated version

of PCIT that incorporated didactic videotapes and telephone consultation. A comparison of standard PCIT, the abbreviated version which consisted of five sessions and five consultations, and a waitlist comparison group demonstrated that both treatment groups significantly improved and maintained behavioral change. Although this change appears relatively minor, the abbreviated format did not contain criteria for mastery, which is a critical feature of PCIT. Creativity in adapting traditional PCIT may be necessary in order to incorporate key components in altered versions, such as abbreviated treatment.

Another format variation that also faces the problem of integrating mastery criteria into the treatment is an intensive, brief PCIT workshop. Although both standard and abbreviated PCIT have clinical utility in treating children referred for disruptive behavior disorders, providing less intensive services may be an appropriate and sufficient method of prevention. McNeil et al. (2005) reported findings from a 2-day workshop that incorporated many of the core elements of PCIT with the exception of skill mastery (i.e., didactic and coaching). Findings indicated that foster parents were satisfied with the intervention and reported significant positive changes in child disruptive behavior following the workshop. Additional treatment adaptations and subsequent research would inform researchers and policy-makers as to whether PCIT can be an effective and efficient prevention program for populations of children at risk for developing behavior problems. Given the current structure of PCIT, which involves considerable time for families to learn and demonstrate skills, this treatment may not be well suited for prevention. However, it is possible that with changes that preserve core components (i.e., coaching, feedback, data-driven progression), PCIT in a shorter format has the potential of being an effective method of prevention.

Another exciting PCIT adaptation involves developing protocols for group treatment. Group treatment has several advantages over individual treatment including being more efficient and cost-effective to implement and providing the opportunity of increased social support (Niec, Hemme, Yopp, & Brestan, 2005). However, the coaching and mastery criteria in PCIT pose unique challenges to providing this treatment in group format (Niec et al.). Investigators interested in group treatment are still devising and empirically testing different mastery requirements.

Given the efficacy of PCIT in traditional clinic settings, research has begun to test the effects of PCIT conducted in other settings. Settings such as the home or schools may help reduce some common barriers to treatment such as transportation or availability (Ware, McNeil, Masse, & Stevens, 2008). Ware and colleagues examined the effects of PCIT provided in the home on parenting behavior and child behavior problems. Although study results suggest that home-based PCIT is efficacious, unique challenges are present in home-based interventions often stemming from the lack of environmental control (Masse & McNeil, 2008).

A version of PCIT has been designed for school settings termed Teacher–Child Interaction Therapy (TCIT). A case study demonstrated that teachers increased the frequency of positive skills utilized as well as decreased child disruptive behaviors and increased child compliance following TCIT (McIntosh, Rizza, & Bliss, 2000). Additionally, comparison of head start classrooms receiving training in TCIT

skills and comparison classrooms indicated that TCIT- trained teachers utilized the skills and child disruptive behaviors decreased (Tiano & McNeil, 2006). Budd et al. (2007) employed a multiple baseline design to examine the effectiveness of TCIT in a low-income, ethnically diverse daycare, finding some improvements in the majority of teachers' behavior immediately following treatment. However, these gains diminished at a 4-month follow-up. Therefore, classroom-modified versions of PCIT likely need refinement to enhance treatment outcome and maintenance.

Attrition. Drop-out from psychotherapy is a serious problem when working with many populations and utilizing a variety of therapies (Wierzbicki & Pekarik, 1993). Parent training interventions are also subject to high rates of attrition (e.g., Kazdin, Mazurick, & Siegel, 1994; Prinz & Miller, 1994). Early termination of treatment is a concern in PCIT and research documents attrition rates ranging from 0–53% (Gallagher, 2003). Attrition is particularly concerning given that families who terminate early do not improve over time from their pre-treatment level of functioning (Boggs et al., 2004). Therefore, recent work has been conducted to identify variables predictive of drop-out (Werba, Eyberg, Boggs, & Algina, 2006). Werba and colleagues found that waitlist status predicts drop-out prior to starting treatment and that parenting stress and inappropriate parenting behaviors (parental sarcasm and criticism) predict drop-out in families who have begun PCIT. Although drop-out from waitlists has implications for clinical research, understanding drop-out in families who have begun treatment has enormous applied value. In particular, even though Werba and colleagues were able to identify several variables predicting drop-out, many factors contributing to early termination of PCIT remain unclear. Additionally, Chaffin and colleagues have been examining the effectiveness of a motivational enhancement add-on for decreasing attrition with clients with a history of child maltreatment (Chaffin et al., 2009).

Effectiveness/Dissemination Research. A major challenge for treatment outcome researchers, including PCIT researchers, is to replicate findings observed in university clinics in community settings. Many hurdles need to be overcome when attempting to transport an evidence-based treatment, including therapist motivation, training, and caseloads, as well as agency and community resources (Franco, Soler, & McBride, 2005; Sukumar, Johnson, McNeil, Brooks, & Manteuffel, 2008). Maintaining and measuring treatment integrity and fidelity is another challenge in community settings (Franco et al.). However, despite these obstacles, effectively disseminating PCIT has the potential to help more families and prevent future disruptive behavior problems in young children. Pade, Taube, Aalborg, and Reiser (2006) implemented a modified version of PCIT in a community setting, finding positive short-term and long-term treatment effects. However, the majority of families enrolled in the study utilized other interventions between the time that they completed PCIT and when they were contacted for follow-up (Pade et al.). Therefore, it is unclear whether functioning measured at follow-up occurred as a result of PCIT or another intervention. More work is needed to address implementation issues and investigate short- and long-term effectiveness in community settings.

Training. Successful dissemination cannot occur in the absence of training competent PCIT therapists. There is a paucity of research on training PCIT clinicians; however, findings indicate that neither solely reading the PCIT manual nor attending a 2-day didactic training workshop is sufficient preparation to facilitate therapist mastery of the PCIT skills (Herschell et al., 2009). Current recommendations set forth by the PCIT National Advisory Board include 40 h of initial training with advanced training and supervision (Herschell & McNeil, 2007; See Chapter 25). However, more research on training is warranted to develop efficient and effective training methods.

Cost-Effectiveness. Evaluating the cost-benefits and cost-effectiveness of specific interventions is imperative in order to convincingly argue for widespread treatment dissemination. Given the efficacy and new developments in PCIT, it is not surprising that several cost-effectiveness analyses have been conducted (i.e., Aos, Lieb, Mayfield, Miller, & Pennucci, 2004; Goldfine, Wagner, Branstetter, & McNeil, 2008; Krivelyova, Sukumar, Stephens, & Freeman, 2007). These examinations calculated similar cost of treatment per child; specifically costs ranged from \$1,025 to \$1,296 (Aos et al., Goldfine et al.). Furthermore, Krivelyova and colleagues found that the cost of PCIT was \$600 less per child than providing services as usual in a system of care program. These costs are all substantially less than the cost to society for persistent disruptive behavior throughout life and the incarceration of individuals whose problems progress into Antisocial Personality Disorder (Goldfine et al.). These favorable monetary results are certainly promising; however, additional cost examinations are warranted.

Suggestions for Future Research

In recent years, many exciting areas have been investigated in PCIT expanding the knowledge in regard to what types of clients benefit from PCIT and how the treatment can be packaged and delivered. Questions regarding what obstacles hinder treatment dissemination and whether the treatment is cost-effective have been posed by investigators. Research is beginning to provide insight into some of these inquiries. Specifically, findings suggest that this treatment is beneficial for many young children including children with a variety of concerns and problems such as mental retardation, developmental disorders, internalizing problems, and chronic illness. Studies also suggest that the treatment is appropriate for families with different cultural backgrounds and parents who are physically abusive. Research has shown support for treatment delivered in an abbreviated format, a workshop format, in classroom settings, and in the home environment. Additionally, the cost of treatment is quite low in comparison to the cost of untreated disruptive behavior disorders that persists throughout life and result in criminal behavior.

Despite these advances in PCIT research, there are many additional questions in need of investigation. Furthering the evidence base with regard to training and dissemination has the potential to benefit many families. However, current models

of training and dissemination are time-intensive and do not result in the same effects in the community as are obtained in university clinics. Therefore, it is evident that more work needs to be done to promote the widespread use and effectiveness of PCIT.

In order to disseminate successfully, additional research is necessary in many of the areas highlighted in this review (e.g., diagnostic, cultural groups, format, and training of clinicians). For instance, clearly specifying which diagnostic, cultural, and age groups the intervention works for helps clinicians select the most appropriate intervention for individual clients. It is likely that PCIT will not be the optimal intervention with certain populations (e.g., teenagers); more research is needed to guide clinical decisions on which families are likely to benefit from treatment.

Although there is a necessity for further research on PCIT with different groups, conducting format and training research is also of utmost importance. This research has the potential to both enhance community effectiveness and maximize cost-effectiveness. Therefore, this area of scientific inquiry is of particular interest to policy-makers.

One format option warranting further empirical scrutiny is the efficacy of in-room coaching. Traditional PCIT utilizes coaching from behind a one-way mirror with bug-in-the ear technology. However, this equipment and costs to modify the structure of a therapy room are fairly expensive (with rough estimates of \$14,000 to start-up PCIT) with the bulk of these costs consisting of technology and therapist training (Goldfine et al., *in press*). Due to the high price tag, standard coaching may not be feasible in the real world (e.g., Sukumar et al., 2008). Although some research suggests in-room coaching results in favorable outcomes (Rayfield & Sobel, 2000; Ware, McNeil, Masse, & Stevens, 2008), there is limited research comparing the two formats.

In addition to evaluating in-room coaching, there is a need for research on the length of PCIT. Specifically, several abbreviated or shortened adaptations of PCIT (e.g., Franco et al., 2005; Nixon et al., 2003) appear promising. However, many of these adaptations do not contain one of the integral components of PCIT, which is data-driven progress through treatment. More research is needed examining whether data-driven progression through treatment can be adapted for shortened formats and whether these formats have similar outcomes and adherence.

Another area lacking in research that has implications for treatment length and cost-effectiveness is component research. The need to evaluate individual components is most obvious in treatments that include many different elements (e.g., school interventions, parent training, social skills training, medication, and problem-solving training) that are often referred to as “kitchen-sink treatments” (Hoza, Kaiser, & Hurt, 2007). Although PCIT is not considered a multimodal treatment, the therapy does utilize many skills, some of which have not been subject to scientific scrutiny. Early PCIT research examined the order of treatment phases by comparing CDI-first and PDI first (Eisenstadt [Hembree-Kigin], Eyberg, McNeil, Newcomb, & Funderburk, 1993) and methods of training children to stay in time-out (McNeil, Clemens-Mowrer, Gurwitsch, & Funderburk, 1994). Recent examination of change during CDI demonstrated that significant positive changes in parenting stress, parenting practices, and the child’s disruptive

behavior occur during this relationship-enhancing phase of treatment (Harwood & Eyberg, 2006). However, some of the skills taught in PCIT are based largely on theory and clinical observations such as parental use of reflections and behavioral descriptions. Currently, ongoing projects are experimentally examining the effects of these components. Obtaining empirical evidence on the utility of individual skills will aid researchers in attempts to maximize efficacy and cost-effectiveness of PCIT.

Even though discovering how to maximize costs and benefits by researching different formats and adaptations is a necessary step to successful dissemination, therapist training also is a critical element in widespread use of PCIT. Therapist training also significantly contributes to PCIT start-up costs (Goldfine et al., *in press*). Currently, there is much debate regarding training, yet little evidence to guide decisions about training requirements. The research that does exist and requirements of different trainers suggest that intensive training incorporating didactic and experiential components with follow-up supervision is necessary (Herschell et al., 2009). However, more research is warranted comparing different training formats. For instance, it is possible that combining education on the behavioral principles underlying PCIT with technical training on the PCIT manual would enhance clinicians' understanding and mastery of PCIT. Given the concerns and controversies surrounding treatment manuals, this represents a relevant area to pursue scientifically.

Research in these areas is necessary to determine what treatment elements are mandatory for positive and clinically significant outcomes. Development in these areas could be used in a "bare-bones" empirically supported version of PCIT (Goldfine et al., 2008), which could be useful for community agencies with limited budgets. Increasing efficiency and reducing cost are key steps in bridging the gap between research and clinical practice.

Additionally, there is a need for researchers to think "outside of the box" to design and test models of delivery. For instance, some researchers have incorporated athletic training and sports skills in behavior parent training in attempts to engage fathers (e.g., COACHES; Fabiano, 2007). Developing novel formats may also enhance widespread PCIT prevention programs. Specifically, the Triple P Positive Parenting Program utilizes different levels of intervention depending on family needs with the lowest level of intervention implemented at a population-level through the media (e.g., mailings, articles about positive parenting, popular television shows about parenting, radio advertisements) (Prinz & Sanders, 2007). Utilizing the media as a tool in PCIT dissemination may aid prevention and intervention efforts.

Lastly, novel delivery methods may help enhance cost-effectiveness and reduce attrition. It is plausible that the once-a-week session schedule is not flexible enough for both clients and treatment providers. Urban environments may be able to capitalize on this need by offering PCIT walk-in clinics. For instance, treatment centers could offer different sessions (CDI didactic, PDI didactic, CDI coaching, etc.) at several different days and times during the week. Additionally, attrition may be reduced by adding incentives for parents and children to attend sessions (e.g., gift certificates, toys). This would enable families to pick the time that works for their

schedule based on what session they needed to attend. A method of tracking family progress such as providing certificates of session completion and mastery requirements would preserve key features while allowing this increased flexibility. Another potential advantage to this approach is that therapists could serve a greater number of clients and would not suffer from costs associated with families who do not attend scheduled appointments.

These research suggestions do not fully encompass every area in which PCIT research can be furthered, and these suggestions also do not guarantee solutions to problems of dissemination and drop-out. However, the ideas presented are intended to spur interest, discussion, and research. Since PCIT was developed in the 1970 s, this intervention has been empirically validated, extended to many new populations, and adapted to better fit unique needs (e.g., TCIT in school settings). All of these findings suggest that PCIT has general utility. Therefore, the expansion of PCIT is as limitless as the innovation and creativity of clinicians and researchers.

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Chapter 3

Intake Assessment and Therapy Orientation Session

What to Bring. . .

- (1) Your Agency's Consent to Treat Form
- (2) HIPAA Form
- (3) Your Agency's Release of Information Form
- (4) Your Agency's Cancellation Policy
- (5) A Structured Intake/History Form of Your Choosing
- (6) Eyberg Child Behavior Inventory
- (7) Sutter-Eyberg Student Behavior Inventory
- (8) Dyadic Parent-Child Interaction Coding System Form
- (9) CDI Homework Sheets
- (10) Sheila Eyberg's Treatment Integrity Checklists/Manual

With new case referrals, we always begin PCIT with an evaluation of child and family functioning. We think this is important for several reasons. First, PCIT is not a “cookbook” approach to child treatment in which therapy procedures are the same with all families regardless of presenting problems. Instead, the particular emphasis and treatment components are tailored to meet the special needs of each family, and the way that we learn about those special needs is through our initial evaluation. The initial evaluation can also elucidate factors that may interfere with treatment progress so that they can be addressed prior to or concurrent with PCIT.

The second reason we begin with an evaluation is that the results of our testing serve as a baseline measure of child behavior and parenting skills against which we judge the family's progress during the course of treatment. We code parent-child interactions and collect rating scale data in every coaching session so that we can evaluate weekly changes in both the parent and the child. If we consistently do not see improvements over our baseline ratings, we consider this a “red flag” for us to stop and assess why treatment is not progressing as expected. Sometimes adjustments need to be made in our teaching strategies. At other times, we find that we missed a key piece of information about family functioning at the time of our initial evaluation and that this particular factor (e.g., substance abuse, severe parental depression) is interfering with treatment progress.

The third reason we believe it is important to begin with an evaluation is accountability. Clinical judgments concerning treatment effectiveness, no matter how strongly held, are subject to many biases and should not be the exclusive source of information about treatment results. Also, in this climate of managed healthcare, mental health professionals are expected to objectively document the effectiveness of services rendered in order to obtain third party reimbursement.

Who Should Attend the Intake Session?

There are several issues to consider in deciding whether this should be a single session with the child in attendance, or whether it is broken down into two sessions with the child attending only the second session. Obviously, coding of parent-child interactions cannot occur without the child, and it is important for the clinician to meet and observe the child before initiating treatment. However, it can be counter-productive to have the child present during the intake interview. Parents often unleash a barrage of complaints and criticism about their child and can be tearful as they express hopelessness and guilt. Also, they are prompted to discuss sensitive topics such as whether it was a planned pregnancy, parental substance use, non-custodial parents, sexual abuse, family trauma, and adoption issues. It would be highly inappropriate to discuss such topics in the child's presence.

It may be possible to conduct the assessment in one session if another caregiver is available to watch the child during the interview. This caregiver could be a grandparent or other relative, co-therapist, student assistant, or office receptionist. If no such person is available, the child may be invited to play on his own in the playroom while the adults conduct the interview in the observation room. That way, the child can be monitored through the one-way mirror without being exposed to sensitive intake information.

Many therapists prefer to break the assessment and therapy orientation session into two parts because of its length. It often takes over 2 h to accomplish all of the goals of this session including completing necessary paperwork, conducting a thorough interview, completing rating scales, coding parent-child interactions, providing feedback on measures, and orienting the family to PCIT. For billing purposes, it may be difficult to obtain reimbursement for more than 1 h of service on a single day. Agencies may also have internal guidelines limiting the length of sessions to allow for a larger caseload. Finally, some of our acting-out child clients may become extremely disruptive over a long clinic visit.

When the child is present at the assessment session, it is important for the therapist to begin developing rapport with the child. The therapist should show respect for the child by providing a developmentally appropriate explanation of who we are, why the child is there, and what will happen. We have been shocked by some of the misperceptions children have shared with us during this first session. We have had young clients express fear that we would give them shots. After one boy observed his infant sister having blood drawn from her heel the previous day, he asked at the intake appointment, "Are you the kind of doctor that hurts babies?" Another girl was

told by her mother that she was being picked up from school to go shopping. Upon presenting at the clinic, the kindergartener said, “You tricked me. That was mean!” From the first contact, we use our CDI skills to form a positive relationship with the child. We find things to praise them for such as their attire, episodes of good behavior, bright smile, creative ideas in play, and polite manners. We reflect back when they speak to us and we describe their appropriate behavior. To help parents view their children in a more positive light, we point out the children’s appealing qualities including sense of humor, physical attractiveness, great vocabulary, athleticism, and strong fine motor skills.

Flexible Battery Approach

We recommend using the core set of assessment procedures outlined in Table 3.1. We use a semi-structured intake interview to collect information regarding the child and family’s history and current functioning. Even though we have conducted thousands of intake interviews, we still bring an intake questionnaire to the session to ensure that we do not omit important information. The Eyberg Child Behavior Inventory (ECBI) is an essential measure for determining whether the child’s behavior is in the clinical range and to track behavior change over time. Similarly, for children in preschool/daycare and early elementary school, the Sutter-Eyberg Student Behavior Inventory (SESBI), completed by the teacher, serves as a measure of behavior in the school setting. In addition to the parent and teacher report, we use the Dyadic Parent-Child Interaction Coding System – Third Edition (DPICS – III) as an observational measure of parent and child behavior. These measures serve as our core pre-treatment assessment and are repeated at the conclusion of treatment.

Table 3.1 Parent-child interaction therapy core assessment procedures

Semi-structured intake interview (approx. 45 min)
Eyberg child behavior inventory (5–10 min)
Sutter-Eyberg student behavior inventory (if child is in school; 5–10 min)
Dyadic parent-child interaction coding system observation (allow 15 min for each parent)

Although we always administer the measures in Table 3.1, we do use a flexible battery approach, including additional measures of child, parent, and family functioning depending upon the needs of the individual family. Many young children referred for PCIT have co-morbid conditions that need to be evaluated. These conditions include mental retardation, autism, anxiety, depression, and hyperactivity. Similarly, many parents have mental health issues that may interfere with treatment success. It is helpful to know from the outset if there is significant marital conflict (including domestic violence), and/or problems with depression, anxiety, substance abuse, stress, or limited cognitive functioning. See Table 3.2 for a listing of supplemental areas of assessment.

Table 3.2 Supplemental areas of assessment

Child functioning
Emotions (e.g., anxiety, depression, bipolar)
Self-esteem
Cognitive ability
Academic achievement
Adaptive behavior
Social skills
Development (e.g., autism spectrum)
Attention, activity level, & impulse control
Parent functioning
Parenting stress
Depression
Marital discord
Anxiety
Cognitive ability
Substance abuse

Semi-structured Intake Interview

After completing intake paperwork (i.e., consent forms, release of information forms, HIPAA forms, other agency-specific forms), we cover the limits of confidentiality (e.g., child abuse and neglect, duty to warn, subpoena for records). Our evaluation begins with a semi-structured intake interview. A variety of semi-structured interview formats are available that are suitable for use in PCIT; we use one that is similar in format to the one published by Barkley (1990, pp. 262–277). For a listing of the elements covered in a thorough intake interview, please see Table 3.3. When conducting an interview with a couple, we set the expectation for equal involvement by both parents and prompt the less vocal member of the pair to share his or her observations and concerns.

In addition to the questions asked in a typical semi-structured intake, we solicit information more specific to PCIT concerns. During the course of the interview, we collect detailed information concerning the family's experiences using time out. Most families referred to us for disruptive child behavior have used time out and found it ineffective. On inquiry, we can nearly always identify problems in the way they were using time out, which compromised its effectiveness. It is important to collect this information up front during the intake interview. Later, when time out is introduced as a discipline method, the therapist will be able to reassure parents that the new form of time out is different from the unsuccessful one they used previously. Given the focus in PCIT on improving child compliance, we ask parents to estimate for us what percentage of time their children comply with the first command given. A simple way to elicit this information is to ask: "If you give your child ten simple things to do throughout the day such as 'go get your shoes,' 'put the sock in the hamper,' or 'brush your teeth,' how many would she do right away without you having to repeat yourself?" We know that typical children who are not referred to treatment comply with an average of approximately 60% of commands first time

Table 3.3 Elements of the semi-structured intake interview

Prenatal history
Birth information
Infant temperament
Developmental milestones
Medical history (e.g., conditions, accidents, injuries, hospitalizations, surgeries)
Genetic history
Sleep problems
Eating problems
Toileting issues
Abuse history
School experiences
Peer relationships
Prior family mental health treatment
Stressful life events
Sibling interactions
Marital history
Primary concerns at home
Discipline strategies used
Child's strengths/interests

given. Referred parents typically report that their children comply at a rate of less than 30%.

Other questions that might be a part of a PCIT intake include the following: (a) What are your child's most challenging behaviors? (b) What are your child's most endearing behaviors? (c) What do you like about your parenting? and (d) What would you like to change about your parenting? (Eyberg, 1999).

Eyberg Child Behavior Inventory (ECBI)

The Eyberg Child Behavior Inventory (Eyberg & Pincus, 1999; Eyberg & Ross, 1978) is an empirically supported, brief parent-report measure of child behavior problems that is appropriate for use with children between the ages of 2 and 16. The Intensity Score provides an estimate of how frequently the child displays each of 36 problem behaviors, and the Problem Score allows the parent to rate whether or not he or she perceives the behavior to be "a problem" for the child in question. The clinical cut-off scores are 131 (60T or higher) for the Intensity Score and 15 (60T or higher) for the Problem Score (Eyberg & Pincus, 1999). We administer the ECBI at pre- and post-treatment as well as at the start of every session as a way of measuring treatment progress.

The ECBI is particularly useful in identifying parents who may have inappropriately high and inappropriately low expectations concerning their children's behavior. For example, one mother we worked with reported that she considered twenty of thirty six behaviors to be problems. However the frequency with which the

child displayed these behaviors was well within normal limits, suggesting that this mother may have had little tolerance for the normal but sometimes irritating behaviors displayed by young children. In contrast, we worked with an overly tolerant father who reported that his son frequently engaged in a large number of problem behaviors, but he indicated that he considered none of them to be problems. He expressed the attitude that “boys will be boys,” and he felt that his wife and his son’s preschool teacher were overreacting to his son’s disruptive behavior. The Problem Score tends to be higher in single-parent versus intact families and high Problem Scores have been associated with marital distress (Eyberg, 1992).

Sutter-Eyberg Student Behavior Inventory – Revised (SESBI – R)

The format of the Sutter-Eyberg Student Behavior Inventory – Revised (SESBI – R; Eyberg & Pincus, 1999; Funderburk & Eyberg, 1989) is similar to that of the ECBI. However, the items have been adjusted to be more appropriate for the classroom setting and the measure is to be completed by the child’s daycare, preschool, or elementary school teacher. Intensity Scores of 151 (60T) or greater and Problem Scores of 19 (60T) or greater are in the clinical range (Eyberg & Pincus, 1999). The SESBI – R distinguishes between preschoolers referred for school behavior problems and non-referred preschoolers, and has been found to be sensitive to improvements in school behavior following PCIT. The teachers we work with especially appreciate the brevity of the SESBI – R, as it can be completed in approximately 5 min. For children in schools or daycares, we administer the SESBI – R at both pre- and post-treatment.

Dyadic Parent-Child Interaction Coding System – III (DPICS – III)

One of the fundamental elements of our pre-treatment evaluation is direct observation of parent-child interactions. We conduct informal observations in which we note how the dyad interacts in the waiting area and while parents complete assessment procedures. In our informal observations we look for the child’s ability to play independently, strategies the child uses to engage the parent’s attention, parental responsiveness to child overtures, parental limit setting, warmth of parent-child interactions, and evidence of clinginess and separation anxiety.

Children sometimes become quite disruptive during the informal observations. Therapists should avoid the temptation to intervene, unless of course there is imminent danger. As the child has not yet received treatment, the therapist’s attempts to get the child to obey or calm down are unlikely to be effective. Instead, the therapist may appear incompetent to the parents, who probably would perceive that the therapist has no better control over the child than anyone else. The best approach is to instruct the parents to manage the behavior as best they can. For example, if

the child runs out of the door, the therapist should ask the parents to retrieve him or her. If the child is throwing toys, the therapist should indicate concern and ask the parents to try to stop the throwing. When parents ask for advice, the therapist can support their desire to learn, but help them realize that the skills are complex. Their questions will be answered in future sessions if they can be patient for a little while longer.

More formally, we conduct a structured observation using the Dyadic Parent-Child Interaction Coding System – III (DPICS – III; Eyberg, Nelson, Duke, & Boggs, 2005). The observation is done in a clinic playroom using a small table, two chairs, a time-out chair (placed in the corner of the room), a large toy box, and five sets of toys (e.g., blocks, Legos, Duplos, Lincoln Logs, Tinkertoys, Mr. Potato Head, and building blocks, puzzles, toy farms). Two of the toys are placed on the table and the others are distributed across the remaining three corners of the room. The toys are taken out of their containers to ensure that there will be enough clean-up to be done at the end to fill the observation time.

Typically, DPICS – III coding is conducted using a one-way mirror and behind-the-ear (BTE) hearing aid device. We find that young children are much more likely to display the behaviors that initiated the referral if the therapist is out of the room. If an observation mirror and BTE device are not available, the DPICS – III is coded with the therapist in the playroom, seated as far away and unobtrusively as possible. The instructions can be provided to the parents by whispering, providing them ahead of time outside the child's earshot, or in written form. If observing from within the playroom, we suggest explaining to the child: "My job is just to sit and watch you and your mom (dad) play. Sometimes I might write something down, but I can't talk to you, not even a little. Your job is to play with your mom (dad) and pretend like I'm not even here, like I'm invisible." In order to avoid becoming a participant in the interaction, the therapist-evaluator must completely ignore any of the child's overtures. After a minute or so, nearly all children will begin ignoring the unresponsive therapist and will interact exclusively with the parent.

Most parents feel somewhat nervous about the observations initially. It helps to anticipate this anxiety, reassuring parents that "most people feel a little nervous about this at first, but just try to relax and play with (child's name) like you would at home." Most 3-and 4-year-olds will be unaware that they are being observed unless they are brought into the observation room. However, older children who are not developmentally delayed usually realize they are being observed. To diffuse curiosity, we often bring the older child into the observation room briefly prior to the formal observation. We find that once the play observation is begun, children quickly forget they are being observed as they become engrossed with the play activity. It is also helpful to show curious children the bug-in-the-ear device. We usually hold the hearing aid up to their ears and are honest about the fact that we will use this to talk to their parents about ways to play with them. Children also are instructed that the hearing aid is not a toy and can break easily. As such, they will not be permitted to play with it. You may wish to allow the family a 5-min warm-up period during each of the first two situations (not clean-up) to allow for anxiety to diminish before coding begins (Eyberg et al., 2005).

We recommend that the pre-treatment observation be videotaped (with written parental consent and child assent) whenever possible. Videotaping allows for the interactions to be reviewed multiple times, especially important for therapists who are inexperienced in observing parent-child interactions. We save the videotape and make it part of our review of treatment progress at the conclusion of therapy. To ensure that the playroom lights stay on during the videotaping, switches can be protected by mounted lock boxes or hidden by electrical tape. We recommend testing audio levels on the recording prior to conducting the observation.

We observe the parent and child interacting in each of three standard 5-min DPICS situations that vary in the degree of parent control required: Child-Led Play, Parent-Led Play, and clean-up. The exact instructions given to parents are reprinted in Table 3.4 (Eyberg et al., 2005). The Child-Led Play situation, in which the child is allowed to play with whatever he or she chooses and has his parent's undivided attention, usually brings out the child's most positive behavior and allows the therapist to see how the parent and child relate to one another under optimal conditions. The Parent Led Play situation, in which the parent gets to pick the activity and asks the child to play along, is more challenging for the child with behavior problems. It provides an opportunity to see what strategies the parent uses to engage the child's cooperation, how the child responds to parental directions, and what particular disruptive and non-compliant behaviors the child exhibits. The clean-up situation is the most challenging of all, and if the child has significant behavior problems, they often are displayed during this final situation. Sometimes the clean-up is not completed within the 5-min observation period. If the child has begun the clean-up, we typically allow the parent to continue to enforce the clean-up instructions after we stop coding. However, with some defiant young children the interaction becomes a

Table 3.4 Dyadic parent-child interaction coding system – III instructions for parents

Child-led play (5 min)

In this situation, tell (child's name) that he/she may play whatever he/she chooses. Let him/her pick any activity he/she wishes. You just follow his/her lead and play along with him/her

Parent-led play (5 min)

That was fine. Please do not clean up the toys at this time. Now we'll switch to the second situation. Tell (child's name) that it is your turn to choose the game. You may choose any activity. Keep him/her playing with you according to your rules

Clean-up (5 min)

That was fine. Now please tell (child's name) that it is time to leave the playroom and the toys must be put away. Make sure you have him/her put the toys away by him/herself. Have him/her put all the toys in their containers and all the containers in the toy box

Eyberg et al., 2005, pg. 16

stand-off and we recommend ending the interaction after the 5 min so as to avoid an escalation in parental anger and potential embarrassment. Occasionally, an otherwise defiant child will clean up quickly, before the 5 min is completed. In such cases, the therapist can prorate the behavioral rating over the remainder of the 5-min period.

During each of these three 5-min segments, we keep track of a number of child and parent behaviors and verbalizations using tally marks on a coding sheet. The DPICS – III coding form provides space for recording 9 categories of parent behavior plus whether the child complies, does not comply, or has no opportunity to comply with parental commands. See Table 3.5 for a summary of categories coded on the DPICS. A sample coding sheet is available in Appendix 1. We strongly suggest that PCIT therapists access the Abridged Manual for the Dyadic Parent-Child Interaction Coding System, Third Edition (Chase & Eyberg, 2005). It is currently available for download at www.pcit.org. This manual provides complete definitions for each of the coding categories, decision rules, as well as the most up to date coding forms. For researchers, we recommend the use of the complete manual (Eyberg et al., 2005).

Table 3.5 Summary of DPICS – III coding definitions

Category	Definition	Example
Labeled praise	Specific statement expressing a favorable judgment	Good job sitting at the table
Reflection	Repeats the child's talk	Child: I made a big square Parent: You made a big blue square
Behavioral description	Describes the child's current activity (usually begins w/"you")	You're drawing the sun
Neutral talk	Describes information other than the child's current activity or provides an acknowledgment	That's a rainbow or okay
Unlabeled praise	Non-specific statement expressing a favorable judgment	Good job
Direct command	A clearly stated order	Please put on your shoes
Indirect command	An implied direction, often asked in question form	Could you put on your shoes?
Question	A comment expressed as a question (could be by inflection)	What are you making? or a dragon?
Negative talk	Expression of disapproval (could be sarcasm)	Don't stand on the table or (sarcastic) that was great

The behavior we see in a clinic observation may or may not approximate the parent's and child's behaviors in other settings. Following the observation, we ask parents to estimate how typical the child's behaviors were in each of the three situations. Many times parents will tell us that the child was on his or her best behavior and we did not see how bad things really get at home. Very rarely will a parent indicate that the child behaved worse than is usual at home. Following the

DPICS – III observations, parents are often worried about how the therapist perceived their parenting. To set them at ease, we make it a point to praise parents for any positive parenting that we observed during the formal coding (e.g., “I liked the way you got down on the floor with him”).

Joining with and Motivating Parents

The pre-treatment assessment and therapy orientation session is more than just an evaluation. It is our first therapeutic contact with the family and it sets the stage for the entire treatment. Joining with and motivating families is critical in this session because PCIT cannot work if families do not come back. It is our job to hook families in, setting expectations that we are going to be very helpful to them. For us, the primary goal of this session is to get families excited about PCIT and committed to making weekly therapy attendance a priority. If we fail to establish excellent rapport in this session, we may not get another chance. In our experience training many therapists we have found that when drop-out occurs, it most often happens in the first three sessions. We think this is because most therapists are so focused on getting paperwork completed in this session, that they lose sight of the bigger picture: meeting the parents’ needs. We ask therapists in training to imagine what it would be like to be a parent contacting a mental health clinic for the first time to seek help. It takes a lot of courage to admit that you cannot handle your child. Parents arrive anxious, defensive, desperate for help, guilty, and often feeling like failures. What they need from the therapist is not a sterile question and answer session. They need much, much more.

They Need to Be Heard. Parents need an opportunity to tell their story to an empathic and non-judgmental listener. Rather than asking a question, getting an answer, and quickly moving on to the next interview question, we think it is very important to reflect back, paraphrase, and summarize the content of their concerns throughout the interview. We say things like, “I can see that aggression is a really big problem both with his sister and with the other children at school,” “So you have been dealing with extreme temper tantrums for over 2 years. Wow!” and “It sounds like you’re seeing a lot of similarities between the problems your older son had and the acting out that Johnny is doing.” We also think it is important to reflect back the feelings behind the client’s words. Examples of emotion-focused reflections include, “This must be really stressful for you. I don’t know how you do it,” “It sounds like you’re blaming yourself for him getting kicked out of school,” “I can tell that you really love your daughter and only want the best for her,” “So you’ve been feeling really down ever since the baby was born,” and “He pushes your buttons to the point that sometimes you worry that you might lose it and hurt him.” Many families enter therapy skeptical that a well-educated counselor can really understand their stress. This is particularly true when there are differences in socioeconomic status and ethnicity.

Frequent use of reflections is important for establishing rapport because it sends the message that the therapist truly understands the family's problems.

They Need to Feel Validated. A parent who finally makes the decision to seek help, needs validation that this was a good choice. Often, parents are being told by their spouse, friends, and extended family that the child is just going through a phase and that sending a 4 year old to a "shrink" is ridiculous. To make matters worse, it is not uncommon for the child to be on his best behavior during the first appointment in this novel setting. Parents then worry that the therapist will think they are exaggerating and overreacting to minor problems, much like when the mechanic cannot find the rattle in their car or the fever goes away at the walk-in clinic. We make it a point to communicate to parents that they are absolutely right to seek assistance. At the end of this session, we provide feedback on testing results usually verifying that the child's behavior is outside normal limits and warrants intervention. We praise parents for having the foresight to catch problems early when intervention is the most potent. Having the therapist confirm their suspicion that therapy is needed helps a tentative parent who started with only a toe in the door, end the session with both feet firmly planted in their resolve to work hard in PCIT.

They Need to Be Supported. When parents enter the first session, many feel isolated and embarrassed, thinking that they are the only ones who cannot control the behavior of their preschoolers. They need to know they are not alone. We tell parents that we work with many families experiencing similar difficulties and that they came to the right place. Often, these parents enter the clinic looking like they have been beaten down by criticism and blame. We hear stories about how the grandparents criticize their parenting, suggesting that if the parent used a firmer hand then the child would be perfectly fine. These parents receive almost daily negative feedback on their child's behavior in daycare, with frequent calls about crises and embarrassing reports at pick-up. These parents, particularly the mothers, feel inept, resentful, and hopeless. It is our job to be sure that they leave the intake session feeling built up, not torn down. Throughout the interview, we look for things that they are doing right and provide lots of supportive comments, particularly labeled praise. We make comments like, "He's really lucky to have a mother who cares so much," "What a great insight," "You're using the same kinds of strategies that most parents use and they're good ones. Sarah is just a really tough kid," "Your parents really need to learn to respect your authority and not interfere in your disciplining" "I can tell you're a very loving mother," "You've been doing your best to support the school by giving consequences at home," "With so many people discouraging you from coming in, I'm so impressed that you followed your instincts and did what you knew was right. You were right to get help for him," and "I am amazed that you have the energy to keep up with all four of those children and work two jobs. You should be proud of yourself." These supportive statements serve several purposes: (1) they enhance rapport, making the client want to come back, (2) they build parental self-esteem, (3) they model the positive tone that we want parents to use with their children, (4) they nurture parents, giving them the internal resources to nurture their children in return, and (5) they decrease defensiveness as the parents realize that this is a safe place to talk about concerns without being judged or blamed. Upon entering

treatment, parents fear that the therapist is going to confirm for them that they are indeed bad parents and that they have caused their children's behavior problems. Parents are usually pleasantly surprised to encounter a very different therapist attitude, one in which the therapist conveys respect and praises parents for what they are doing well.

They Need to Hear Positive Things About Their Children. We consider it our job to help parents recognize their children's strengths and appealing qualities. In order to improve the parent-child relationship and strengthen attachment, parents need to learn to like their children again. At the start of treatment, most of our parents react to the stress of dealing with behavior problems by becoming angry, critical, and rejecting toward their children. It can be hard for them to recognize any positives in their children as their good qualities are overshadowed by the biting, hitting, spitting, and other misbehaviors. Indeed, other people in the child's life (teachers, relatives, neighbors) become focused on the negative and provide the parent with a steady diet of child criticism. They are constantly being told that their children are mean, bossy, hyper, annoying, destructive, and rude. As PCIT therapists, we even have to work hard sometimes to find positive qualities in some of these children at the beginning of treatment. It is our mission to look carefully for each child's strengths and to comment aloud about them to parents (in front of the child when possible). We make positive observations such as "What a cute boy," "She's got a great smile. It lights up a room," "His sense of humor cracks me up," "He really knows a lot about dinosaurs," "She speaks her mind. She may make a great lawyer some day," "He has a clear vision of how he wants things to be. He'll make a great leader when he's older," "He's quite the engineer" (during Lego building), "I love his haircut," and "You sure dress her cute." Parents with children with disruptive behavior are seldom given credit for their children's strengths, but they are always blamed for their children's shortcomings. We listen carefully for times when children use polite manners as this is a great opportunity to give parents credit for good parenting. We say, "I noticed that he just said, 'thank you,' You've done a great job of teaching him to be polite. I know he doesn't always use the good manners that you've taught him. But, he knows 'please' and 'thank you' because you have been a good teacher."

They Need to Be Educated. When parents arrive at an intake session, they are eager to ask the expert about why their child is misbehaving. Yet in most intake interviews the therapist asks almost all of the questions. It is important to provide opportunities for parents to ask questions and to provide education throughout the intake. For example, prior to asking questions about the pregnancy, the therapist could explain how prenatal factors affect preschool behavior. Similarly, the contribution of genetics to mental health disorders could be discussed prior to soliciting the familial history. It is our goal for each parent to leave the intake session with questions answered and with a better understanding of the probable etiology and recommended treatment for the child's behavior problems.

They Need to Feel Encouraged and Hopeful. A major fear of parents at the assessment session is that the therapist will be unable to help them. We need to convey to the parent that we have special knowledge and expertise that will help their child.

If the entire intake is spent asking historical questions for the purpose of writing a comprehensive intake report, the parents leave the session not knowing whether the therapist can help or not. It is important for therapists to intersperse their intake questions with encouraging statements forecasting that the family’s problems can be effectively treated. Sometimes, at the most emotional point in the session, when parents are telling their story and get teary with despair, we have an opportunity to provide encouragement and hope. For example, we can say, “I am so glad you’re here. These are exactly the kinds of problems that respond beautifully to behavior therapy, and I have exactly the right treatment program for your family. I’m really excited to get started. I think I can help.” As families discuss presenting problems, we intermittently respond with encouraging and confident comments like, “I’ve got a great strategy for that,” “We deal with that a lot here in this clinic,” “He sounds like my kind of kid. This is my specialty,” and “I’m making some notes about a plan for that. I know where we need to start.” We want parents to leave the session feeling better than when they entered. They should have a little optimistic spring in their step and be eager to start therapy the next week.

Sharing Test Results with Parents

At the end of the initial evaluation session, we meet alone with the parents to answer questions and provide preliminary feedback on the results of testing. Although we may not have all of the formal measures scored and interpreted, we usually have sufficient information from the interview, behavioral observations, and ECBI to determine whether we will be able to provide a helpful service for the family. We go over the results of testing in detail and share our impressions of the child’s behavioral and emotional development. We ask parents to share with us their ideas about why their child is experiencing problems. Often, parents will express guilt and wonder aloud whether there was something they should have done differently during the pregnancy or during early child rearing. In most circumstances, we are unable to precisely determine what factors are responsible for the child’s problems. We explain to parents that young children’s problems are multiply determined and that it is nearly impossible to say that any one factor caused the problems.

Explaining “Specialized Parenting”

We emphasize that even though the parents are probably not directly responsible for the development of their child’s problems, they are the only people with the power to successfully resolve them. Our goal is to try to diminish non-productive guilt, while giving parents back the responsibility for treating their children, thus enhancing their motivation to work hard in PCIT.

This process of reducing parental guilt while encouraging responsibility is a tricky one because the therapist must always be non-judgmental. If parents think

that the therapist views their parenting as “bad” or “inadequate,” they will “yes, but. . .” the therapist throughout the skills training. Yet, the therapist is advising the parents to undergo parent training, which to many parents implies inadequacy. One way to deal with this problem is to explain the notion of “specialized parenting.” We explain to parents that some children are temperamentally more difficult to parent than others because of short attention spans, difficulty handling change, willfulness, or developmental problems. While typical parenting is usually effective for children with easy dispositions, other children have special needs. Rather than viewing PCIT as a remedial program for dysfunctional parents, we then can view it as a mechanism for helping parents to form a better fit with their children. This “fitting process” involves teaching highly specialized skills that enable parents to manage children who do not seem to respond well to “typical parenting.” The message here is that we accept and respect the parents’ current skill levels, but we still aim to elevate their parenting to the level of “expert.” We motivate parents to work with us in PCIT by telling them that our goal in therapy is to teach them to be experts in behavior modification. We even say, “By the end of treatment, you’ll probably have relatives and friends coming to you for parenting advice.”

Introducing PCIT to Parents and Children

After sharing with parents the results of testing and answering their questions concerning the causes and expected course of behavioral and emotional problems, we introduce them to PCIT. The goal of this introduction is to get parents excited and hopeful about treatment, and to establish the expectation that PCIT will require an intensive effort on their part. We explain that we offer a service that was designed specifically for young children with problems similar to the ones displayed by their child. We then provide an overview of PCIT with brief rationales for the importance of both the Child-Directed Interaction and the Parent-Directed Interaction components. Parents are told that PCIT takes approximately 12 sessions, and the structure of PCIT is explained to parents. Specifically, we explain that there will be only two sessions without the child, one in the first part of treatment to teach the CDI skills and one in the second part to teach PDI. We explain that the other sessions involve direct skills coaching using the bug-in-the-ear device. We tell parents that if they work very hard and their child responds like most of our other clients, they can expect significant improvements in cooperative behavior, happier mood, more affectionate interactions, and improved behavior in the classroom. Here is an example of what we might say when introducing PCIT to a new client:

I’m going to be recommending a program called Parent-Child Interaction Therapy or PCIT. This program has been used with thousands of children across the country who have behavior problems just like Roberto’s. Many research studies have been conducted on this program and they show that children’s behavior goes from severe levels to normal levels, so that they behave like most children at the end of treatment. PCIT is unique in that we won’t just talk about parenting and ways to manage Roberto’s aggression, defiance, and tantrums. Instead, I will actually coach you, telling you exactly what to say and exactly what to do

to handle his misbehavior while it is happening right here in this clinic. We'll do this by using this little hearing aid device. This allows me to coach you from another room. This is helpful because children tend to show their true colors when I am not in the room. PCIT will take about 12 sessions, once a week, for 1 h each session. And, it will take a commitment from you. In particular, you will need to find 5 min a day to do homework with Roberto. Although that might not seem like much right now, it is actually a very big commitment because I need you to do it almost every single day for the next 12 weeks. Is that something you can make a commitment to right now? I need for you to be sure about this because the program will not work without the commitment of 5 min each day for practicing the PCIT skills at home with Roberto.

There are two parts to PCIT. The first half focuses on making your relationship with Roberto even stronger than it is right now. We need for the relationship to be very, very close so that Roberto wants to please you. This will set the stage for helping him succeed with the second part of the program which is a very firm discipline program. Researchers studied hundreds of children with behavior problems even more severe than Roberto's. They tried lots of different discipline approaches and they found *one* that worked better than all the rest. This discipline program focuses on over-training Roberto to follow your directions, to do exactly *what* you tell him to do, *when* you tell him to do it. So, when we get to the first discipline coaching session, I can make a promise to you. I will not stop that session until Roberto is obeying your commands. If I have to stay here late and bring in pizza, I will, because we will not stop that session until he has learned to listen. How does this sound?

If the parents indicate that they would like to participate in PCIT, we bring the child back into the room and provide him or her with a developmentally appropriate explanation about treatment. For example, for an oppositional 4-year-old boy, we might use the following explanation:

Sometimes you get mad at your mom and dad, and sometimes they get mad at you. I'm somebody that helps moms and dads learn how to play better with their boys. I also help little boys learn how to listen better to their mommies and daddies. From now on, you'll get to come here with your mom and dad each week. You'll play with my fun toys while you learn to get along better with your mom and dad. Does that sound okay to you? What toy should I have out for you when you come to play next time?

Expectations for Attendance

Many of the clients we work with come from impoverished backgrounds and rely on government assistance for their medical care. They are used to attending free clinics on a first come, first served basis. These clinics usually do not have expectations that clients show up on time or call ahead to cancel. We cannot assume that these families understand our need for formal scheduling. They probably have never considered the fact that the therapist is holding an entire hour open on the schedule just for them. It is our job to educate these parents about the importance of keeping their appointments, arriving on time, and calling in advance when needing to cancel. Often, we ask that they sign an attendance policy to formalize our expectations.

A sample attendance contract is presented in Table 3.6. Attendance policies will vary based on agency guidelines. To view the attendance policy used in Dr. Eyberg's clinic, please see the PCIT Treatment Manual (Eyberg, 1999).

Table 3.6 Sample attendance contract

PCIT attendance expectations

PCIT is a powerful program for children with behavior problems. Research shows that most children who complete PCIT no longer have problems with aggression, non-compliance, and temper tantrums. And, these improvements can be seen up to 2 years after treatment is over

However, PCIT only works if parents can commit to coming in for treatment once a week for approximately 12 weeks. We understand that this is a big commitment. So, we are going to start with smaller expectations

To get the program off to a strong start, we ask that you agree to come to *4 sessions*. At the end of those 4 sessions, you should see a nice change in your child and in yourself. Then, we will ask you if you would like to commit to 4 more sessions

If you agree to this treatment, it is important that you call ahead of time if an emergency arises and you cannot come to your session. This is necessary because it saves your therapist a lot of time and allows another family to be seen during that hour

Before each session, your therapist takes time to review notes, develop handouts, and set up the room. By calling ahead when you cannot make it, you are allowing your therapist to use that time to help other families. Also, your therapist will work with you on trying to schedule another appointment as soon as possible to keep your child from losing gains in therapy

I, _____, agree to come to 4 sessions of Parent-Child Interaction Therapy. I also agree to call ahead of time if an emergency arises and I cannot make my appointment. I understand that if I do NOT attend as expected, my therapist may have to discontinue PCIT with me and my child to make room for other families who also need the help

Signature

Date

Reducing Barriers to Treatment

Particularly for stressed, multi-problem families, it is important in the first session to identify barriers to treatment. Many of these families rely on buses or friends or family for transportation to therapy. Others risk losing their jobs if they must take off time from work on a weekly basis. Still others have difficulty attending because of childcare issues. If we can identify these barriers early on, we may be able to work to reduce them. For example, some agencies have transportation funds in which a taxi can be provided to bring the family to sessions. We have completed family leave forms for employers so that parents can take off work without fear of sanction. Sometimes, we are able to schedule sessions in the evenings or on Saturday mornings to accommodate work schedules. For families who travel a great distance, we will sometimes schedule lengthier and less frequent sessions. When parents are unable to secure childcare for siblings, it may be possible to arrange for childcare within the clinic. By accommodating parents we both build rapport and reduce factors that contribute to treatment attrition.

Explaining the First Homework Assignment

From the outset, we instill expectations that families will have daily homework to do between each of their sessions. This first week, we ask parents to find 5 min each day to spend one-on-one time with their child doing “whatever you might ordinarily do together – you can play together with your child’s toys, or read a bedtime story to your child, or go for a walk outside” (Eyberg, 1999, p. 8). The purpose of this homework is for parents to identify a time each day that will later turn into their play therapy time. We want to send a message to parents that they will be accountable for completing homework. To accomplish this, we send them home with a homework sheet on which to record their daily home practice (see Appendix 2).

Summary

The pre-treatment assessment and therapy orientation session(s) involves a number of key components. Parents must complete administrative forms and rating scales. Parent-child interaction observations are conducted and the therapist conducts a thorough intake interview. We share with parents the results of testing and talk with them about how PCIT can help their family. After parents agree that they would like to participate in PCIT, we try to identify any barriers to regular attendance and explain our attendance policy. Finally, the first homework assignment is presented. Yet, with all of the procedures that need to be conducted in this first session, it is easy to lose sight of the most important goal, namely engaging parents. If we do not effectively join with parents and motivate them, they will not return for treatment and we will have missed our opportunity to help that child. Table 3.7 lists a series of questions to help therapists evaluate the effectiveness of this critical first PCIT session. We hope that this chapter helps therapist to better engage multi-problem families and reduces the attrition that may occur early in treatment.

Table 3.7 Was this an effective therapy orientation session?

(1)	Did I praise the parents, pointing out ways that they are parenting well?
(2)	Did I make positive comments about the child to the parents?
(3)	Did I reflect the parents concerns?
(4)	Did I educate the parents about their child’s behavior problems?
(5)	Did I instill positive expectations that the treatment will work?
(6)	Did I tell the parents about my experience and expertise?
(7)	Did I validate for the parents that these problems are significant and need intervention?
(8)	Did I help the parents understand that they are not alone and that other parents also struggle with these problems?
(9)	Did I conduct all of the assessment procedures and provide parents with feedback on the results?
(10)	Did I reduce parental blame by explaining PCIT as a program that teaches “specialized parenting?”
(11)	Did I explain the structure of PCIT?
(12)	Did I help to reduce barriers to treatment, such as providing assistance with securing transportation and childcare?

Table 3.7 (continued)

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- (13) Did I review the no-show/cancellation policy and convey an expectation that the family should attend all scheduled sessions or call to cancel?
- (14) Did I give the parents a homework assignment to spend 5 min of one-on-one time with their child each day?
-

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Chapter 4

Teaching Child-Directed Interaction

What to bring. . .

- (1) Toys for demonstrating CDI
- (2) ECBI
- (3) Suggested Toys for CDI – parent handout
- (4) CDI PRIDE Skills – parent handout
- (5) CDI Homework sheet
- (6) Sheila Eyberg’s Treatment Integrity Checklists/Manual

After the initial PCIT evaluation and feedback have been completed, parents are asked to attend a “teaching” session in which the therapist introduces the family to the basic skills of Child-Directed Interaction play therapy. Because a great deal of information is shared and it is important to have the parents’ undivided attention, we request that parents arrange for child care during this session. Most often, the teaching session lasts at least 60 min to allow for the many questions that arise in response to the material presented. Before this session begins, we have the parents complete the ECBI. Then, later in the session, we show the parents the changes that have taken place in the ECBI (either positive or negative) since the last session.

Overview of Teaching Session

In this session, we review the homework from the last session. We then provide a description of the goals of this part of PCIT, emphasizing how Child-Directed Interaction may help resolve the specific problems that were identified during the intake evaluation. Parents are told that they should not worry about taking notes or trying to memorize material because they will be given a handout at the end of the session including all of the information. The rationale for use of brief daily home “play therapy” sessions is described. Next, the therapist presents a set of “Avoid” skills and a set of “Do” skills. Each skill is described along with its rationale, examples are given, and the skill is briefly demonstrated by the therapist. The best

teaching sessions are interactive, with the therapist inviting the parents to comment on how they think the skills will help their child, how they believe their child will respond, and any problems they foresee. Because relationship-building is an important goal of this part of treatment, *both* parents should individually conduct daily home play therapy sessions with their child.

By the end of the presentation, we want the parents to be able to name all of the “Do” and “Avoid” skills, so we use a repetitive teaching style in which we summarize all of the previously presented skills before moving on to the next skill. A mnemonic aid regarding the “Do” skills is the acronym “PRIDE,” combining the first letter of each of the “Do” skills (i.e., Praise, Reflect, Imitate, Describe, and use Enthusiasm). After covering all of the “Avoid” and the “Do” skills, the therapist presents the concepts of “strategic attention” and “selective ignoring” for shaping behavior. The therapist then models using all of the skills in combination, often with the parent or a co-therapist playing the part of the child. Finally, parents are invited to role-play with the therapist providing some gentle in-the-room coaching.

At the end of the session, parents are given a handout summarizing the behavioral play therapy skills and their rationales (see Appendix 3). We save the handout for the end of the session so that parents are not distracted by reading ahead. In addition to the skills handout, parents are given a “homework sheet” on which to record their daily practice and any problems that come up so they can be addressed in the next session (see Table 4.1 for an overview of the CDI teaching session).

Table 4.1 Steps for teaching Child-Directed Interaction skills

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1. Review homework
 2. Describe goals of Child-Directed Interaction
 3. Discuss 5 min of daily home practice
 4. Present and model the “Avoid” skills
 5. Present and model the “Do” skills
 6. Discuss use of strategic attention
 7. Discuss use of selective ignoring
 8. Model all skills in combination
 9. Coach parents as they role-play skills
 10. Discuss logistics of play therapy at home
 11. Assign new homework
-

We have sometimes been asked by parents if it would be okay to record the teaching session so they can listen to it again at home. There are several advantages to taping. First, it is good to have a tape available when there is a reluctant spouse at home who may be convinced to listen. Second, there may be other caregivers who were unable to attend the session but would like to learn the material (e.g., grandmother, head start teacher). Third, many people learn best through repetition and their retention of the information is enhanced if they can review it on tape one or more times. And fourth, sometimes we work with parents with reading difficulties who are able to make better use of a tape than a written handout.

Review Homework

The session begins with a review of the homework assigned in the pre-treatment assessment and therapy orientation session. We set a precedent that parents are expected to complete their daily homework and to remember to bring their homework sheet with them to sessions. This is done by beginning every session by asking parents to turn in their homework sheet. If the parent brought the sheet and completed most of the daily practice sessions, we praise them (e.g., “It was wonderful that you made time to fit in the homework most days. Your daughter is lucky that you care enough about her to make this a priority”). If they brought in the homework sheet but it is evident that they did minimal homework, the therapist praises them for bringing in the sheet, reminds them of the importance of daily practice, and problem-solves with them about possible solutions (e.g., a reminder note on the refrigerator, do it during the baby’s nap time). For parents who forget to bring their homework sheets, we bring out a new form and get it completed before continuing with the session.

Presenting the Goals of Child-Directed Interaction

Next, we remind parents of the overall structure of PCIT (i.e., CDI and PDI) and its goals. We explain to them that the relationship enhancement work of CDI sets the stage for the firm discipline program of PDI to be effective. We often use the analogy of building a house. For the house to be structurally sound, there must first be a strong foundation. That foundation is the solid attachment and warm relationship that develops during the CDI phase of treatment. We forecast for parents that this foundation-building phase takes time and patience and is not as glamorous as the PDI phase when we quickly and directly address behavior problems. Yet, we explain that the house will not stand without a strong, well-built foundation. We ask parents to be patient with this slower, more subtle phase of therapy and reassure them that we expect to see dramatic and rapid change once PDI begins.

There are many more specific possible goals for Child-Directed Interaction, and the goals that are emphasized for any particular family should be based on the presenting problems identified during the intake evaluation. We find that Child-Directed Interaction often improves children’s self-esteem, improves the parent-child relationship, helps children to attend longer to play activities, makes oppositional children less angry, and improves frustration tolerance and perfectionism. As presented in Part II of this text, Child-Directed Interaction may be tailored for children with developmental problems, foster and adopted children, children with internalizing problems, and abuse survivors.

Explaining the Five Minutes of Homework Each Day

The skills presented in this chapter are to be used by parents in a daily 5-min “special playtime” at home. Five minutes may seem brief, but it offers several advantages.

First, the brief amount of time removes much of the resistance to home practice that comes with longer practice periods. Parents cannot in good conscience tell us that 5 min is not available in their busy schedules each day to devote to their young children. Second, by using a brief practice period, parents are able to sustain a high degree of quality during their special playtime, making them less likely to just “go through the motions.” Third, although 5 min does not sound like a great deal of time, it is perceived as a long time by novice play therapists who are concentrating hard on using their skills correctly. Longer practice periods early in therapy often lead to fatigue and diminished enthusiasm for treatment. Fourth, a major reason why behavioral play therapy is child-directed is to set up a situation in which the child is most likely to display pro-social behaviors. For children who have behavioral problems, the longer the play session, the more likely it is that their good behavior will deteriorate, detracting from parent-child relationship building. And finally, very young children have a different sense of the passage of time than do adults. To preschoolers, 5 min feels like a long time when you are driving in the car, waiting for the food to come, or the parent is on the telephone. It also feels like a sufficiently long time to have their parents’ undivided attention.

We explain the 5-min home sessions to parents by saying,

The rules that I will be describing are to be used during a daily 5-min special play period at home. I certainly don’t expect that you or anyone would be able to keep up this high-quality, condensed therapeutic time for extended periods each day. In fact, I find that parents who try to spend longer than the 5 min actually burn out on play therapy because it takes so much energy. I don’t want that to happen to you. So, the key to making play therapy work is to do a little bit consistently every day, not to do it irregularly but for longer periods.

We explain to parents the importance of a 5-min play session, explaining that providing sporadic longer sessions (e.g., 15 min) can lead children to feel cheated on days when they receive their standard 5-min special playtime. Because we emphasize the use of a 5-min play session, some parents become overly focused on preventing themselves from running overtime. Some even ask us if they should set a kitchen timer to go off at the end of playtime. We discourage such rigid adherence to time rules because it is distracting to both the parent and the child, and detracts from the naturalness and pleasure of the playtime. Furthermore, ending the session abruptly at 5 min may prematurely cut off an activity that the child has worked hard on and nearly completed, causing unnecessary frustration. Instead, we encourage parents to look for a natural breaking point after about 5 min, even if playtime is extended by 2 or 3 min. At the end of the playtime, parents are encouraged to praise their child for the positive qualities they observed during the session and to express their own pleasure in having shared the time together.

As power struggles can easily develop around the issue of putting the toys away, we encourage parents to handle clean-up in one of two ways (1) to say, “I’m going to pick up the toys now. You can help if you want” or (2) to allow the child to continue playing by saying, “Special playtime is over now. You can continue playing with the toys if you want. But, I have to do some other things right now.” Therapists can help parents accept these non-confrontive suggestions for picking up the toys by

reminding them that clean-up strategies will be covered extensively in the discipline part of the program.

It is important to make it clear to parents from the beginning that play therapy should not be viewed as a “privilege” that is contingent on good behavior from the child. Highly stressed and/or punitive families have a tendency to withhold play therapy when they have had a difficult day with their child, unless this point is addressed directly in the teaching session. We emphasize that special play time is actually more important on days when the child has displayed a great deal of misbehavior. On those days, the play therapy can help to interrupt the negative cycle by allowing the parent and child to have a very warm and positive time together.

“Selling” CDI to Skeptical Parents

Highly stressed parents may not be particularly motivated to improve their relationships with their children. They typically do not see the importance of playing with children, allowing children to lead conversations, and looking for positives in children’s behavior. They usually have no model for nurturing, responsive parenting, as it was not a part of their own upbringing. These stressed parents are seeking punitive strategies to quickly reduce acting-out behavior. They may be coming to the clinic with a child who has been kicked out of daycare for choking and biting classmates, cussing, spitting, hurting the class pet, and breaking toys. The highly stressed parent often views the therapist as naïve and clueless when the therapist suggests that these problems will get better by playing with the child for 5 min each day. These parents will often tell the therapist, “I already play with my child all the time and he’s still bad.” With these parents, we cannot simply list off the goals and skills of therapy and then expect them to buy in. If these families are going to return for the first CDI coaching session, the therapist must *sell* CDI.

CDI Is “Therapy” Not “Play.” First, we must make it clear that CDI is “therapy” not play. Parents must see that it is very different from any other way that they have interacted with their children before.

Suppose that you brought Rocky to me for individual therapy. I went to college for a long time to learn special techniques for working with children who have behavior problems. In my sessions with Rocky, I would use those skills to teach Rocky to behave better. I could get Rocky to obey *me*, to be polite to *me*, to share his toys with *me*, and to try to please *me*. But, when I brought him back to you in the waiting room, his rude and defiant behaviors would return because you would not know the special techniques. So rather than have you bring Rocky to me once a week so I can teach him how to behave with me, I am recommending that *you* learn the special techniques and become Rocky’s therapist. Then, rather than having therapy just once a week, you can be his therapist all day, every day. The skills therapists use with young children Rocky’s age are called “play therapy skills.” In the next hour, I am going to give *you* a crash course on how to be a play therapist.

Thus, it is important to convey that special playtime is a therapeutic intervention. If parents perceive this as “just playing,” they will discount the importance of this

portion of PCIT, and will either drop out or display only minimal compliance with homework.

To sell stressed parents on the notion that only 5 min per day really could have an impact on their children's behavior, we use an exercise analogy. Exercise is effective when conducted consistently over time. With consistency, small changes can produce large cumulative effects. We might tell "Rocky's" mother:

I want you to do play therapy with Rocky for 5 min each day. Now, 5 min may not sound like very much. In fact, 5 min is not very much time unless you do it consistently every single day. Imagine that you are trying to strengthen your bicep muscle. If you lift weights with that arm for 5 min one day, it will not do much good. But, if you exercise that muscle a little every single day for a couple of weeks, you will see a noticeable change. Play therapy is the same way. If you spend 5 min on Monday, and then you don't do the practice again until Friday, and then maybe you wait another week before you practice again, I do not expect you to see any improvements. But, just imagine how much more positive your relationship with Rocky would be if you spent 5 min every single day telling him how special he is. Imagine that for 2 weeks in a row, every single day you had this very intense connection with Rocky in which you told him how much you love him and reminded him of all the things that make him a wonderful kid.

Parents who come to see me typically say that their kids act out all day. Their time with their children is spent yelling at them and correcting them with words like, "stop that," "don't run with that stick," "leave him alone," and "I said 'no'!" In the midst of all that conflict, it is very hard for them to remember to say something nice. By setting aside 5 min every day, it guarantees that a day will never go by without Rocky hearing that you love and care for him, that you think he is a neat kid, and that you are happy to be his mom. So even though the rest of the day may have been awful, with lots of screaming and nagging, the play therapy will ensure that you and Rocky will have a special connection each day. It's true that doing this on Monday will not make much difference. But if you do this almost every day of the week for weeks at a time, those little daily connections will add up. Over time he will begin to work for your positive attention outside of the 5-min playtimes. Rocky will develop into a child who works hard to please you.

Have Parents Reflect on How Their Own Self-Esteem Might Have Been Different if Their Parents Provided Daily CDI. Sometimes we find it helpful to sell CDI by having parents reflect on their childhood relationships with their own parents. We typically hear that these parents had childhoods in which they were criticized, misunderstood, and emotionally neglected. As adults, their feelings are often still raw and many express resentment toward their parents. We help parents to see that they have an opportunity not to repeat the same mistakes and to parent their children differently. A discussion like this might motivate an otherwise skeptical parent:

I want you to imagine how your life would be different if your mother set aside 5 min every day to spend with you. In those 5 min, she did not answer the phone, dust the table top, pick up clutter, or wash dishes. She devoted that time entirely to you. So every single day you could count on having 5 min with your mom in which you were the center of attention. In those 5 min she complimented you, did what you wanted to do, smiled at you, hugged you, and let you know how happy she was to be your mother. During that time, she really listened to every word you had to say and told you over and over again what she liked about you. Imagine that you had a little bit of high-quality time with your mother every day of your life from the time you were 3 years old. How would your relationship with your mother have been different and how do you think you would have felt about yourself growing up?

This exercise helps parents recognize the relationship between messages they give their young children and the development of self-esteem. We tell parents that we want their children to grow up liking themselves, being confident, feeling secure, and always knowing that they have a parent who will listen. These are characteristics that help protect children later in life against the influence of drugs and alcohol, gangs, abusive relationships, and teen pregnancies. Daily Child-Directed Interaction is a start along the pathway to responsive parenting and raising successful, resilient children.

Over-Learning of Skills Until They Become Habits. To sell the idea of daily special playtime to a resistant parent, we point out that the 5 min serve as a practice period to over-learn skills to the point that they generalize effortlessly throughout the day. We might explain this idea to Rocky's mother in the following way:

Your homework for Child-Directed Interaction is to do special playtime with Rocky for 5 min each day. You may think that isn't a lot of time, but you haven't heard yet what I'm asking you to do during those 5 min. It is short, but it is intense. During that 5-min period, I am going to have you overuse the types of skills that are used in play therapy. You will use them at such an extremely high level that they will become over-learned. By over-practicing them in an exaggerated way, they will become habit and you will find yourself using those skills outside of the 5-min special playtime. You won't have to work at it or concentrate on it. It will just happen naturally.

Now, I'm about to teach you how to use a skill called "labeled praise." A labeled praise involves telling Rocky exactly what you like about what he is doing. I'm not going to ask you to praise him just a little bit. I am going to ask you to praise him 10 times in 5 min! That amounts to one labeled praise every 30 s. So, once every 30 s you'll be saying things like... "I like the way you shared the toys with me,"... "That was a beautiful boat you built,"... and "Nice job of using your manners." Compare that to how many labeled praises Rocky received all day yesterday. Now I don't expect you to give him a labeled praise every 30 s throughout the whole day. That would be weird. The purpose of doing the play therapy skills in such an extreme way for 5 min each day is that they will become a habit. You will find yourself giving Rocky labeled praises at the dinner table, in the car, at the doctor's office, and while he's playing with his sister. All you have to concentrate on right now is your 5 min a day, and the skills will spill out naturally. In a few weeks, you will find yourself being a "therapist" much of the rest of the day as well.

With a lot of the families that we work with, it is unrealistic to expect that we can just launch into a description of the skills of play therapy and expect them to just buy in to the basic premise that playing with their child for 5 min a day is going to help with the presenting problems. Instead, the parents need help getting motivated to do CDI by having the therapist explain the rules and rationales in language they can understand. Using real-life analogies and simple explanations, the therapist must actively and passionately "sell" CDI to skeptical parents.

Explaining the Overriding Rule of Letting the Child Lead

Parents are told that the most fundamental rule of Child-Directed Interaction is to allow the child to lead the activity. We explain to parents that children are at their best when they get to choose the activity, and we want them to get a great

deal of high-quality attention while they are behaving well. We also point out that there are few naturally occurring opportunities for young children to be in the lead. All day long they are told what to do by adults, and they are often perceived as the least capable members of their families. Having a brief period of time each day in which they get to be the ones who are most knowledgeable about the activity and make most of the decisions helps to relieve some of the frustrations inherent in early struggles for autonomy.

Teaching the “Avoid” Skills of Child-Directed Interaction

Commands. The first “Avoid” is “Avoid Commands.” Commands take the lead away from the child and set the stage for unpleasantness if the child disobeys. We talk about two specific categories of commands: direct and indirect. Direct commands are obvious demands made of the child, such as: “Hand me that crayon,” “Sit on your chair,” and “Hold my hand.” Indirect commands are less obvious and are often phrased as a question. Many parents use them without realizing they are subtle forms of commands. Examples of indirect commands include: “How about using the pink now?” “You might want to sit down to do that,” and “Could you sing me a song?” Both direct and indirect commands take the lead away from the child and should be avoided during CDI.

Questions. The second “Avoid” is perhaps the hardest of all of the CDI skills to learn: “Avoid Questions.” Questions direct the conversation instead of following and tend to take the lead away from the child. Many questions are indirect commands in disguise, and asking questions can also lead the child to believe that the parent is not really paying attention or disagrees with what the child is doing. For example, “Are you sure you really want to play with Mr. Potato Head?” is a leading question implying that the parent thinks the child should play with a different toy and suggesting disapproval of the child’s original choice.

Questions may begin with an interrogative such as “who,” “what,” “when,” “why,” “where,” or “how.” Alternatively, statements may be turned into questions by the inflection in the parent’s voice or by a question tag. For example, “You want to put that there?” (voice rising at end of statement) becomes a question because of voice inflection. Similarly, “That’s a rainbow, isn’t it?” becomes a question due to the tag on the end. Many parents have particular difficulty eliminating the voice inflection questions and question tags because they do not hear or recognize the subtle changes in inflection and semantics. Examples of common question tags to avoid include “isn’t it?” “right?” “huh?” “aren’t they?” “didn’t it?” “okay?” and “alright?” It can be helpful to briefly model and have parents rehearse various statements alternating declaratives versus interrogatives to maximize the contrast.

In explaining the rationale for avoiding questions during special playtime, we tell parents the following:

Adults ask children far too many questions. If you were to go to a shopping mall, and eavesdrop on conversations adults are having with children, you would find that almost 75% of what an adult says to a child comes out in question form. Adults ask so many questions because they want children to talk with them. Unfortunately, excessive questioning usually

has the opposite effect. Imagine what it would be like if you were constantly bombarded with questions. After awhile, you would probably start to feel interrogated and would just give very brief, perhaps one-word responses. Many parents complain that when they ask their children questions about their school day, they get brief, uninformative answers. Often this is caused by a pattern of excessive questioning from early childhood.

Sometimes we do an exercise in which we demonstrate for parents what it would sound like to ask questions at a high rate during CDI. It might go something like this:

Is that a snowman you're drawing? Are you going to give him a carrot nose? Are those buttons going up the front? Do you remember making a snow man at Granny Jean's house? What color is that scarf going to be? Wouldn't it be cool to make it striped? Is he missing something from the top of his head? What are you going to draw next? This is pretty fun, huh?

We then ask parents how many of those questions they think their child would actually answer and how comfortable it felt to be questioned at that rate. Most parents recognize themselves in that example and quickly become more aware of their rate of questioning. Of course, there are times throughout the day when it is important for parents to ask questions of their young children. We agree that questions are great for eliciting important information, stimulating creativity, developing conversational skills, and enhancing language development. We want parents to use questions outside of special playtime. However, we discourage the use of rapid-fire questions that provide no opportunity for the child to answer. And during the 5 min of special playtime, we ask parents to give children “a break” from all questions. We tell parents that in a few minutes, we will talk with them about a much more effective strategy for encouraging their children to talk with them.

Criticism and Sarcasm. The third “Avoid” skill is “Avoid Criticism and Sarcasm.” There are several reasons why criticism is discouraged during CDI (as well as in all parent-child interactions). First, criticism is not effective for decreasing problem behaviors, and it may even increase some undesirable behaviors. We explain to parents that nearly all young children strive for attention from adults. While they would prefer positive attention, they will work for negative attention if they don't know how to get positive attention. For example, we might say:

Imagine that you are at a restaurant with Ross and one of his calmer friends. It is taking a long time for the dinner to arrive. Ross and his friend are getting bored and they begin to make designs with the ketchup on their placemats. In annoyance, you resort to a skill that all parents use to teach right from wrong, criticism. You say to both Ross and his friend, “That's gross. You're making a mess.” How will the calmer friend feel? That's right. He probably will feel uncomfortable, embarrassed, and sorry that he disappointed you. And what will happen to the ketchup behavior? That's right, he will probably stop making designs. Now remember that you used exactly the same skill with Ross, telling him that the ketchup was gross and messy. What do you think is likely to happen to Ross' behavior? That's right; he's likely to pour out even more ketchup and get even sillier with the designs. So when you criticized him, the behavior increased. That means that criticism was a reward for him, like giving him an M&M or some money. Whereas criticism might decrease the problem behavior of typical children like his friend, it actually worsens and reinforces the problem behavior of disruptive children like Ross.

In this way, we help parents understand that criticism is ineffective for decreasing the problem behaviors of children who are referred for PCIT. We also make it a point to reassure parents that we will teach them skills that are much better than criticism at reducing disruptive behavior.

A second reason to avoid criticism is that it causes unpleasantness during the interaction, and we want the special playtime to be enjoyable for both the child and the parent. The third and most important reason for parents to avoid criticism is that it may result in self-esteem problems. We explain that,

Young children do not have the cognitive ability to reason critically and independently. Their attitudes and beliefs are heavily influenced by the things that adults tell them, particularly statements made by trusted adults like parents. If a parent tells a preschool child that horses fly then as far as the child is concerned, horses *do* fly. Most preschoolers do not have the ability to hear this statement, think back on what a horse looks like, realize that wings are needed to fly, recognize that horses have no wings, and come to the conclusion that their trusted parent has made an error. Similarly, if a parent tells a preschool girl she is dumb, then the girl incorporates that information into her self-image without scrutiny. She does not have the cognitive sophistication to think back to earlier in the day when she was successful at putting together a difficult ten-piece puzzle and realize that the parent is wrong and she is pretty smart after all.

We consider criticism to be any negative or contradictory statement about the child or what the child is doing. Some criticisms are blatant and uttered only during times of considerable parent consternation. Examples of blatantly critical remarks include “That was a dumb thing to do,” “Don’t act like such a jerk,” and “You sure are ugly when you whine like that.” Few parents we work with admit to saying such things to their children, yet we have observed them making these comments when embarrassed by their child’s disruptive behavior in the waiting area before or after a session. Thus, even when parents assure us that they do not criticize their children, we still feel it is important to give examples of blatant criticism and to discuss its ill effects.

In addition to obvious forms of criticism (e.g., “dumb,” “mean,” “idiot”), we teach parents to avoid the more subtle forms of criticism that often are expressed in the words: “no,” “don’t,” “stop,” “quit,” and “not.” These words tell the child what “not to do,” instead of what “to do.” For example, phrases beginning with “Don’t” are typically negatively stated commands and take the lead away from the child. Inherent in a negative command is disapproval of the child’s activity or behavior. We know that negative commands do not work well with young children who are oppositional, because words like “no” trigger defiance and cause many of these children to feel almost honor bound to do the misbehavior one more time. Similarly, negative correction is a subtle form of criticism that nearly all parents use. A negative correction occurs when the child makes an error and the parent points out the mistake before offering corrective information. For example, a child may color the dog purple and say “I’m gonna make him blue.” A negative correction would be “That’s not blue. You’re making him purple.” The first three words in this parental response are subtly critical and serve no purpose other than to call attention to the child’s mistake. Instead, we encourage parents to leave off the first part, while retaining the

noncritical second part of the correction. In this way, parents are able to teach their young children during special playtime without leading or criticizing.

Another form of subtle criticism is sarcasm where otherwise neutral or even positive statements can be made critical through a parent’s tone of voice. Parents are asked to avoid using a sarcastic tone while making comments such as, “Nice going,” “Now isn’t that just lovely?” “Like that will really work,” “Swell,” “Smooth move!” and “Thanks a lot.” Young children pick up on sarcasm at an earlier age than many would think, and they perceive their parents’ disapproval. Also, we like to remind parents that their young children imitate them, and they need to watch out for sarcasm unless they want to teach their children to use a sarcastic tone.

Teaching the “Do” Skills of Child-Directed Interaction

After presenting all of these “Avoid” skills, parents are often left wondering what is left for them to say and do during special playtime. Sometimes we have parents say, “What do you want me to do, just stare at her for 5 min?” We assure them that we have plenty of things for them to do! There are five “do” skills. When the beginning letters of the five words are compiled, they spell out the acronym, “PRIDE.” See Table 4.2 for an overview of the “avoid” and “do” skills.

Table 4.2 The “Avoid” and “Do” skills of Child-Directed Interaction

Avoid commands
Avoid questions
Avoid critical statements and sarcasm
Do praise pro-social behavior
Do reflect appropriate verbalizations
Do imitate appropriate play
Do describe appropriate behavior
Be enthusiastic!

Praise. The first “Do” skill is “Praise.” We encourage parents to provide numerous praises during special playtime, an average of one praise every 30 s! Two particular types of praise we discuss are unlabeled (general) and labeled (specific) praise. Unlabeled praises are ones that convey approval or affection without specifying exactly what it is that the parent likes. Examples of unlabeled praises include “Terrific,” “Nice Job!” “You’re so sweet,” “I’m proud of you!” and “Good.” In contrast, labeled praises tell the child exactly what it is that the parent likes. Unlabeled praises can be converted to labeled praises as follows: “Terrific counting!” “Nice job of playing so quietly,” “You’re so sweet to share with me,” “I’m proud of you for being polite,” and “Good choice of colors.” While both unlabeled and labeled praises are good for children and add to the warmth of the parent-child relationship, labeled praises are particularly valuable teaching tools. Labeled praises are so important that they are the only type of praise included in the mastery criteria. Thus, parents must provide one *labeled* praise every 30 s.

Young children will work hard for praise. Whatever behavior or quality the parent praises is more likely to be displayed by the child in the future. Labeled praises are more efficient than unlabeled praises at conveying to the child exactly what can be said or done to earn praise in the future. We explain this concept like this:

Suppose that Sally is coloring a heart. If you provide an unlabeled praise, like “That’s pretty,” Sally will feel good about herself, but she probably won’t know exactly what she did that earned the praise. But, if you say, “That’s pretty the way you colored inside the lines,” she will try very hard to color inside the lines again. If you say, “That’s pretty the way you colored softly with the crayon,” Sally will softly shade her pictures in the future. If you say, “That’s pretty the way you used several different colors,” a different behavior will increase. Similarly, if you say to Sally, “You were good at McDonalds,” she will feel good about that, but her behavior may not change. But, if you say, “You were good at McDonalds because you stayed close to me when we were in the long line,” she will probably stay close to you the next time. If you say, “You were good at McDonalds because you ate your whole cheeseburger,” you will get an increase in cheeseburger eating behavior. In this way, we can use labeled praise to strategically increase certain good behaviors.

We also teach parents that labeled praise can be used to prevent and reduce problematic behavior. This is accomplished by teaching parents to first identify the negative behavior that they would like to have diminished. Then, they are asked to think of what their child could be doing instead that would make it impossible to do the negative behavior. For example, if a parent would like to see whining decrease. The next step is to determine a behavior that is the opposite of whining, which might be “using a big boy voice.” A powerful tool to get whining to diminish is the use of labeled praise for the incompatible behavior. As the parent praises the big boy voice, the child will use the big boy voice more, thereby reducing the frequency of whining. Here’s what we say:

Did you know that you can improve *problem* behaviors with labeled praise? The way we do that is by identifying the opposite of the problem behavior and providing a labeled praise for the opposite. So, you mentioned that Jacob has a problem with hurting the dog. When he first wakes up in the morning and comes to the living room with bed head and sleep in his eyes, has he hurt the dog yet? This is a great opportunity to praise the opposite. You can tell Jacob that you are proud of him for being a good friend to the dog. Then, Jacob will try to be gentle with the dog from his first interaction. As gentle behaviors go up, hurting behaviors go down. We call this “proactive” parenting. Rather than being a “reactive” parent who waits for bad behavior to occur and then criticizes and punishes, a “proactive parent” anticipates problem behavior and praises the opposite before the child has a chance to misbehave.

Praising incompatible, pro-social behaviors is a very positive strategy for reducing problematic behavior without needing to give commands, yell, punish, or criticize.

In addition to increasing behavior, an important goal of labeled praise is to improve self-esteem. Because preschoolers believe what adults tell them, self-esteem at this age is fluid. It basically varies as a function of the feedback the child receives from people in his environment. Children referred for PCIT are at risk for self-esteem problems because their behavior inspires so much criticism and correction. Unless they receive very large amounts of praise to offset the criticism, these children tend to think of themselves as “bad” kids.

We explain to parents that,

Although praise is one of the most powerful tools available for improving young children’s behavior, it is equally powerful for improving your child’s self-esteem. As explained earlier when we discussed criticism, preschoolers believe what their parents tell them in a very profound way. They do not yet have the cognitive sophistication to reason analytically and reject false information. If a preschool boy consistently hears from his mother that he is “smart” and a “good helper,” he is likely to incorporate that information into his self-image. Thinking of himself as a boy who is smart and knows how to do things is likely to make him persist longer in problem-solving efforts and increase his confidence in trying new and difficult tasks. Similarly, thinking of himself as the kind of boy who is a good helper will make him more likely to volunteer to help with tasks at home and at preschool or kindergarten.

I’d like for you to think about a scale. On one side of the scale is criticism and on the other side of the scale is praise. If your son receives more praise than criticism, he will have positive self-esteem. It is your job to try to keep the scale tipped in the positive direction by giving as many labeled praises as possible each day. In this way, your daily special playtime is very important. If you do your 5 min each day, you will have ten labeled praises in the bank to help you keep the scale tipped in the right direction.

To help parents increase their use of praise outside the special playtime, we sometimes give praising exercises in which parents are asked to put 10 pennies in one pocket. Their job is to transfer a penny to their other pocket each time they give a labeled praise. Their goal should be to get all 10 pennies into the other pocket within an hour. By increasing the number of labeled praises used both within and outside of special playtime, we hope to build a more positive atmosphere in the home that will in turn promote positive self-esteem.

Sometimes, parents share with us their observations that praise seems to backfire with their children. They have noticed that their children tend to respond to praise by immediately doing the opposite of what was praised. For example, when a parent says, “Thank you for putting the Legos back in the box so quietly,” the child immediately begins to noisily slam the Legos into the box. Or, the parent might praise the child for chewing with his mouth closed with the result being that the child immediately laughs and displays the partially chewed contents of his mouth. We know that stimulation-seeking children often display this puzzling response to praise early in treatment. But, we also know that we can easily make labeled praise work for them by ensuring that they receive no negative attention when they misbehave when praised. Here is what we say to parents:

Many parents tell us that praise does not work for their children. And we may even see that happen with Jackson next week during our first coaching session. You may say to Jackson, “I like the way you are coloring inside the lines.” Jackson may respond by scribbling all over the paper, looking at you and laughing. It would be easy to conclude that praise makes Jackson worse instead of better, but that is not true. When a child scribbles all over the paper like that, it is natural for a parent to respond angrily with, “I said something nice. Why do you have to go ruining it?” Remember that children like Jackson work for stimulation. So when the child in our example was coloring inside the lines, he got only a little bit of stimulation in the form of a labeled praise. But, when he scribbled on the paper, he got a lot of stimulation from his mother’s criticism, her red face, and her raised voice.

If given the choice, children like Jackson will always choose the more stimulating criticism over the less stimulating praise. We can make labeled praise work by making sure

that you ignore or under-react when he responds to praise by increasing misbehavior. So when he is coloring inside the lines, you say, "I like it that you colored inside the lines." Then when he scribbles on the paper, I want you to say nothing. In my experience, children respond to this by saying, "I'm coloring inside the lines now. I'm coloring inside the lines now." If the choice is between a *little* bit of stimulation from a labeled praise and a *lot* of stimulation from criticism, children like Jackson will choose to behave negatively and get the criticism. But if the choice is between a *little* bit of labeled praise or *nothing*, they will start working for the labeled praise.

Praise comes more easily to some parents than others. Sometimes parents have a difficult time praising their young children because they are not demonstrative people by nature. Such individuals seem to benefit from the direct coaching of how to praise. Other parents have difficulty praising their young children because of tensions associated with the coercive nature of the parent-child relationship. Some are so angry with their children that they have difficulty recognizing positive child behaviors and attributes that are praiseworthy. When a "praise-able" behavior is recognized, some parents express the attitude that "He should have been doing that all along. Why should I make a big deal out of it when he's been so bad all day long?" Of course, we help them to understand that it is on these difficult days that their praise is especially important. For these parents, initial praises may sound less than genuine and be given in a begrudging fashion. However, we find that with consistent practice and coaching in Child-Directed Interaction, the parent learns to focus more and more on the child's positive attributes and praises come much naturally.

Reflect. The second "Do" skill is "Do Reflect." By reflect, we mean that the parent should repeat the basic message of what the child has said, a form of verbal imitation. The message can be extended, elaborated on, or subtly corrected through reflection. For example, if the child says "I builded a house," the parent might respond with any of the following: "You built a house" (grammar correction), "You built a house with a front door" (grammar correction with elaboration), or "You built a green and blue house" (grammar correction with elaboration of pre-academic concept). Parents may also use reflections to gently correct phonological process errors such as omitting the last consonant sound of a word (saying "mow" for mouse"), devoicing consonants (saying /p/ for /b/), stopping consonants (saying /t/ for /th/), and omitting unstressed syllables (saying "ghetti" for spaghetti).

Initially, many parents are able only able to "parrot back" the exact content of the child's message, without elaboration. Although these literal reflections sound less natural than elaborative reflections, they are still beneficial, particularly for very young preschoolers. Five- and 6-year-olds will often appear puzzled when parents first begin to reflect their verbalizations. However, most adapt quickly as parents become more skillful at elaborating and extending reflective communications.

Reflections communicate acceptance and understanding and let the child know that the parent is really listening. Adults often get into the pattern of simply acknowledging young children's statements with a head nod or nondescript verbalization such as "Uh-huh," while their attention is clearly elsewhere. While it may not be practical for parents to give their children their undivided attention upon demand

throughout the day, parental attention should be clearly communicated during Child-Directed Interaction. Acknowledgments such as “Yeah,” “I see,” “Uh-huh,” and “How about that” are considered missed opportunities for something more therapeutic. Acknowledgments should be replaced with reflective statements clearly communicating understanding of the child’s message. In teaching parents to reflect, we encourage them to become aware of times when their child repeats a phrase over and over such as, “Mommy, look at my star. Do you see my star? Here’s a star I made.” Repetition can be a signal to parents that the child wants a reflection.

Reflective statements also keep the child in the lead during conversation, encouraging the child to elaborate. As mentioned earlier, the most effective verbal stimulation for young children comes in the context of an ongoing activity of interest to the child. Reflection allows the parent to provide an immediate reward for the child’s verbal initiations, encouraging the child to speak more and more often. Reflections are much more effective for encouraging children to speak than are questions, and we ask parents to reflect nearly all appropriate verbalizations the child makes during special playtime.

When using reflections, it is important that they be phrased as a statement. When parents repeat children’s words with a questioning inflection, children may perceive that parents are not listening, do not believe them, do not approve of them, or do not understand them. We tell parents:

When reflecting, I want you to be careful to avoid making it sound like a question. For example, if Tara says, “Mr. Potato Head is jumping to the moon.” I want you to avoid asking, “Is Mr. Potato Head jumping to the moon?” When you ask it that way, Tara might think that you don’t believe her. Or, she might think that you weren’t listening very carefully and that you didn’t understand her. Instead, I want you to reflect with absolute certainty in your voice: “Mr. Potato Head *is* jumping to the moon.” When you say it definitively like that it sends a message to Tara that you agree with her and that you understand her. This will help your overall communication with Tara, helping the two of you to be on the same wavelength.

Imitate. The third “Do” skill is “Do Imitate.” It is important that parents be active participants in the play activity and not just passive onlookers. By imitating the child, the parent demonstrates that he or she is paying attention to the child’s activity and thinks it is interesting enough to do also. Imitation is indeed the sincerest form of flattery, and being imitated by powerful grown-ups is a self-esteem boost to young children. Imitation of the child also enhances the child’s imitation of the parent (Roberts, 1979) and forms the basis for one of the most important social skills for young children, turn-taking. Parents should keep in mind that any behavior they imitate is likely to be repeated by the child and to increase. Therefore, good judgment should be used in selecting appropriate child behaviors to imitate.

By imitation, we mean that the parent should play with the same or a similar toy and attempt to manipulate the toy in a way that approximates what the child is doing. We do not mean for the parent to imitate in a literal sense, with every action and every block color perfectly replicated. Basic imitation is a form of parallel play in which the parent approximates the child’s activity, always a step or two behind, but keeps the focus of attention on the child’s play. For example, if the child is

building a tower with the blocks, the parent should also build a tower, making sure to keep it shorter and perhaps less well balanced than the child's tower. The parent may occasionally draw the child's attention to the imitation by saying "Yours looks great. I want to try and build mine like yours." However, the parent should continue to keep his or her attention primarily on the child's activity, maintaining the running commentary.

Imitation is very useful for parents who are not accustomed to playing in a developmentally appropriate way with young children. It removes the burden of thinking up an engaging activity that is appropriate for their child's developmental level. Instead, the child teaches them how to play at the appropriate level. Some perfectionistic and high-achieving parents have difficulty with imitation. Some lose sight of goals of imitation and have a tendency to want to create the "Taj Mahal" rather than a modest structure that is within the child's developmental capability. We have found this more often in fathers who are accustomed to playing the role of builder and toy assembler/repairer at home. Thus, we caution parents to be sure that it is their child who comes up with the novel, creative ideas during play and to be sure that their "replication" does not appear more attractive than the child's original. Otherwise, the special playtime rapidly deteriorates, with frustration on the part of the child and a loss of interest in the too-advanced activity.

Depending on the child's level of development, the parallel play that is encouraged through imitation may be shifted toward more interactive play, with the parent placing a block on the child's tower and initiating turn-taking. However, if the child resists interactive play, we encourage the parent to select a different toy and casually manipulate it while continuing to focus on the child's solitary activity. Depending on the goals we have for a particular family, we evaluate whether it is more important to maintain the positive, warm tone of the interaction and avoid attempts at interactive play, or to gradually shape basic social skills. Appropriate shifts from parallel to interactive play are difficult to teach didactically, and we usually reserve this topic for direct coaching sessions.

Describe. "Do Describe." Parents are encouraged to watch their child's activity closely and to comment on the child's appropriate play. Specifically, Eyberg (1999) and Eyberg, Nelson, Duke, and Boggs (2005) coined the term "behavioral description" to refer to a running commentary of the child's ongoing activities. To be coded as a behavioral description, the comment has to refer directly to the child's behavior, usually including the word "you." Examples include, "You're drawing with red marker," "You put the chimney on the house," "You're sitting in the chair," and "You're looking at all the toys." In contrast, Eyberg uses terms like "information description" or "neutral talk" to refer to comments that introduce information but do not refer directly to the child's ongoing or immediately completed activity. Neutral talk includes statements such as, "I am building a bridge (parent describes own behavior)," "The doll is sleeping (does not contain the word 'you')," and "You played with the blocks last time we were here (contains 'you' but refers to the past)." We want parents to use a lot of behavioral descriptions, commenting on the child's current behavior, as these allow the parent to join in and show interest without leading the play.

To explain describing, we employ the analogy of a sportscaster broadcasting a play-by-play description of the game. For example, as a child assembles Mr. Potato Head, the parent might say “You’re looking at all the pieces. Oh, you put a green cowboy hat on Mr. Potato Head. Now you gave him a mustache. You picked the green glasses that match the green hat (child struggles to put them on). You’re trying really hard.”

Descriptive statements provide several benefits. First, if a parent is describing the child’s activity, the child is always kept in the lead. That way, the child has opportunities to come up with his or her own ideas, to problem-solve with minimal intervention, and is not rushed to keep pace with the parent. Second, continuous descriptions are a clear demonstration that the child has the parent’s undivided attention. The child does not need to whine or bang toys on the table to get a response from the parent. Undivided attention can be self-esteem building in that it communicates that the parent thinks the child’s choice of activity is interesting. Third, descriptions can be used as a teaching tool for pre-academic or early elementary school concepts. For example, parents can say, “You put one, two, three, four, five blocks on your tower,” and “You grouped all of the green beads together.” The parent can make observations about the sizes and shapes of toys and describe sorting activities. Descriptions can also be used to model and correct grammar and phonological processes. Simple descriptions of child-centered activities are particularly important for stimulating language development in young children.

A final benefit of descriptions is that they help to organize young children’s thoughts about play, increasing the length of time they are able to attend to the task-at-hand. During a running commentary, the child’s attention is much less likely to wander because each statement made by the parent maintains the child’s focus on the activity. We observe that during special playtime, young children make fewer switches between toys, and they are more likely to persist and problem-solve in the face of a challenge. See Fig. 4.1 for an example of how behavioral descriptions can increase attention to task and productivity. In the first picture, the child was told to draw a Christmas tree and the therapist simply observed quietly. Five minutes later, the therapist told the child to draw another Christmas tree but provided behavioral descriptions throughout the task. The child was more persistent, provided more detail, and stayed with the task longer when behavioral descriptions were provided. We have also observed that after several weeks of consistent Child-Directed Interaction, our clients often describe their playmates’ activities during interactive play and describe aloud their own activities during solitary play. Over time, these vocalizations diminish and we believe that children internalize this running commentary as “private speech” so that they continue to do a silent play-by-play, assisting themselves in maintaining their focus.

Be Enthusiastic! The last PRIDE skill is “Be Enthusiastic!” Parents need to display excitement during the play to make Child-Directed Interaction fun and to engage the child. Enthusiasm involves talking with an animated voice with varied inflection. This warm interactional style communicates interest and makes the playtime more enjoyable for both the parent and the child. Some of our parents,



Fig. 4.1 Child's drawing of a Christmas tree with and without the use of behavioral descriptions

particularly those who are somewhat anxious or depressed, struggle with generating the energy to sound enthusiastic during the play. Other parents, particularly fathers who have a history of playing very rambunctiously with their children, may readily pick up the enthusiastic style. Whereas with our depressed and anxious parents, we have to work hard to elicit even a small spark of excitement, we often have to reign in our rambunctious parents to prevent them from over-stimulating their children. To help parents learn to be enthusiastic, we model for them how it sounds to conduct CDI skills with and without inflection. We also encourage them to pick toys for Child-Directed Interaction that they enjoy playing with so that it is easier to be enthusiastic.

Using Strategic Attention

Before PCIT, referred families may be stuck in a negative cycle in which the child frequently misbehaves and the parent responds with negative attention. Here is an excerpt from a typical day in the life of one of our families before treatment:

Timmy is feeling a little bored so he walks over to his sister and pulls her hair. The sister screams, "Mommy!" and Timmy's mother shouts, "Stop that Timmy!" He then pushes his sister and jumps up on the couch shouting, "You can't catch me." Timmy's mother yells "Get down off the furniture. You're going to break it." Timmy jumps even more on the couch and says, "No. It's a trampoline. Bouncy, bouncy, bouncy!" Mom says, "Get down now or I'm going to tell your Dad how mean you've been all day." Timmy runs away from his mother and goes to his room. While he is in his room, Timmy gets interested in his Legos. He quietly builds spaceships, cars, and helicopters for about 20 min. During this time, Timmy's mother is relieved to have a break from his constant misbehavior. She wipes her brow and takes this rare opportunity to load the dishwasher. When she walks past Timmy's room, Timmy's mother is careful not to talk to him, as she does not want to stir

up trouble. Having been alone for 20 min, Timmy starts to crave his mother's attention. As she ignores him when he is good, he has to misbehave to get her to pay attention to him. So, when his mother walks by his door the next time, Timmy runs after her and spansks her on the back side. She responds by yelling, "That wasn't nice!" He then jumps on the couch again, chanting "Bouncy, bouncy, bouncy!" His mother again screams, "Stop that. You're going to break the couch. Do you know how much that couch cost?"

This negative cycle of the child's misbehavior followed by parental negative attention plays out repeatedly throughout the day. And, as can be expected, the exhausted parent responds to the infrequent periods of positive behavior by withdrawing attention and seizing the opportunity to take a much-needed break from the child. Unfortunately, the only way for the child to then get attention is by misbehaving once again, thereby re-activating the negative cycle.

We can interrupt this negative cycle by teaching parents to use "strategic attention." By strategic attention, we mean using the "Do" skills of Child-Directed Interaction to carefully reward the behaviors or qualities that we would like to see the child display more often. The first step in using strategic attention is to identify those behaviors or qualities that the parent sees as desirable and pro-social, even if the child rarely displays them at first. Often these desirable behaviors become more apparent as parents are encouraged to think of the opposite of problematic behaviors. For example, behaviors or qualities that might be targeted include using polite manners, making good eye contact when speaking, smiling, being gentle with the toys, using an indoor voice, persisting at difficult tasks, playing quietly while the adults talk, sharing, and taking turns.

Once these behaviors are identified, the second step in strategic attention is for the parent to be on the lookout for the targeted behavior to occur, trying to "catch the child being good." For example, "Sarah" was a loud child who played roughly with the toys, banging them on the table. Her father learned to look for even the briefest moment when Sarah was holding a toy gently or placed a toy on the table quietly. Immediately when that occurred, we coached him to say something like "You put that toy down so quietly and softly (descriptive statement). I really like it when you treat the toys nicely (labeled praise). I think I'll play gently with the toys just like you do (imitation)." By the end of the clinic CDI session, Sarah was working very hard to earn her father's praise for being gentle and she was playing with the toys with exaggerated care. Her father was able to accomplish this without ever giving her a command or making a critical statement. We encourage parents to use strategic attention whenever possible through the day, not just during the 5 min of CDI practice.

Using Selective Ignoring

Just as parental attention in the form of the PRIDE skills can be used strategically, many behaviors can be shaped by withdrawing attention strategically. We call this technique "Selective Ignoring." As soon as we bring up the technique of ignoring, most parents tell us that they have already tried it and it simply does not work with

their child. We explain to them that selective ignoring is an advanced skill that few people use effectively without special training. There is a great deal to know about what problems can be reduced with ignoring, what situations are most appropriate for the skill, and how to ignore effectively. Although we encourage parents to use selective ignoring for particular problems during CDI, we make our discussion of the technique more general so that the principles can be applied outside of special playtime.

The first step is for parents to identify child behaviors or qualities that they would like to see diminished; we write these on the chalkboard to aid in further discussion. The types of behaviors often named by parents include whining, talking back, hitting, throwing a tantrum, tearing things up, fighting with siblings, lying, being sad, nagging, and running away in public places.

Ignoring Only Works for Attention-Seeking Behaviors. Unfortunately, not all of these problem behaviors can be diminished through the use of ignoring. The first important principle for parents to understand about ignoring is that it is only effective when the function of the child's misbehavior is to elicit a reaction from the parent. We illustrate this concept by saying,

If you want your son to stop eating a cookie, will it work to turn your back to him and ignore? No, of course not. The reason he is eating the cookie is because it tastes good, not because you are watching him. If your daughter is jumping up and down on the mattress and box springs, will it work just to leave the room? No, of course not. She's probably not jumping up and down just to "get your goat"; she's probably enjoying the bouncing motion. If your son whines about wanting to go to McDonald's, could it work for you to say "We don't have time to go today" and then leave the room, ignoring any further whining? Yes it could. Children don't whine because whining is fun all by itself. I've never seen a child sitting alone in a room whining. What makes whining rewarding for young children is the reaction they get from the parent. If you consistently deprive him of that reaction (and your presence) when he whines, his whining should dramatically decrease.

After giving these examples, we go through the list of behaviors that the parents would like to see diminished. The therapist helps the parents to analyze whether or not their attention rewards the child for engaging in each of the behaviors and whether removal of attention should be expected to impact the behavior. Many of the behaviors the parents listed will not be appropriate targets for selective ignoring, and will need to be addressed later in the discipline portion of PCIT. We erase those from the chalkboard. Most often sibling conflict is not being rewarded by attention from parents, but instead by the negative attention from the sibling. Lying and stealing are not good candidates for ignoring because they are not reinforced by parental attention. Most preschoolers who lie are doing so to avoid getting into trouble for something they have done, and most young children who steal are rewarded simply by having possession of the desired object.

Ignoring Causes Behavior to Get Worse before It Gets Better. A second important principle for parents to understand about ignoring is that the behavior that is ignored will get worse before it gets better. When a child is accustomed to getting a particular reaction from the parent and one day that reaction does not come, most will respond by escalating to a more disruptive level that has a better chance of

getting the parents' attention. In deciding whether a particular problem can be diminished through ignoring, the parent must make a judgment about whether he or she can tolerate having the behavior get worse before it gets better. We ask parents to look back at the list of remaining behaviors on the chalkboard and help them to decide whether they are ones they can tolerate having get worse. It is never a good idea to ignore behaviors that could escalate and potentially be dangerous to the child or to others. For example, playfully running away in public places and physical aggression should not be ignored.

Some parents will feel able to ignore problem behaviors at home such as whining and talking back, even if they escalate. However, they may not feel prepared to ignore these behaviors when they occur in the presence of relatives or in public places. We encourage them to talk with us about why they feel unable to ignore in public settings. Many express the belief that onlookers will scrutinize them and be critical of their parenting if they simply ignore whining and talking back. Often such fears are markedly exaggerated. Later in PCIT, we will work with parents on disciplining their children in public places and it is helpful to identify early any cognitions that may interfere with follow through on behavior management in public. Some parents tell us that they cannot tolerate having any of their child's problem behaviors get worse before they get better. Such individuals are usually highly stressed and feel considerable anger toward their children. They may require special explanations and extra support to be open to using selective ignoring as a behavior management tool.

Ignoring Must Be Continued Until Child Exhibits Some Positive Behavior. The third principle parents must understand to use selective ignoring effectively is that once they begin to ignore a behavior, they must ignore it all the way through to the end. If the parents give in and reward the child with their attention (even if it is negative attention) after the child's behavior has already escalated, they will teach the child that a higher level of disruptiveness will be necessary to have the desired effect in the future. This principle is explained to parents using the "check-out line" example.

Imagine that while you are waiting in the check-out line at the grocery store, your daughter asks you nicely for a tootsie-pop. You tell her "no" because she has already had enough candy for one day. She responds by whining softly that she wants "just one, please." You decide to ignore her whining and pick up a magazine and start thumbing through it. Your daughter reacts by whining more and more loudly, becoming increasingly demanding. You notice that she has gotten the attention of people nearby and you are feeling embarrassed. You tell her in an angry tone, "Be quiet. I told you *no*." She throws a full scale temper tantrum and you buy her the tootsie-pop to calm her down. What you have accidentally taught her is that asking politely is not the way for her to get what she wants. But, she can get exactly what she wants if she is loud and obnoxious enough, particularly when you are in a public setting with lots of people around. Next time you take her to the store, you can expect her behavior to escalate even more quickly.

Parents are invited to briefly problem-solve with the therapist about alternatives. Parents need to decide right away about whether they are up to ignoring all the way through. For parents who indicate that they cannot or will not ignore throughout, we discuss the principle of "giving in early." It is better to reward the early-stage

whining than the later-stage tantrum. Given that ignoring is not a strategy for all situations nor for all parents, other discipline strategies are presented later in PCIT.

Ignore, Distract, Model the Opposite Behavior, and Praise the Opposite Behavior. During CDI, most young children are on their best behavior and display only minor problems (e.g., whining, talking back, bossiness, loud voice, rough play with toys). These can be addressed through selective ignoring of problematic behavior with strategic attention paid to incompatible behaviors. The therapist models for parents the use of strategic attention and selective ignoring in rapid succession. When the child (role-played by a parent or co-therapist) is behaving appropriately, the therapist leans toward the child, makes good eye contact, and uses the PRIDE skills (praise, reflection, imitation, description, and enthusiasm). Particular attention is paid to behaviors that are incompatible with identified problem behaviors such as using a big boy voice, asking politely, using an indoor voice, and playing gently with the toys. As soon as the child begins to whine or talk back, ignoring begins with the therapist turning in the chair to face away from the child. No further eye contact is made, no words are exchanged, facial expression stays blank (even if the child is clowning) and the therapist pretends that the child is not there. However, the therapist unobtrusively watches for the first possible moment when attention can be returned to the child, in other words when the child pauses or ceases the disruptive behavior. When that happens, the therapist swings back around in the chair, makes eye contact with the child and says something like “Thank you for playing gently with the toy. It’s so much more fun to play with you when you treat the toys nicely.”

When ignoring, we want to maximize the contrast between how the parent responds to the child when the child is behaving appropriately and how the parent responds when the child is disruptive. Exaggerating both the attention and the ignoring will help children to learn pro-social behavior more quickly. Sometimes the child’s disruptive behavior is prolonged and it is difficult to find a momentary pause during which to return attention. In those situations, we encourage parents to “ignore and distract,” which involves moving away, playing with a different toy, and enthusiastically describing their own play, but as though talking to oneself. Most often, the child will quickly cease the disruptive behavior to join the parent in the new and attractive activity. The parent then has the opportunity to provide strategic attention for appropriate behavior. Another skill used while ignoring is “modeling the opposite behavior.” For example, while ignoring his child who has begun a barnyard brawl with all of the farm animals, the father can begin to enthusiastically describe how much fun his animals are having because they are good friends who play exciting games together. It is not uncommon for the child to begin participating in the more cooperative play being modeled by the parent. The “ignoring” teaches the child what “not to do,” and the “modeling” teaches the child what “to do” instead to obtain the father’s attention. For brief and transient misbehaviors, it may not be necessary to turn away from the child. Instead, the parent may simply avert their eyes and either say nothing or describe their own play until the child is behaving appropriately again. Sometimes “Grandma’s rule” is best: “If you don’t have anything good to say, just say nothing at all.”

Handling Disruptive Behaviors That Cannot Be Ignored

Although unusual, some of the children we work with who have severe behavioral disturbances display a variety of conduct problems that cannot be ignored during special playtime because they are dangerous to the child or to others. Such behaviors include standing on top of furniture, throwing or breaking toys, hitting or biting the parent, putting small toys in their mouths, and banging on observation room mirrors. These behaviors are more apt to occur during our coaching sessions in the clinic which last up to an hour than during the very brief daily sessions at home. In both settings, we encourage the parent to intervene as needed to ensure the child's and parent's safety. At home, parents respond to aggressive or dangerous behaviors by discontinuing that day's special playtime session and disciplining the child using any safe method of their choosing. For most children, discontinuation of the play session is sufficiently punishing that repeated episodes of aggression during CDI rarely occur. In the clinic, we do not want to lose out on valuable coaching time by ending the session. Instead, when one of these behaviors occurs, we typically walk into the playroom and ask the parent to leave. We then clearly and firmly restate for the child the safety rules of our playroom. The departure of the parent and the serious voice of the therapist generally gets children's attention and interrupts the escalating behavior. After a minute or two, the parent is brought back into the room and coaching resumes. For a description of additional strategies for managing the behavior of highly aggressive and explosive children, please see Chapter 16.

Modeling Skills in Combination

While describing each of the CDI skills, the therapist briefly demonstrates the skill in isolation. However, this does not give parents an accurate picture of how the skills are used in combination. We have sometimes modeled use of the combined skills with a parent or co-therapist pretending to be the child. At other times, we have used a post-treatment videotape segment of a real parent using the skills with a child who presented with problems similar to those of the referred child, or with a child and parent of similar cultural background (written authorization to use tape for teaching purposes is required for this purpose). Social learning research tells us that modeling is most useful when the parent perceives the model to be similar to him or herself. We also find the videotape particularly helpful in that we can pause it, make observations, and review the segment again.

Role-Plays of Child-Directed Interaction

After demonstrating the combined skills, we ask parents to briefly role-play. For single parents, the therapist can ask a co-therapist to play the role of the child, or if a co-therapist is not available, the therapist can do double-duty as both the child

and the therapist-coach. In two-parent families, we ask one parent to play the part of the child while the other parent plays himself or herself. Most parents experience some performance anxiety during this first role-play. We try to lessen that anxiety by maintaining a positive, praising tone and interjecting anxiety-defusing humor whenever possible. After getting over their initial anxiety, many parents find this the most enjoyable part of the teaching session.

We recommend doing two brief role-plays of about 2 min each. In the first, the child should be perfectly behaved and present no behavior management challenge. In the second, the child should show intermittent minor disruptive behavior but behave appropriately most of the time. These role-plays should be repeated with the spouse if present. The therapist-coach should encourage parents to begin with describing, and gradually add in the other “Do” skills. If the parent is having difficulty getting started, the therapist may suggest specific phrases for the parent to repeat. After nearly every parent verbalization by the parent, the therapist-coach should immediately and quietly provide brief feedback such as “nice description,” “good reflection,” and “good labeled praise.” Because they are concentrating hard on their verbalizations, most parents will need to be gently prompted to imitate. It is not necessary to correct every error that the parents make during these role-plays; there will be plenty of opportunities to correct errors during subsequent coaching sessions. The purpose of the role-plays is to introduce the parents to how it feels to do the skills and what it is like to have someone providing them with frequent, largely positive feedback on their performance. The parents should leave the session recognizing that CDI will be a challenge, but one that is well within their grasp.

Appropriate Toys for Child-Directed Interaction

Parents should have a set of three to five toys that are always available for the child to play with during special playtime. Most parents find that their children already have several appropriate toys and they need not purchase new ones. Other parents have found it helpful to purchase two or three inexpensive toys that are put away and brought down only during special playtime. This strategy helps to preserve the novelty of the toys and adds to the child’s anticipation of daily special playtime. However, we do not advise parents to restrict the child’s access to toys that he or she already possesses, as this would (justifiably) seem unfair and detract from the goal of enhancing the parent-child relationship.

We give parents a handout summarizing the types of toys that are good for CDI and those that are to be avoided (see Appendix 4). In general, constructional toys without preset rules are best. Examples of constructional toys include Duplos (for 3–5-year-olds), Legos (for 5- or 6-year-olds), Waffle Blocks, building blocks, Tinkertoys, magnetic blocks, Lincoln Logs, Erector Sets, Mr. Potato Head, magnetic picture boards, crayons and paper, and chalkboards and colored chalk. Sets of plastic figurines and building structures are also appropriate for special playtime. Examples include farms and stables, doll houses with miniature people, and

race tracks or garages with toy cars. All of these toys encourage creativity and provide developmentally appropriate opportunities for problem-solving in the context of play. They are calm, pro-social, sit-down activities that set children up to be on their best behavior.

We caution parents to avoid toys that are conducive to rough play like bats, balls, boxing gloves, and punching bags. With these toys, children who are prone toward behavior problems become overly excited, often requiring parental interventions that take the lead away from the child and cause unpleasantness. Similarly, we encourage parents to avoid toys that set the stage for aggressive play. Such toys would include toy guns, swords, cowboys and Indians, and superhero characters. For very disruptive children, toys that are messy and can get out of hand (such as paints, scissors, and Play-Doh) should also be avoided during CDI.

Board games and card games with preset rules do not work well in special playtime. The child may be the loser in the game, have trouble taking turns, or cheat, causing unpleasantness during a time when we want to encourage parent-child bonding, not competition. Although we strongly encourage parents to read to their children daily, we discourage reading during special playtime because it interferes with spontaneous parent-child conversation. Because we want parents and children to talk directly to one another during special playtime, we advise parents to minimize the use of toys that encourage participants to pretend they are other people, speaking through their pretend characters. Examples of these are puppets, costumes, toy telephones, and dolls.

Dealing with One-Parent and Two-Parent Families

CDI is particularly helpful for one-parent families. When parents are highly stressed and overburdened with the responsibilities of single parenthood, quality playtime with children is rare. Special playtime provides a practical mechanism by which a single parent can assure spending some individual, high-quality time with the child each day. In fact, our research suggests that children from single-parent families demonstrate larger gains in self-esteem after PCIT than do children from two-parent families (Eisenstadt [Hembree-Kigin], 1990).

In two-parent families, sometimes we are asked whether the parents can alternate days conducting special playtime. Because one of the goals is to improve the quality of the parent-child relationship, we feel it is important that *each* parent do CDI *every day*. Dynamics in the marital relationship often become apparent during CDI treatment sessions. Some couples are very supportive of one another and are able to serve as effective “coaches” for each other at home. That is, they can observe one another unobtrusively and offer constructive feedback, much as the therapist-coach will do in treatment sessions. Other couples are highly critical of each other or have power imbalances such that it is best if they do not critique one another. We share with parents the potential advantages and disadvantages of observing one another at home and allow them to come to their own decisions about how they will practice. We encourage parents to be positive, constructive, and supportive of their spouses throughout PCIT.

Incorporating Siblings

We suggest that parents also do CDI at home with all of the referred child's siblings who are between the ages of 2 and 6. Children love special playtime. Directing this special attention only to the referred child often causes jealousy in siblings. Moreover, we find that siblings of the referred child often experience clinical or sub-clinical problems and may benefit from the effects of special playtime. Parents can usually generalize use of the skills from the referred child to the siblings with little difficulty. Most often, we recommend that parents begin special playtime at home with all of the children, but bring only the referred child for initial practice sessions in the clinic. If the parents are experiencing difficulty adapting their skills to the siblings, we sometimes schedule an extra session of brief coaching with the siblings. Please see Chapter 11 for more detailed information regarding sibling issues in PCIT.

Adjusting CDI to the Child's Developmental Level

When parents are doing behavioral play therapy with siblings of the referred child, they sometimes need help adapting the skills to the appropriate developmental level. The skills as described in this chapter are appropriate without modification for 4- and 5-year-old children. For children who are 2 and 3 years old, parents should expect the child to play more comfortably on the floor than at a table. The selection of toys should be adjusted to the toddler age range, using perhaps stacking rings, soft blocks, toy trucks, and push toys. Hand games like patty-cake may also be used. Toddlers change activities more frequently than do older preschoolers and a sufficient number of toys should be available to maintain the child's interest. Parent verbalizations should be shortened and simplified to provide the most effective stimulation. If the child has few words, any attempts at verbalization or sound-making should be reflected and imitated and praise should be specifically directed at verbal communication attempts. To maintain toddlers' interest, parental affect should be exaggerated, with highly animated praise and even hand-clapping (see Chapter 9). For 6- and 7-year-olds, we recommend that parents concentrate on a very natural tone, as exaggerated animation and overly enthusiastic praise will sound artificial and condescending (see Chapter 10). Reflections with older children should never sound "parroted." Instead, they should be highly elaborative without directing the topic of conversation.

Problem-Solving with Parents on Logistical Issues

Parents need to make decisions about where they will conduct their home play sessions and at what time of day they will occur. Rather than leaving parents to sort these problems out for themselves, we prefer to spend a few minutes assisting them

with their problem-solving. CDI should be conducted in a place that is quiet, private, and free of interruptions and distractions. It should not be done in a room with the television playing or with siblings intruding. Nor should it be done in the child's bedroom if there are toys in sight that would be inappropriate for special playtime but tempting to the child. Many parents find that the kitchen or dining room table works best, while others prefer playing on the floor in the parent's bedroom or a guestroom.

We recommend that the play sessions occur at about the same time each day and be incorporated into the family's daily routine. When special playtime occurs at differing times each day, young children often become anxious about missing out on their time with their parents. They can sound like a broken record, continuously nagging the parent to play with them. This situation certainly detracts from the pleasure experienced by both the parent and the child and does not contribute to relationship enhancement. Many parents use special playtime as a way to calm children down before bedtime. We find that bedtime preparations often go more smoothly when children know that special playtime always comes after brushing teeth, putting on pajamas, and so forth.

Some children have difficulty accepting the end of their playtime and try to manipulate the parent into spending extra time. If the parent wishes to extend the interaction, we recommend discontinuing the specific skills of CDI and engaging in another activity that is appealing to the child such as playing with dolls, reading a book, or tossing a ball in the backyard. In this way, the child continues to enjoy the parent's company but the parent does not "burn-out" on the play therapy skills. Another way to help children accept the end of special playtime is to schedule it at a time immediately preceding another desirable activity such as a favorite television show, snack time, or story time. Most scheduling problems occur in families with two or more young children. If it is a two-parent family, they may choose to take turns providing childcare for one another during play therapy sessions. When only one parent is available, special playtime can often be accommodated by staggering naptimes or bedtimes.

Assigning Child-Directed Interaction Homework

Each parent is asked to commit to practicing special playtime at home with their child for 5 min every day for 1 week until the next clinic session. They are reminded that they are not expected to become "play therapists" overnight. For the next few sessions, they will be coached in how to use these CDI skills. They are given a recording sheet on which to mark whether they got in their practice and the types of toys/activities involved in the playtime (see Appendix 2). For many families, this homework sheet serves as a reminder to practice, although the children are so fond of their special time that they rarely allow the parent to forget. The homework sheet also helps to make parents feel accountable, as it is reviewed with the therapist at the beginning of the next session.

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Chapter 5

Coaching Child-Directed Interaction

What to Bring. . .

- (1) ECBI
- (2) DPICS – III Coding Sheets
- (3) CDI Homework Sheets
- (4) PCIT Progress Sheet
- (5) Sheila Eyberg's Treatment Integrity Checklists/Manual

When therapists coach CDI skills, we employ the same strategies and philosophies that parents are taught to use with their children during special playtime. Therapists provide labeled praises to parents to increase particular CDI skills. We also use strategic attention and selective ignoring to increase certain parent verbalizations while decreasing others. Just as a goal of special playtime is to improve the parent-child relationship, therapists use coaching strategies that enhance rapport with the parents. For example, therapists avoid criticism when coaching, particularly the words “no,” “don’t,” “stop,” “quit,” and “not,” in order to prevent parents from feeling judged or incompetent. These negative feelings damage our relationships with the parents and lead to treatment attrition. Rather than criticizing, we enthusiastically give attention to their positive behaviors by describing and praising. When correcting the parent, we use constructive feedback telling them what “to do” rather than what “not to do.” Just as we teach parents to allow their children to lead the play, we allow parents to take the lead in their use of PRIDE skills. That is, we want parents to use their own words and develop a play style that is comfortable for them within the CDI guidelines. We only use constructive corrections when the parent is having difficulty with a particular skill. In fact, in the first CDI coaching session Sheila Eyberg (1999) discourages use of any correction at all, so as to make rapport a priority. When parents are using the CDI skills well, we follow their lead, using descriptions and praise to demonstrate acceptance. According to Dr. Eyberg, (2005) the basic principles of client-centered therapy (empathy, genuineness, and

positive regard) should guide our coaching. We want parents to leave coaching sessions feeling good about themselves, good about their child, and good about their progress in treatment.

Novice PCIT therapists can coach the basic Child-Directed Interaction skills with little or no prior experience. However, coaching is an art that continuously develops as the therapist gains experience working with parents from diverse cultural groups, with various communication styles and disparate child-rearing attitudes, and with children who present unique challenges. Although skillful coaching develops from experiences working with dysfunctional parent-child dyads, it is also grounded in an understanding of early childhood development and normative parent-child interactions. We feel it is particularly important for the PCIT therapist to develop and maintain an “internal barometer” for the wide range of interaction styles and communication patterns that characterize healthy, nurturing parent-child relationships. In this way, the therapist will broaden his or her repertoire of coaching strategies and reduce the tendency to develop professional “myopia,” in which similar interaction sequences are coached in all families, without regard to the family’s unique communication strengths and style.

Overview of a Typical Coaching Session

Table 5.1 presents the steps involved in typical coaching sessions for families in which one or both parents are participating. Upon arrival to each PCIT session, parents complete the ECBI Intensity Scale in the waiting area. The therapist quickly tallies the score and records it on the PCIT Progress Sheet (see Appendix 5) and provides feedback to the parent about changes over time, often using a graph to visually display the changes in ECBI scores. The session begins with a review of the homework. After problem-solving issues that arise with the homework and inquiring about other familial stressors, we observe the parent conducting a 5-min play therapy session with the child, without any direct coaching. Parental use of CDI

Table 5.1 Steps for conducting a Child-Directed Interaction coaching session

<i>One parent participating</i>		
Step 1	Check-in and review of homework	10 min
Step 2	Coding of CDI skills	5 min
Step 3	Coaching of CDI skills	35 min
Step 4	Feedback on progress and homework assignment	10 min
<i>Two parents participating</i>		
Step 1	Check-in and review of homework	10 min
Step 2	Coding of first parent’s CDI skills	5 min
Step 3	Coaching of first parent’s CDI skills	15 min
Step 4	Coding of second parent’s CDI skills	5 min
Step 5	Coaching of second parent’s CDI skills	15 min
Step 6	Feedback on progress and homework assignment	10 min

skills during these 5 min is recorded on a Dyadic Parent-Child Interaction Coding System (DPICS – III) recording sheet (see Appendix 1) and later transferred to the PCIT Progress Sheet (see Appendix 5) so that parents can view session-to-session changes. After this 5-min observation period, the parent is directly coached by the therapist while continuing to practice the PRIDE skills with the child. For two-parent families, the coaching session is divided in half so that each parent receives coaching. The parent who is not being coached learns through observation and is often taught to code from behind the mirror. The observing parent should be quiet so as not to interrupt the coaching. The last 10 min of the session is spent providing parents with feedback on their progress (see Appendix 5 for PCIT Progress Sheet) and identifying areas that should receive special focus during the next week's home practice. The therapist may choose to reserve an additional few minutes at the end of each coaching session for individual rapport-building as needed. This individual time can decrease resistance to therapy by encouraging children to view the therapist as an ally rather than as a conspirator with the parents. Although the number of CDI coaching sessions will vary based on how quickly parents master the skills, the basic steps outlined in this chapter are used in each coaching session.

Setting Up for the Coaching Session

The parent and child meet with the therapist in a childproofed playroom equipped with a table, chairs, and three to five toys that are appropriate for special playtime (e.g., creative, construction oriented). The parent and child often play on the floor during CDI, with the parent following the child around the room as the child plays with the toys that are available. However, it is up to the child to choose whether to play on the floor or at the table. For example, if the child chooses to color at the table, the parent should sit at the table as well. A few minutes later, the child may choose to drive cars on the carpet and the parent should move to the floor to join in the play. Toys that are inappropriate for CDI should be removed from the room to avoid the unpleasantness that may occur if the child insists on playing with an inappropriate toy. Because parents will be asked to avoid limit setting during CDI, the playroom should contain no items that may inspire the child to misbehave and require parental intervention. In our playroom, we do not include lamps, glass framed pictures, nicely upholstered furniture, sinks, boxes of tissues, or personal items such as handbags. Light switches are kept in the “on” position using lockable covers or tape.

If the therapist will be coaching via a bug-in-the-ear microphone device, the earpiece should be sterilized with an alcohol wipe and tested prior to the start of the therapy session. Additional materials that will be needed during each session are as follows: one DPICS – III coding sheet for each parent, one homework sheet for each parent, one PCIT Progress Sheet for each parent, and a clock or stopwatch.

Check-In and Homework Review

The session typically begins with the child playing independently nearby while the parent and therapist review the child's home and school adjustment during the previous week, discuss familial stressors unrelated to the child's behavior, and review the week's homework practice. We ask parents to bring in a homework sheet each week indicating whether or not they were able to practice each day and noting any questions, observations, or concerns they had during the course of the week. Because one of the goals for the CDI stage of PCIT is for parents to become more adept at recognizing and praising their child's positive qualities and behaviors, we are careful to prompt parents to note progress and accomplishments by the child, not just problems. We also use this check-in period as an opportunity to teach parents to shape independent play by giving their child intermittent labeled praises for playing quietly while the adults talk.

In order to maximize the amount of time spent in direct coaching of CDI, we restrict this initial "check-in" to 5–10 min. Occasionally, the parents we work with have difficulty sticking to this time limit or bring in concerns about important marital or individual issues. If this occurs on a consistent basis, diverting focus away from the parent training intervention and slowing PCIT treatment progress, we recommend inviting parents to participate in adjunctive interventions such as individual treatment, support groups, or marital therapy. Thus, important concurrent issues may be addressed in a planned manner often enhancing the effectiveness of PCIT. With some parents who tend to offer overly lengthy and detailed descriptions of their child's misbehavior, we choose to sequence the session so that this check-in period is saved for the last 10 min of the session. This limits non-productive focus on child misbehavior both by decreasing the time available for it and by inviting parents to review child behavior only after they have been coached to focus on their child's positive attributes.

During the first CDI coaching session, the check-in period should include a brief review of the "Do" and "Avoid" skills. Most parents feel quite self-conscious about performing these new skills in front of the therapist. It is helpful to directly address this anxiety, letting parents know that it is a common experience that will quickly pass, and reminding them that the therapist does not expect them to be "masters" of play therapy after practicing it for only 1 week. Finally, the check-in period during the first CDI coaching session should be concluded with a developmentally appropriate explanation of the coaching process for the child. If the therapist-coach will be recording and coaching the skills from an observation room and the child is old enough to perceive that the parent is receiving instructions over the bug-in-the-ear, the following explanation might be given:

It's time for me to leave now so you can have special playtime with your mom (dad). But, I'm going to watch you and your mom (dad) play. I'll be watching from behind that mirror. Do you want to see? [Allow child to enter observation room and briefly view the playroom]. I'm going to help your mom (dad) learn to play in a special way. Sometimes I might say things that she (he) will hear in that funny thing in her (his) ear. That thing is not a toy. You can look at it but you can't play with it. Your job is to just play along with your mom (dad) and have fun, OK?

If the therapist-coach will be recording and coaching from within the playroom, the child might be told something like:

It's time for you to have special playtime with your mom (dad) now. I'm going to stay here and watch you and your mom (dad) play. My job is to help your mom (dad) learn to play with you in a very special way. Sometimes I will watch quietly and write things down, and sometimes I will say some things to your mom (dad). Your job is to keep playing and pretend like I'm not even here, like I'm invisible! That means you don't look at me or talk to me. You just play with your mom (dad) and pretend like I'm not here, OK?

Both of these explanations should be adapted to fit the cognitive and language development of the individual child, and some therapist-coaches may choose to have the parent repeat the explanation in their own words to enhance the child's understanding. If coaching from within the room, some children will have initial difficulty remembering not to interact with the therapist. The first time this occurs, the therapist should remind the child to pretend that the therapist is not there and subsequently the therapist should completely ignore any further overtures from the child. Most children will quickly learn to tune out the therapist's coaching and to attend to the play with the parent. If the therapist continues to respond to the child's overtures, the latter will become more frequent and coaching will be compromised.

Parental Non-compliance with CDI Homework

Although parents often leave the early CDI sessions with the best of intentions to complete their daily homework, we find that the majority of parents have great difficulty getting their homework done on a consistent basis. Therapists should expect homework non-compliance and be proactive about problem-solving homework issues. Because clinic improvements will not readily generalize to the home without practice, both therapists and parents must view homework as a critical element of the treatment. We recognize that it is rare for families to be able to complete 100% of the assigned homework. And, we find that many families can progress well through treatment if they complete most of their homework. When parents complete homework fewer than 3 times per week, we become seriously concerned that treatment may not progress. In those cases, we analyze the possible reasons for the homework non-compliance and employ strategies to correct the problem. Table 5.2 provides four common functions of homework non-compliance and associated remedies.

Parent Does Not "Buy In" to CDI. Some families enter treatment more motivated for CDI than others. Our highly educated parents are typically convinced easily of the potential benefits of CDI. In contrast, our court-ordered, school-referred, and less educated families tend to be harder to persuade. Homework non-compliance may be an early indicator of treatment resistance in these families. We find it helpful to address the resistance directly. We might say, "I'm sensing that you don't really believe that special playtime is going to make any difference." This opens the door for parents to directly discuss skepticism and provides us with an opportunity to further "sell" CDI. As discussed in Chapter 3, five points to emphasize when

Table 5.2 Functions of homework non-compliance solutions

1. Parent does not “buy in” to CDI	1. Put the issue on the table 2. “Sell” CDI again (see Chapter 3) 3. Introduce idea of an “experiment”
2. Parent is too stressed and disorganized to make homework a priority	1. Give them a folder 2. Night-before reminder call 3. Give them a physical reminder for refrigerator 4. Mid-week reminder call 5. Incentives 6. Help them develop a routine for CDI
3. Therapist has not sent a consistent message that homework should be a priority	1. Avoid inadvertently reinforcing non-compliance with supportive statements such as “It’s okay. I can see it was a tough week” 2. Consistently pick up homework sheet with ECBI, making homework sheet a “ticket” to the session 3. Give labeled praises for remembering homework sheet 4. Give labeled praises for completing most of the homework (e.g., 4 out of 7 days) 5. Require parents to re-create the homework sheet if it is forgotten 6. Repeatedly educate the parents about the importance of homework and attribute child changes to home practice (or lack thereof)
4. Parent practice is being sabotaged by others in the home	1. Attempt to engage the significant others in therapy 2. Problem-solve ways for parent to practice with privacy 3. Empower the parent to be assertive with others 4. Educate parent that others who have been criticized for CDI practice have found ways to complete homework 5. Forecast that significant others will stop sabotaging when they see the treatment work

“selling” CDI are (1) the parent must have a strong relationship with the child for the intensive discipline program to work, (2) daily practice leads to faster mastery of CDI so that the family progresses to the discipline program more quickly, (3) CDI is “therapy” not just play, (4) having a short daily connection with the child adds up and leads to the child wanting to please the parent, and (5) by practicing each day, the parent over-learns important behavior management skills that become habits that occur naturally throughout the day. For parents who remain resistant even after receiving the five “selling points” above, we encourage parents to think of CDI practice as an “experiment.” As part of the experiment, we have the parent generate the number of CDI practices that they are willing to commit to for the upcoming week. We write the agreed upon number on the top of the homework sheet and introduce the experiment in the following way:

So you think you can get in 4 times this week. Is that a realistic number? Are you able to commit to that for this week only? Great. Then, when I see you next week, the first thing that I will ask you is whether you were able to do *your* part of the experiment. I will ask you whether you did special playtime 4 times during the week. It is important for you to get in all 4 practices so that we give the play therapy a chance to work. Together we will look at whether the practice time led to any good changes in your child's behavior, your relationship with your child, or your own skills as a play therapist.

We use this "experiment" as a way of shaping homework behavior. If we get the parent to do 1 week of homework, then we have a foot in the door. We can praise the parent for the accomplishment and make observations about how it is helping.

Parent Is Too Stressed and Disorganized to Make Homework a Priority. We find that many of our multi-stressed, disorganized families are sufficiently sold on the merits of CDI but have just been unsuccessful at making it happen at home. These families lack routine, are often just trying to get through their days, are responding to crises, and feel overwhelmed by the addition of one more task. When we recognize a family as disorganized and stressed, we often give them a folder at the beginning of treatment. We tell them to put all of their handouts and homework sheets in this folder. We also help them pick one place in the home to keep the folder, and we emphasize that they need to bring this folder to every session. When possible, we instruct our staff to provide the family with a reminder call the day before their next session. In the call, the family is reminded about the time of the session, who should come to the session, and the need to bring the folder. Sometimes these families benefit from posting a visual reminder to practice special playtime at home. Instead of expecting them to generate the reminder, we may hand them a sign to post on a wall, the door, or the refrigerator. Some therapists may choose to give the family a reminder call midway through the week to get them going on the homework and make them feel more accountable. Finally, therapists and/or agencies may choose to implement an incentive program for homework practice. Examples include the following: (1) collecting a deposit early in treatment that is refunded as parents practice homework, (2) allowing the child to select something from the "homework treat box" whenever a sufficient amount of homework was completed, and (3) awarding raffle tickets for larger prizes based on successful homework practice.

Therapist Has Not Sent a Consistent Message That Homework Should Be a Priority. When training to be mental health professionals, we are taught to be supportive and client-centered with an emphasis on following the client's lead in sessions. We have found that this supportive approach can sometimes undermine our message that homework is critical for treatment progress. When our multi-stressed families present with crises, we can be easily derailed by focusing our efforts on providing support. It is not uncommon for therapists to use active listening, empathic responding, and questioning to encourage parents to talk more about the weekly crises. We often respond by saying, "You've had a really rough week," "You've got a lot on your plate," "What did you do when your ex-husband did not return her on time?" and "What did you say to the teacher when she called you?" Although it is our job to be supportive, we must be careful not to inadvertently send a message to

parents that homework is not very important. For example, in the midst of providing supportive statements and inquiries about crises, we can easily find ourselves halfway through the session before we ever ask about homework. And, sometimes we get so caught up in the crises that we forget to ask for homework at all. At other times, parents report to us that they were unable to do their homework because of stressful life events (e.g., death of a grandparent, a Child Protective Services report, overtime at work, a sick child, out-of-town visitors). Our training in supportive therapy leads us to respond by saying, "That's okay. It was a tough week." Yet, with multi-problem families, *every* week is tough. If PCIT is to progress, we have to avoid giving these families permission to not do their homework because of stressful life events. As good clinicians, we work hard every session to maintain balance between providing support and making it clear to families that we expect them to do their daily homework.

To ensure that we communicate to parents that homework is a priority, we can employ several strategies. First, just as we collect an ECBI from the parents before they enter the session, we can also collect their homework sheet up front. In this way, we can make the homework sheet a sort of "ticket" to the session. Consistently asking for the homework sheet prior to the session has two benefits: (1) it increases the chance that the therapist will remember to ask for the homework, and (2) it sends a message to the client that daily practice is so important that we do not even begin the session without examining the homework sheet. If the parents turn in their homework, they should receive labeled praise for remembering the sheet, regardless of how many times they actually practiced at home. If the parents forget to bring the homework paper, the therapist should require them to recreate the homework sheet in the waiting area prior to the beginning of the session. Completing the homework sheet in the waiting area is aversive to parents because it postpones their access to the therapist and the supportive aspects of the treatment. During sessions, we repeatedly educate parents about the importance of daily practice. We teach them that the 5 min per day of special playtime is critical for (1) the development of their parenting skills, (2) improvement in the parent-child relationship, and (3) generalization of child behavior improvements from the clinic to the home setting. To help parents perceive the link between homework practice and treatment progress, we review ECBI and DPICS results. Behavioral improvements reported on the ECBI and skill improvements coded on the DPICS are directly attributed to how well parents have followed through on their homework. When progress is slow, parents are educated about the need for them to increase homework completion in order to speed up treatment gains. Finally, in those cases in which parents actually succeed in completing all or most of their homework, we make it a point to provide labeled praise for their efforts.

Parent Practice Is Being Sabotaged. Many parents tell us that it is hard to complete homework because significant others in the home observe and interfere. These significant others usually include spouses and extended family members, like grandparents, who are not participating in PCIT. Examples of interference include interrupting, showing non-verbal disapproval (e.g., shaking head, rolling eyes), inducing guilt ("Why are you wasting time playing instead of making

dinner?”), and using blatant criticism (“You’re stupid if you think this is going to do any good.”). If we do not give parents specific strategies for dealing with interference from family members, there is a good chance that the participating parent will discontinue homework, hampering treatment progress. Our efforts to deal with sabotage include the following: (1) attempt to engage the significant others in therapy, (2) problem-solve ways for parent to practice with privacy, (3) empower the parent to be assertive with others, (4) educate the parent that other clients have encountered the same types of interference and still have found ways to complete their homework, and (5) forecast that significant others will stop sabotaging when they see the treatment work.

Observing and Recording Child-Directed Interaction Skills

As mentioned earlier, we devote a brief period of time at the beginning of each session to recording parental skills progress. This allows us to closely monitor the effectiveness of our previous coaching, provides us with objective information that can be charted and shared with interested parents, and supplies us with information about what skills should receive particular focus during the subsequent coaching.

We get the most accurate picture of how parents are performing their skills at home when we conduct our recording period early in the session, before doing any coaching. If recording is done at the end of the session, after several minutes of skills coaching, nearly all parents are able to perform at a high skill level. However, this performance is artificially enhanced by short-term retention and typically is uncharacteristic of how parents perform independently in home play therapy sessions throughout the week.

We begin the recording period by telling parents:

I would like for you to go ahead and begin special playtime now. I’ll just watch you for 5 min and make some notes to myself before I jump in and begin coaching, OK? Show me your best CDI skills.

We then allow a minute or so to go by so that the parents may warm up and let any initial nervousness subside as they devote their full attention to their child. We begin timing for 5 min and record tally marks in the appropriate boxes on the DPICS – III recording form. At the end of the 5 min, we take a minute or so to make notes about qualitative aspects of the interaction that we would like to address in the coaching or discuss with the parent at the end of the session. We then quickly transfer the data from the recording sheet to the parent’s PCIT Progress Sheet. This form makes it easy for the therapist to track the family’s week-to-week progress.

Immediately after the 5 min coding, we find it helpful to provide the parent with a “constructive feedback sandwich.” The feedback sandwich consists of a hefty slice of labeled praise, followed by a delicately sliced suggestion regarding what the parent could do even better, and finished with another substantial slice of labeled praise.

For example, the therapist might say, “You did a great job of increasing your reflections this week. You went from three to eight. And congratulations, you met mastery on behavioral descriptions with 12 of those. The one thing that you might want to focus on is increasing your labeled praises. I counted four and you need 10 for mastery. But overall, I thought that your play was warm, and fun, and you did a good job of letting Sasha lead the play.”

The skill progress information we collect also helps us to determine how close the family has come to meeting a pre-determined set of criteria for mastery of CDI skills and progressing to the discipline portion of PCIT. The “gold standard” for mastery of CDI skills established by Eyberg (www.pcit.org) is presented in Table 5.3. Because the mastery criteria involve using 10 each of labeled praises, reflections, and behavioral descriptions, when talking with parents we often refer to the mastery criteria as the “ten–ten–ten.” It should be noted that the criteria presented in Table 5.3 were established based on the concept of “over-learning.” We know that after treatment is concluded and parents no longer receive weekly coaching, their CDI skills will backslide. However, if they have over-learned the skills, we expect that their skills will still be sufficient to maintain the child’s positive behavior over time, even if some backsliding occurs. Over-learning also is important because it enhances generalization outside of the playtime. A goal is for the positive parenting skills to become over-learned habits that occur effortlessly throughout the day. For example, when the child tells an elaborate story in the car on the way home from school, we hope that the parent will automatically provide a reflection of the content. Or, when the two children in the family are playing amiably together in the living room, our goal is for the parent to reflexively provide a labeled praise. It is the over-practicing and over-learning of skills during playtime sessions that lead to the spontaneous use of these skills throughout the day.

Table 5.3 Criteria for mastery of Child-Directed Interaction skills during a 5-min play session

10 Labeled praises
10 Reflections
10 Behavioral descriptions
3 or fewer commands + questions + negative talk (criticism and sarcasm)
Ignore all negative attention-seeking behaviors
Imitate the child’s play
Be enthusiastic

Coaching the “Do” and “Avoid” Skills: Tips for Therapists

Skillful coaching of the parent-child interactions requires that the therapist-coach provide frequent, specific feedback to parents while not disrupting the natural flow of the interaction. That is a tall order for novice therapists who feel awkward sandwiching their comments between parent and child verbalizations. The following general principles are important for effective skills coaching.

Make Coaching Brief, Fast, and Precise. The best coaching statements contain few words. Full sentences and lengthy explanations interrupt the flow of the interaction and may cause parents to become flustered as they attempt to divide their attention between the therapist-coach and their child. Not only should the coaching statement contain few words, it should also be fast in that it should be delivered immediately after the parent’s verbalization. Because every word must count, the language used should be precise rather than general or vague. Occasionally, a situation will arise in which the therapist-coach needs to provide a longer explanation or observation. In those rare situations, the coach could ask the parent to allow the child to play independently for a moment while the coach provides feedback. Situations in which we have done this include times when a parent is not responding to our coaching (e.g., remains flat for 10 min despite intensive coaching on enthusiasm) and when we are providing instructions for a special exercise (e.g., praise exercise). Another situation in which we have taken a moment to talk to a parent in more detail is one in which a parent becomes emotional during the coaching. For example, we worked with a mother who was so touched by a picture her child drew that she became tearful. Her son, who had seldom seen her cry, became worried that she was hurt or that something bad had happened. The mother became flustered and did not know how to proceed with the special playtime. We talked with her for just a moment while the child played, giving her suggestions for how to explain “happy tears” to her son. Yet, the overwhelming majority of coaching should be brief so that it promotes rather than interferes with rapid skill acquisition. The coaching statements may take the form of labeled praises, gentle corrections, directives, and observations. Table 5.4 presents examples of commonly used coaching statements in each of these four categories.

Coach After Nearly Every Parent Verbalization. Every verbalization the parent makes provides the therapist-coach with an opportunity to teach, and the more input the parent receives, the faster and better the skills will be learned. Also, by providing feedback after each verbalization, parents learn to pause and wait for therapist input. Coaching will proceed more smoothly when the therapist and parent develop this type of pacing. Providing intensive feedback requires that the therapist think quickly and react with an appropriate labeled praise, gentle correction, observation, or direction. For novice therapists (and even very experienced ones!), this requires intense concentration and sustained effort, which can be exhausting. Therapists must resist the inclination to reduce the frequency of their feedback or to coach in a mechanical fashion.

Give More Praise than Correction. Many parents begin therapy feeling incompetent in their parenting roles. It is critical for good outcome in PCIT that parents feel supported and successful from the outset. For that reason, the therapist-coach must stay in tune with the proportion of praise to correction being provided.

Most parents correctly perform many of the skills from the beginning, providing natural opportunities for the therapist-coach to provide a preponderance of labeled praises. If parents are not producing descriptions, reflections, and praises on their own, the therapist should use directives to get the parent to make

Table 5.4 Common Child-Directed Interaction coaching statements

<i>Labeled praises</i>	
Good imitation	Nice physical praise
I like how you're ignoring now	Good description
Great job of following his lead	Good answering his question
Good encouraging his creativity	Great teaching!
Nice timing on giving him back your attention	Terrific enthusiasm!
Nice eye contact	Nice labeled praise
<i>Gentle corrections</i>	
Oops, a question!	Sounds a little critical
Looks like a frown	Was that a command?
A little leading	Might be better to say. . .
You're getting a little ahead of her now	
<i>Directives</i>	
Try to label it	Can you reflect that?
Say "Nice manners!"	More enthusiasm!
Say it again, but drop your voice at the end	Let's ignore until he does something neutral or positive
Say "I like it when you use your big girl voice"	Say "It's so much fun to play with you when you're careful with the toys"
Praise her for sharing	
What can you praise now?	How about a hug with that praise?
<i>Observations</i>	
He's enjoying this	Sounds very genuine
He's sitting nicely now	Now he's imitating you
She wants to please you	He loves that praise
He's talking more now because you're reflecting	She's handling frustration a little better now
She's staying with it longer because of your descriptions	There's a big self-esteem smile!
That praise is good for her self-esteem	You see, anything you praise will increase
	By saying "I'm sorry" you just set a good example for polite manners
That's good practice for fine motor skills	

particular statements, followed by labeled praises after the statements are made, and observations concerning the child's responses. For example:

Parent: (watches child build but does not speak)
 Therapist: (gives directive) "Say, 'Good idea to make a zoo!'"
 Parent: "I like that zoo you're building!"
 Therapist: (gives labeled praise) "Nice labeled praise. (makes two observations)
 She really lights up when you praise her. She's working even harder now."

Although it is important to provide feedback as frequently as possible, it is not wise to correct every mistake the parent makes, particularly early in treatment when errors are frequent. Correcting every mistake, even if done in a gentle way, can tip the scale in the negative direction, causing a parent to feel criticized, inept, and discouraged. We recommend that therapist coaches strive for a ratio of at least 5

supportive statements for every correction. An alternative to corrections is the use of selective ignoring for incorrect skill use, followed by strategic attention when the skill is used properly. The following is an example:

Parent: “What do you want to do now?”
Therapist: (selectively ignores question)
Parent: “Are you pretending to take the dog for a walk?”
Therapist: (selectively ignores question)
Parent: “Your dog is going for a walk.”
Therapist: (provides strategic attention) “Terrific description! You said it as a statement. Good job reducing those questions.”

After the first coaching session, most parents are performing so many skills correctly that most of the errors can be gently corrected while still maintaining the overall positive tone of the coaching.

Coach Easier Skills Before Harder Ones. Some of the “Do” and “Avoid” skills are generally easier to learn than others, and parents are more likely to feel immediate success if more focus is placed on the easier skills initially. In our experience, describing is typically the easiest of the CDI skills, followed by imitating, reflecting, avoiding criticism, and avoiding commands. The skills that appear to be most difficult for parents to master are avoiding questions and giving praise. We believe that eliminating questions is particularly difficult because of the very high rate of questions most parents give young children at baseline. Asking questions is a difficult habit to break. For some parents, praising is difficult because they are not comfortable expressing affection verbally. Others may believe that too much praise will spoil their child or cause him or her to become boastful. Many parents resist praising because they are caught up in a coercive cycle in which they do not want to praise during special playtime if the child has displayed disruptive behavior earlier in the day. Still other parents simply have difficulty identifying their child’s positive and praiseworthy qualities and behaviors. Most parents find that praise comes more easily and naturally after they have been practicing play therapy for a couple of weeks and have been coached on praise for one or two sessions. If the parent continues to experience difficulty generating praise, we recommend processing this issue with the parent in detail.

Use Special Exercises for Difficult Skills. When the parent is performing many skills at the desired rate but one skill appears to be lagging well behind, we may interrupt the CDI to conduct special exercises in which the parent is encouraged to concentrate on the particular skill. For example, we may tell the parent, “I want to try a little experiment. I want to see how many times in the next minute you can praise Katie, OK? Are you ready? Now begin.” During that minute, we stop coaching other skills, and count aloud for the parent the number of praises given. For example,

Good, there’s one... that’s two... three... now you’re really going... think of another one... four... time is up. That was fantastic! You gave 4 praises in only one minute when you really concentrated on it. I knew you could do it. If you kept up that pace you would have 20 in 5 min, that’s 10 more than you need for mastery. Well done!

An exercise such as this one provides encouragement and incentive as well as good practice for parents who are struggling with a particular skill. It is often a better strategy than continuing to provide frequent corrective feedback which can become disheartening for the parent. Other exercises that help parents to focus on particular skills include (1) asking parents to reflect everything appropriate the child says in a 2-min time period, (2) asking parents to catch every question they ask and restate it as a description or reflection, (3) asking parents to turn unlabeled praises into labeled praises, (4) asking parents to practice alternately dropping and raising the inflection of their voices to make a phrase a statement or a question, and (5) giving parents the assignment to be “extra silly” and excited for the next 3 min to promote enthusiasm.

Use Observations to Highlight Effects. Often, we find that abstract discussions of how children respond positively and negatively to particular communications from parents are not sufficiently potent teaching tools. Many times, it is not until the parent actually sees it demonstrated during a coaching session that they are able to recognize and strategically alter their communication patterns to elicit desirable child responses. Therefore, in addition to coaching parental use of “Do” and “Avoid” skills, the therapist-coach should comment on the ways in which the child is responding to the parent. For example, if the parent praises the child for putting the red blocks together and then the child reaches for another red block, the therapist-coach may state an observation such as, “Your praise is powerful. Whatever you praise him for, he’ll probably do again.” Similarly, after the parent reflects the child’s verbalization and the child speaks again, elaborating on the same topic, the therapist-coach may make an observation such as “You’ve given him positive attention for talking to you without taking his lead away, so he’ll keep the conversation going.” Because observations can be wordy and may interrupt the flow of the interaction, they should be used strategically. If a particular observation is lengthy or requires an extended discussion, we may choose to review our observations with the parent at the end of the coaching period.

The therapist-coach may also make observations about the child’s negative responses to less desirable parental verbalizations and behaviors. For example, if a parent’s “imitating” turns into the building of a far more elaborate structure than the one the child is making (despite warnings about this pitfall during the teaching session), the child may be expected to show any of several unfavorable responses: losing interest in the activity and leaving the parent to play with another toy; making negative comments about his or her own ability; or expressing frustration by damaging the parent’s structure. Rather than coaching the parent early in the sequence to tone down the complexity of the building, it is sometimes more instructional to allow the parent to continue and the child to respond unfavorably, and then help the parent to recognize how he or she precipitated this negative child response. In this situation, the therapist-coach might offer an observation such as “He’s showing you that your building was too advanced for him and took away his chance to lead the play.”

One of our goals in PCIT is to help parents improve their attitudes toward their children. One way that this can be accomplished is by pointing out to the parent good

qualities about the child. During coaching, we frequently comment on the child’s appearance, manners, intelligence, creativity, curiosity, sense of humor, problem-solving ability, building skills, speed, artistic prowess, and attire. Early in this book we recounted how we often have parents tell us that they love their children but they just do not like them anymore. When parents have given up on finding the good in their children, it is our job to train their eyes to see the positive qualities that we see. We look hard for improvements in the child’s behavior and share those observations with parents. We make it a point to comment on how parents are responsible for these improvements. For example, we might say, “He’s sharing much more this week. That is because you have been praising sharing.” We find that if we do not show parents the direct link between their changes in parenting and their children’s behavioral improvements, they often credit the child’s changes to extraneous factors, such as sleeping, eating, allergies, the toys in the room, and the phase of the moon. Observations can help parents feel proud of their children and take responsibility for their children’s behavioral improvements.

Make Use of Humor. Although coaching and learning Child-Directed Interaction is hard work for both the therapist-coach and the parent, it need not be an overly serious and formal process. In healthy parent-child interactions, most parents and children relax, laugh, and find humor in their activities and interactions. We find that the session is much more enjoyable for all involved if the therapist makes use of humor for reducing parental performance anxiety and helping to increase the warmth of the parent-child interaction.

Progress from More Directive to Less Directive Coaching. A goal of CDI coaching is to empower parents to use the skills autonomously. This can be accomplished by gradually reducing the use of directives and corrections as parents display increased mastery of play therapy skills. For example, in the beginning of a first CDI coaching session, the therapist may need to give parents the exact words for labeled praises. As the session progresses, the therapist may only need to provide a brief prompt, such as “How about a praise?” Toward the end of the session, the parent may have developed the ability to generate his or her own praises. When this happens, the sensitive therapist-coach will step back and simply reinforce the parent’s good use of praise and provide observations on its effects. Once parents near mastery of CDI skills, the therapist should rarely need to provide directives or offer suggestions for the words parents say. Toward the end of CDI, the coaching basically sounds like this: “Nice job. You’re so good at this. . . . You’ve got it. Just keep going. . . . Beautiful reflection. . . . She’s smiling!. . . . Your praises are so warm.”

Coaching Strategic Attention and Selective Ignoring. To maximize the effectiveness of Child-Directed Interaction, parents must understand the concepts of strategic attention and selective ignoring described in Chapter 4, and they must be able to implement them in tandem to shape desirable child behaviors. The therapist-coach should look for child behaviors that are pro-social, occur with low frequency, and are appropriate targets to increase through strategic attention. Often these behaviors are naturally incompatible with identified problematic behaviors. For example, a child who is bossy may have “asking politely” as a target of strategic attention. Using the double-pronged approach, bossiness in turn may be identified as a target

for decrease through selective ignoring. Examples of problematic behaviors responsive to selective ignoring and their incompatible pro-social behaviors that may be increased through strategic attention are presented in Table 5.5.

Table 5.5 Behavioral targets for strategic attention and selective ignoring

Strategically attend to. . .	Selectively ignore. . .
Polite manners	Bossiness, demandingness
Playing gently with the toys	Banging toy on the table
Using a “big boy (girl)” voice	Whining
Talking softly	Yelling
Driving toy cars safely	Repeatedly wrecking cars
Being nice to toy people	Dropping people on floor
Sharing toys	Grabbing toys away
Building pro-social structures	Making toy guns
Trying even when it is hard	Giving up in frustration

When an appropriate target for selective ignoring is presented during the coaching session, the therapist-coach first identifies the problematic behavior, coaches the parent in selective ignoring until the child ceases the problematic behavior, coaches the parent to return attention to the child for positive or neutral behaviors, and coaches the parent to keep an eye out for pro-social behaviors (which are incompatible with the problem behavior) that can be responded to with strategic praise. The following example illustrates the use of selective ignoring and strategic attention in tandem.

Child:	“Pow, pow, pow. You’re all dead.” (mimics shooting Lego people with a Tinkertoy gun he has made)
Therapist (to the parent):	“That’s aggressive. Now is a good time to begin ignoring. Drop your eyes, quickly turn away, and begin building something of your own with some Tinkertoys. Describe out loud what you are making, but speak as though you’re just talking to yourself, not to him.
Parent:	(turns away from child and picks up wheels) “I think I’m going to build a swamp buggy. Here’s one wheel. . .”
Child:	(louder this time) “Look mom, I’m killing all of them! Pow, pow.”
Therapist:	“Great job of ignoring. Keep looking away. Good describing your own play. Let’s see if we can get him interested in what you are doing so he stops the shooting. Be very enthusiastic about your buggy.”
Parent:	“I’m going to make the coolest, baddest, freshest swamp buggy in the whole world! It’s going to have red wheels. Now, I’m going to put a green seat here. I guess I’d better find a driver for my swamp buggy.”

- Child: “Oh, I know, this Lego-man can drive it! Here, I’ll show you.”
- Therapist: “Perfect! You got his attention away from the aggressive play and now he’s playing appropriately with you. Let’s give him your full attention now and some labeled praise.”
- Parent: (turns to face child) “What a great idea to have the Lego-man drive! Thanks for playing nicely with the toys so I can play with you again.”
- Therapist: “Nice labeled praise. You did a great job of getting him back on track.”
- Parent: “Now you’re adding a back seat so more people can ride.”
- Therapist: “Good describing.”
- Parent: “I’m really glad you’re playing swamp buggy with me. I like gentle play.”
- Therapist: “Excellent labeled praise.”

Sometimes, during selective ignoring, parents will try to speed up the process by trying to coax children to re-engage in CDI. This looks like the following: While the child is pounding aggressively on the dollhouse, the parent selectively ignores the pounding and starts talking out loud about how they like to play gently with the Tinkertoys (modeling opposite behavior). When the child does not discontinue the pounding immediately, the parent rushes the process by saying, “I sure wish that Freddie would come over here and play gently with the Tinkertoys.” This verbalization breaks two of the CDI rules. First, it provides attention to Freddie for his disruptive behavior. And, second, it is an indirect command, making it hard for Freddie to lead the play. We coach parents to be patient and let the selective ignoring work. Parents can combine ignoring with distraction in which they enthusiastically describe their own play activity as though talking to themselves, rather than to the child. But, we do not want parents to use any form of distraction that involves looking at the child, addressing the child by name, or providing either direct or indirect commands.

There are times early in CDI coaching when children have extended tantrums and parents must ignore for up to 20 min. During the ignoring, parents who are not yet fully invested in treatment will give the coach non-verbal cues that they do not approve of this strategy. They roll their eyes, sigh, raise their hands in frustration, look into the observation window skeptically, and sometimes even say out loud “This isn’t working people.” If the therapist wants these families to return to the next session, it is important to stay confident and use motivational strategies during the extended period of ignoring the tantrum. We anticipate that the parent will have a hard time withholding attention for a prolonged period and prevent them from providing negative attention by continuously talking to them about the need to look away and enthusiastically describe their own play. We also take this opportunity to remind them that CDI is not the entire treatment program. We reassure these parents

that ignoring is not the only strategy that we will be recommending for misbehavior. We remind them that an intensive discipline program, in which we will teach them more direct and hands-on strategies for handling tantrums, will be the next phase. If the session ends on a negative note, we often provide a mid-week call to motivate parents to hang in there with CDI.

Occasionally, we want to target a pro-social behavior that occurs so infrequently that there may be no naturally occurring opportunity to reinforce the behavior during coaching. For example, we worked with a 3-year-old who was extremely bossy and rude, demanding that his mother do things for him (e.g., “Get me a drink,” “You sit there,” “Give me that!”). After three coaching sessions, we had never heard the child ask appropriately for anything. We decided to “prime the pump.” Before we began CDI coaching, we showed the mother how to teach the child the skill of “asking nicely” (e.g., role-playing with the toy people). In this way, we were able to increase the chance that the child would ask nicely for something during the coaching session, and we could then coach the parent to provide labeled praise. With older children, we can prime the pump by simply telling them what we are looking for in the session. We might say, “Today, we are going to be working on using the words ‘please’ and ‘thank you.’ Your mom is going to be listening very closely for those words. If she hears you say them, I know she will get very excited, and so will I.” Sometimes, after a few CDI sessions, children just need to be told directly (before CDI begins) what behavior we are hoping to see and they will come through with it to please both the parent and the therapist. Once CDI begins, commands and reminders about the identified skill are no longer used because they take the lead away from the child.

Coaching Qualitative Aspects of the Parent-Child Interaction. Although parents are instructed in a set of “Do” and “Avoid” skills for special playtime, these skills do not encompass all relevant aspects of parent-child interactions or the parent-child relationship. Novice PCIT therapists often focus their coaching exclusively on these “Do” and “Avoid” skills, neglecting other qualitative aspects of the interaction. This “tunnel vision” may result in play therapy that meets the letter but not the spirit of the mastery criteria cited earlier in this chapter, and which would not be described by an objective observer as warm, nurturing, or promoting parent-child relationship enhancement. Experienced PCIT therapist-coaches integrate coaching of the core skills with coaching of more qualitative aspects of relationships, including physical closeness and touching, eye contact, vocal qualities, facial expressions, turn-taking, sharing, polite manners, developmentally sensitive teaching, task persistence, and frustration tolerance. For a DVD demonstrating advanced PCIT coaching skills with an actual client, see the American Psychological Association video by McNeil (2008).

Physical Closeness and Touching. There is no “gold standard” for the optimum amount and type of physical closeness during CDI. Healthy parent-child dyads vary widely in the nature and degree of physical closeness and touching exhibited in parent-child interactions. In securely attached parent-child dyads, preschoolers will frequently move from very close physical proximity with their parents (e.g., sitting on parent’s lap) to wider and wider exploration of the environment with

frequent returns to the security of “home base.” However, when the parent is a participant rather than observer of the child’s play, such as occurs during CDI, most securely attached children will play for extended periods of time within two or three feet of their parents, and parents will intermittently touch their children in an affectionate way.

In our work with less functional parent-child dyads, we have observed anxiously attached, clinging children as well as young children who show unusually little interest in interacting closely with their parents. We have also observed parents who hover over their children, engaging in an excessive degree of controlling physical contact, as well as those who appear to be uncomfortable with physical affection (e.g., hugs, sitting on lap) expressed by their young children. Thus, depending on the needs of the particular family, the therapist may coach parents to: (1) praise their children for more independent behaviors incompatible with clinging, like sitting in one’s own chair, (2) combine verbal praise with physical praise such as stroking the child’s hair, offering a hug, patting the child’s knee, (3) refrain from “restraining” gestures such as grabbing the child’s hand to prevent a response, or (4) move closer to the child who has distanced him- or herself from the parent, praising the child for allowing the parent to join in the game.

Eye Contact, Facial Expressions, and Vocal Qualities. Among US Caucasian populations, it is expected that the listener will make eye contact with the speaker during conversation, and a lack of eye contact may be interpreted as avoidance of emotional contact or poor social skills. Some of the parents we work with have significant social skills deficits or discomfort with emotional exchanges and profit from direct coaching in how to model good eye-contact during special playtime. Modeling good eye contact is helpful but sometimes insufficient for encouraging young children to improve their own eye-contact patterns. For young children who only occasionally make eye contact, parents are coached to praise their children strategically and enthusiastically for good eye contact. When eye contact is a very low base-rate behavior, we coach parents to shape eye contact by lifting a toy that has captured the child’s attention to the parents’ eye level while they are speaking, and then strategically praising the child for good eye contact when the parent’s and the child’s eyes meet (e.g., “I like it when you look at me when we’re talking”). This is a helpful strategy for young children with atypical development, such as those with Autism Spectrum Disorders. Please see Chapter 12 for a full description of working with children with developmental disabilities.

Sometimes, parents master the mechanics of the praising, reflecting, imitating, and describing, but the play therapy takes on a monotonous and boring quality. These parents appear to be “going through the motions” but not to have their hearts in it. On reflection, the therapist may notice that he or she is coaching in a monotone as well. When we first notice this occurring, we exaggerate our own animation, then coach parents to play in a more animated fashion, increasing the enthusiasm in their voices, adding clapping to praises for young preschoolers, and exaggerating facial expressions. As the parents add more animation to their play, we offer observations on its effect such as: “He’s looking at your face more and making better eye contact now,” “Look at her face beam. Your enthusiasm means a lot

to her,” and “Now she can really tell you’re enjoying this time with her.” When a parent does not respond to this coaching by brightening his or her affect, it is sometimes an indicator of depression, substance use, or chronic fatigue. At other times, it is an indicator that the parent is resistant to treatment. When this occurs, we temporarily suspend coaching in order to have a “heart-to-heart” discussion with the parent in which we explore these issues. Sometimes adjunctive interventions for depression or substance abuse are recommended, strategies for stress reduction are presented, and sources of resistance to treatment are identified and addressed.

Turn-Taking, Sharing, and Polite Manners. The “Do” skills of CDI, at a basic level, represent social communication skills that people of all ages use in their interpersonal relationships. Imitation begets imitation, and when parents describe, imitate, praise, and reflect during special playtime, their young children in turn imitate these skills. Over time, young children begin spontaneously praising their parents, reflecting parental verbalizations, and describing their own and their parents’ play. For many children, we believe these positive social communication skills generalize to sibling and peer interactions as well. Other valuable social skills for young children that are not listed as “Do” skills for CDI may be targeted and coached, particularly turn-taking, sharing, and polite manners.

The “Do” skill of imitation presents a natural opportunity to coach turn-taking. As the child performs an action, the parent may be coached to label it as the child’s turn and then describe it. Then, as the parent imitates the child’s action, the parent may be coached to label their own turn in play and to praise the child for allowing them to take a turn. To clarify for the parents how this sequence of interactions may be helpful to the child, the therapist may add an additional observation such as in the example below:

Child:	(puts block on tower)
Therapist:	“Now label his turn and describe it.”
Parent:	“You’re taking a turn and putting a blue block on the tower.”
Therapist:	“Good. Now label your own turn and describe it.”
Parent:	(picks up another block) “Now I’ll take my turn and add another blue block to the tower.”
Child:	“OK, go ahead mom.”
Parent:	“Thanks for letting me take my turn! Taking turns is fun.”
Therapist:	“Good labeled praise.”
Child:	“Yeah, and we’re good at it! Now I get to go, right?”
Therapist:	“You’ve taught him that taking turns can be fun, and if you keep praising him for it, he’ll probably do it more when he plays with his sister.”

Just as young children can be taught the early social skill of turn-taking during the context of CDI, they can be shaped into sharing and using polite manners. Most young children will offer the parent a toy at some point during the course of a play therapy session. We encourage parents to recognize this as sharing and reward the child with enthusiastic labeled praise followed by a parental act of sharing.

Similarly, many young children will say “please” or “thank you” at least once during a CDI coaching session. Parents are coached to label these verbalizations as good manners, provide labeled praise, and be sure to say “please” and “thank you” as appropriate to the child. For young children who do not spontaneously share or use polite manners, we coach parents to periodically model these early social skills, clearly labeling their own behavior so that the likelihood of imitation by the child is enhanced.

Developmentally Sensitive Teaching. Many parents choose to use CDI as a vehicle for developmental stimulation as well as parent-child relationship enhancement. Unfortunately, during our baseline observations of parent-child interactions, it may become apparent that the parent is not well-tuned into the child’s developmental capabilities. With preschoolers, parents may overestimate the child’s fine motor ability (e.g., building, drawing), grasp of spatial concepts, ability to remember sequentially presented information, and speed of mental processing. They may also underestimate the child’s ability to persevere at a difficult task, to pick up after him- or herself, or to select the next item needed while building. This lack of accurate perception of a child’s developmental level may become apparent during coaching. We have seen parents (1) command the child to perform a task that he or she is incapable of, (2) impatiently interfere in the child’s problem-solving by taking over and completing a task for the child, (3) fail to recognize and praise the child for small increments of developmental advancement, and (4) model inappropriately advanced levels of play. Errors such as these may cause the child to feel bad about his or her own abilities or to lose interest in performing a play task that is too difficult. In addition, the parent’s ability to effectively teach is compromised when input is pitched at either too high or too low a level.

To ensure that play therapy is conducted at the child’s level of development, parents are encouraged to adhere to the overriding rule that the child is to remain in the lead. Parents are told that it is at this level that children are most interested in the play activity and most receptive to teaching from parents. The therapist should coach parents to (1) accurately perceive their child’s developmental capabilities, (2) introduce new vocabulary when reflecting and describing, and (3) reinforce developmentally appropriate learning by naming colors, counting objects, and identifying shapes.

Task Persistence and Frustration Tolerance. Many of the children we work with are easily frustrated during play as well as during early academic tasks at school. They may show their frustration by giving up when the activity becomes challenging, becoming destructive with materials, whining, crying, or throwing temper tantrums. Once a child has been identified as having difficulty in this area, several coaching strategies may be used to teach parents how to improve their child’s frustration tolerance. It is important to note that in many cases, the parents do not have a high degree of tolerance for frustration themselves. This presents a double-edged sword. The parents may find it more difficult to teach positive coping techniques to their own child, but they may also benefit from learning new skills to cope with their own frustration, in turn modeling more appropriate coping skills for their young children.

After mastering basic CDI skills, parents can be coached to provide strategic praise for task persistence, attempting difficult tasks, and staying calm when experiencing frustration. For example, a child who is having difficulty putting a toy together might yell, “Stupid thing. It never goes in. I can’t do it.” The parent should be coached to ignore the negative talk while modeling the opposite behavior of remaining calm while working on a hard task. When the child ceases the negative talk, the parent should return to the PRIDE skills. The parent should watch carefully for examples of task persistence and coping positively when efforts fail. Parents can be coached to provide labeled praises such as, “I like it when you keep trying even though it is hard,” “Nice job of staying calm when the house fell,” and “It’s great to see that you stayed calm and just started building something else.”

Helping Parents Handle Aggressive and Destructive Child Behavior. Most children are on their best behavior during special playtime and are rarely disruptive. After all, they have their parent’s undivided attention, are playing with novel toys, and get to be in the lead. However, parents must have a strategy for handling disruptive behavior if it occurs during coaching sessions and during play sessions at home. As mentioned earlier, when children engage in mildly disruptive behavior (e.g., whining, talking back) during CDI in either the clinic or the home setting, parents are coached to address these problems using strategic attention and selective ignoring described earlier in this chapter. For more serious behaviors such as physical aggression and destructive behavior during home play sessions, we encourage parents to immediately end the special playtime. However, if aggressive or destructive behavior occurs during a clinic coaching session, we usually do not choose to suspend CDI because doing so will result in lost session time and inhibit treatment progress. Instead, we coach the parent to give the following warning, “If you _____ (e.g., hit me again, throw another toy), I will leave the room until you calm down.” The parent then will follow through on the warning by leaving the room when a repeated dangerous or destructive behavior occurs. Upon the parent’s exit, the therapist should enter the room to supervise the child. In the room, the therapist may choose any of the following actions: (1) watch the child out of the corner of the eye while ignoring, (2) review the rules regarding dangerous or destructive behavior, (3) distract escalating behavior with CDI skills, and/or (4) put the room back together (e.g., picking up overturned chairs) and remove any toys that were being misused, thrown, or broken. Once the child ceases the dangerous or destructive behavior, the parent immediately re-enters the playroom and the CDI coaching is resumed. Please see Chapter 16 for additional strategies for coaching parents with extremely aggressive and explosive children.

Coaching Sessions with Siblings. Most parents are able to extend the Child-Directed Interaction skills to the targeted child’s young siblings with little difficulty. However, when children are at different developmental levels, generalization of skills can be enhanced by having one session in which the parent is coached with the referred child and with each of his or her siblings in turn. Usually the referred child feels somewhat proprietary about special playtime in the clinic setting. For this reason, we always include some period of coaching for the referred child, even though the greater focus in this session may be on coaching the parent’s use of skills

with the siblings. For a more complete discussion of how to incorporate siblings into PCIT, please see Chapter 11.

End of Session Debriefing and Homework Assignment

We reserve the last 10 min of each coaching session for providing feedback to parents on their skills progress and discussing the upcoming week's homework. Many parents are motivated by viewing the PCIT Progress sheet. This is a record of their CDI skill acquisition and ECBI changes across sessions. They are able to view their progress from week to week, as well as monitor how close they are to reaching the mastery criteria for CDI and moving on to the discipline portion of PCIT. Feedback should begin by noting for parents areas of progress in the "Do" and "Avoid" skills, child responsiveness to these skills, and improvements in qualitative aspects of the parent-child interaction. It is important that constructive feedback that highlights areas needing further work be given as well. However, as with the coaching, the therapist must carefully attend to the balance of positive and corrective feedback so that parents leave the session feeling both encouraged by their progress and motivated to work hard in the upcoming week. Between CDI coaching sessions, parents are asked to complete a daily 5-min special playtime at home, and to record their practice on their homework sheet.

Progression of CDI Coaching Sessions

The strategies and procedures described in this chapter apply to all CDI coaching sessions. Yet, there is a typical progression in what is emphasized in each coaching session (Table 5.6 presents the typical progression of CDI coaching sessions). Across CDI coaching sessions, the therapist should use the DPICS coding to identify one or two skills that are in need of improvement during the session. The therapist then should focus coaching on these skills. Later sessions may include specific drills for particular skills that have not been mastered. There is no fixed number of CDI sessions. CDI coaching continues until parents meet the ten-ten-ten set of mastery criteria (with 3 or fewer commands + questions + negative talk). Thus, some families may be coached in CDI for only three sessions, whereas others may require six or more CDI coaching sessions. Occasionally, a parent meets mastery in the first or second CDI coaching session. In those cases, we typically will continue with CDI coaching for a couple of weeks to allow for sufficient home practice so that the following goals can be accomplished: (1) improved child outcomes (e.g., enhanced self-esteem), (2) improvements in the parent-child relationship, and (3) generalization of skills outside of the daily 5-min special playtime. Finally, therapists should realize that the 10-10-10 mastery requirement is designed to promote a warm and engaging relationship. If the parent engages in the required number of verbalizations in each category but does not perform the skills with warmth, genuineness, and enthusiasm, CDI should be continued until the relationship also improves.

Table 5.6 Typical progression of CDI coaching sessions

Session #1

Labeled praise for all PRIDE skills and ignoring
 Provide only positive feedback. Do not point out mistakes in this session
 In homework, parents are encouraged to focus on decreasing questions and increasing reflections

Session #2

Review “Parents are Models for their Children” handout and discuss anger control
 Labeled praise for all PRIDE skills and ignoring
 Go over CDI mastery criteria
 In homework, parents are encouraged to focus on increasing labeled praise

Session #3

Review “Getting Support” handout and discuss family’s social support network
 Fine tune all PRIDE skills and ignoring
 In homework, parents are encouraged to focus on skills not yet mastered

Session #4 and beyond

Review “Kids and Stress” handout
 Labeled praise for all PRIDE skills and ignoring
 Conduct 2–3 min coaching drills on whatever skills are weak
 If mastery criteria are met, introduce PDI and remind them that child does not attend the next session
 In homework, parents are encouraged to focus on skills not yet mastered

For handouts listed above, see Eyberg (1999) available at www.pcit.org

What if a Caregiver Does Not Reach CDI Mastery?

We often are asked how to handle cases in which a caregiver has had numerous CDI coaching sessions (e.g., 10 or more) and still has not reached mastery. The therapist should try coding this type of family more than once during a CDI session to determine whether coaching and anxiety reduction enhance performance. Sometimes a family can meet the mastery criteria at the middle of a session but not at the beginning. Unfortunately, however, these cases often involve families who do not practice CDI at home as prescribed. The first question for a therapist to consider is whether he or she has done everything to motivate the parent to buy in to CDI and to complete homework. Then, the therapist should examine the issues in failing to reach mastery. If the parent is able to follow the child’s lead and is missing mastery by only a couple of questions or a few PRIDE statements, it is possible that moving forward is an appropriate step. After all, CDI coding and coaching will continue in the PDI sessions. Sometimes parents have greater CDI buy-in after PDI has begun to work. If the parent simply is “not getting CDI,” the therapist should be cautious about moving forward. PDI is likely to be difficult and possibly ineffective without the relationship enhancement. Occasionally, a family may only be motivated by the

consequence that treatment may be suspended or even terminated unless the caregiver is able to commit to the daily homework requirement (e.g., a family with a history of abuse that is doing only the bare minimum to regain parental rights). A similar issue that often arises is in dual-caregiver families when one parent reaches mastery faster than the other parent. Do we move forward with the caregiver who has reached mastery or hold the family until both caregivers attain mastery? With this decision, we usually consider the degree of involvement of the caregiver who has not reached mastery. If that caregiver is the primary caregiver or highly involved in the parenting of the child, we might choose to slow progression to allow that caregiver to “catch up.” For us, however, the ultimate issue question is this one: “What is in the best interest of the child?” If a family is getting highly frustrated with the over-abundance of CDI sessions and is at risk of dropping out of treatment, it may be in the child’s best interest to move forward to PDI. Whereas PCIT will be less effective when the family does not reach mastery, it may be even more ineffective if the family terminates prematurely. As these decisions are made on a case-by-case basis using clinical experience, we recommend taking advantage of a seasoned PCIT consultant or colleagues on the PCIT listserve (sign up at www.pcit.org) when making such judgments. One of the strongest aspects of PCIT is the large change in parenting skill that occurs when enforcing the high standards of the mastery criteria. Allowing a parent to move forward without mastery should be a rare exception. By valuing and following the mastery criteria, we can ensure that each family receives its optimal “dose” of CDI.

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